



National STD/AIDS
Control Programme

SRI LANKA

2022

Annual Report

National STD/AIDS Control Programme
Ministry of Health, Sri Lanka

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Colombo 10, Sri Lanka .

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Contents

Foreword	iv
Acknowledgment	v
Abbreviations	vi
Introduction	01
Chap-1 Current Status of the HIV Epidemic	04
Chap-2 Epidemiology of Sexually Transmitted Infections	15
Chap-3 Sexual Health Clinic Services in Sri Lanka	21
Chap-4 HIV Testing Services	27
Chap-5 HIV Treatment and Care Services	35
Chap-6 Elimination of Mother to Child Transmission of HIV and Syphilis	48
Chap-7 Pre-Exposure Prophylaxis (PrEP)	51
Chap-8 Post-Exposure Prophylaxis for HIV	52
Chap-9 Prevention Interventions for Key Population Groups and Viral Hepatitis B and C	59
Chap-10 Laboratory Services	65
Chap-11 Condom Promotion	72
Chap-12 Multi-Sectoral Collaboration	77
Chap-13 Training and Capacity Building	83
Chap-14 IEC and Advocacy Programs	89
Chap-15 World AIDS Day 2022	92
Chap-16 Update of Electronic Record Systems of NSACP	96
Chap-17 Update of GF Supported Activities 2022	101
Chap-18 Financial Summery	105
Contact Information of STD Clinics	109
Annex-1	120
Annex-2	128

FOREWORD



National STD/AIDs Control Program (NSACP) being the main government organization under the ministry of health is responsible for the treatment care and prevention of sexually transmitted infections and HIV at in Sri Lanka.

Together with other government and non government stakeholders NSACP and its island-wide network of STD clinics act as the focal point for the national response for HIV and working towards the goal of ending AIDs by 2030.

Over the past few decades NSACP has been successful in many goals in the field and able to maintain the HIV prevalence at a very low level in the country. Elimination of mother to child transmission of HIV and syphilis, providing ART free of charge for all diagnosed patients with HIV covering all 24 districts, Initiation of electronic patient records within network of its STD clinics, expanding HIV testing to wider population including marginalized population groups, introducing HIV self testing, maintaining online platform for patient care and HIV testing were some of the achievements. Further the National reference laboratory is equipped with modern facilities to provide most standard laboratory diagnosis for HIV and STI management. Going parallel with current digital communication trend NSACP has taken many steps forward to expand its services through digital platforms in order to reach most vulnerable and at risk population.

This annual report contains most reliable data related to HIV and STI situation in Sri Lanka and the progress our clinical and preventive services. I acknowledge the hard work done by all the contributors to the report and appreciate the dedication by the SIM unit for maintaining an updated data source for the National program.

Dr Janaki Vidhanapathirana

Director- National STD/AIDs Control Program
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ABBREVIATIONS

ABC	Abacavir	FTC	Emtricitabine
Ab	Antibody	GFATM	Global Fund to fight against AIDS, TB and Malaria
AIDS	Acquired Immune Deficiency Syndrome	GC	Gonococcal
ANC	Antenatal Clinic	GH	General Hospital
ART	Antiretroviral Treatment	GoSL	Government of Sri Lanka
ARV	Antiretrovirals	GVAC	Global Validation Advisory Committee
ATV/r	Atazanavir/ritonavir	H2H	Heart to Heart
AZT	Zidovudine	HBsAg	Hepatitis B Surface Antigen
BB	Beach boys	HCW	Health Care Worker
BCC	Behaviour Change Communication	HCG	Human Chorionic Gonadotropin
BH	Base Hospital	HCV	Hepatitis C Virus
CBO	Community Based Organization	HDL	High Density Lipoprotein
CD4	Cluster of differentiation	HIV	Human Immunodeficiency Virus
CMV	Cytomegalovirus	HIVST	HIV Self Test
CMP	Communication Material and Publication	HPV	Human Papillomavirus
CPT	Cotrimoxazole preventive therapy	HPNP	Health Products Non Pharmaceuticals
CSDF	Community Strength Development Foundation	HPE	Health Products Equipment
DGH	District General Hospital	HR	Human Resources
DGHS	Director General of Health Services	HSS	Health System Strengthening
DOT	Directly Observed Therapy	HSV	Herpes Simplex Virus
DMH	De Soysa Maternity Hospital for Women	HTC	HIV Testing and Counselling
DQA	Data Quality Assessment	HTS	HIV Testing Services
DRV	Darunavir	ICU	Intensive Care Unit
DTG	Dolutegravir	IDU	Injecting Drug User
DTM	Diploma in Transfusion Medicine	IEC	Information, Education and Communication
DU	Drug User	INAH	Isoniazid
DUO	Dual testing	IPT	Isoniazid Prophylactic Therapy
EFV	Efavirenz	INF	Infrastructure
EIMS	Electronic Information Management System	KP	Key Population
ELISA	Enzyme Linked Immunosorbent Assay	LFU	Lost to Follow Up
EMTCT	Elimination of Mother To Child Transmission	LPV	Lopinavir
EPTB	Etravulmonary TB	LPV/r	Lopinavir and ritonavir LSCTP Living support to client/target Population
EPS	External Professional Services	MAC	Mycobacterium Avium Complex
ETU	Emergency Treatment Unit	MCH	Maternal and Child Health
FHB	Family Health Bureau	MDR	Multidrug Resistance
FPA	Family Planning Association	MLT	Medical Laboratory Technologist
FSW	Female Sex Worker	MO	Medical Officer
		MoH	Ministry of Health

MSM	Men Who Have Sex with Men	PMTCT	Prevention of Mother To Child Transmission
MTCT	Mother To Child Transmission	PrEP	Pre Exposure Prophylaxis
M&E	Monitoring and Evaluation	PI	Protease Inhibitor
NAC	National AIDS Committee	PIMS	Prevention Intervention Management system
NHP	Non Health Equipment	PSE	Population Size Estimation
NCD	Non communicable diseases	PSM	Procurement and Supply chain Management
NBTS	National Blood Transfusion Service	PWID	People Who Inject Drugs
NGO	Non-Governmental Organization	PWUD	People Who Use Drugs
NGU	Non-Gonococcal Urethritis	RAL	Raltegravir
NIID	National Institute of Infectious Diseases (IDH)	RDHS	Regional Director of Health Services
NPEP	Non Occupational Post Exposure Prophylaxis	SARD	Strategic Alliance for Research and Development
NRL	National Reference Laboratory	SIM	Strategic Information Management
NRTI	Nucleoside Reverse Transcriptase Inhibitor	STD	Sexually Transmitted Diseases
NNRTI	Non Nucleoside Reverse Transcriptase Inhibitor	STI	Sexually Transmitted Infections
NSACP	National STD/AIDS Control Programme	TB	Tuberculosis
NSP	National Strategic Plan	TDF	Tenofovir
NSES	Needle Syringe Exchange Services	TDR	Totally Drug Resistance
NTN	National Transgender Network	TG	Transgender
NVC	National Validation Committee	TOT	Training Of Trainers
NVP	Nevirapine	TPPA	Treponema Pallidum Particle Agglutination Assay
OI	Opportunistic Infections	TRC	Transport related Costs
OPEP	Occupational Post Exposure Prophylaxis	UNAIDS	Joint United Nations Programme on HIV/AIDS
OPD	Outpatient Department	UNICEF	United Nations International Children Emergency Fund
OT2	Operation Theater 2	UNFPA	United Nations Population Fund
PAC	Provincial AIDS Committee	USAID	United States Agency for International Development
PCR	Polymerase Chain Reaction	VCT	Voluntary Counselling and Testing
PCU	Primary Care Unit	VL	Viral load
PCP	Pneumocystis Carinii Pneumonia	VDRL	Venereal Disease Research Laboratory Test
PDHS	Provincial Director of Health Services	WAD	World AIDS Day
PEP	post exposure prophylaxis	WHO	World Health Organization
PHI	Public Health Inspector	XDR	Extensively Drug Resistant
PHLT	Public Health Laboratory Technician	3TC	Lamivudine
PHNS	Public Health Nursing Sister		
PLHIV	People Living with HIV		

Introduction to National STD/AIDS Control Programme



INTRODUCTION

The National STD/AIDS Control Programme (NSACP) of the Ministry of Health is responsible for coordinating the national response to HIV and sexually transmitted infections in Sri Lanka in collaboration with many national and international stakeholders. Administrative, clinical and laboratory sections of the NSACP are situated at No. 29, De Saram Place, Colombo 10. Units of the NSACP that are responsible for Strategic Information Management, Multi-sectoral collaboration and GFATM project implementation are located at No. 26, Ministry of Health Sub-office, Medi house building, Sri Sangaraja Mawatha, Colombo 10.

NSACP networks with 41 district STD clinics providing full time service delivery. Of these, 29 STD clinics provide antiretroviral treatment (ART). The only other ART facility in the country is situated in the National Institute of Infectious Diseases (NIID), Angoda. Altogether there are 30 ART facilities in Sri Lanka

VISION

“Contributing to a healthier nation, free of new sexually transmitted infections and HIV, Free of discrimination and AIDS related deaths.”

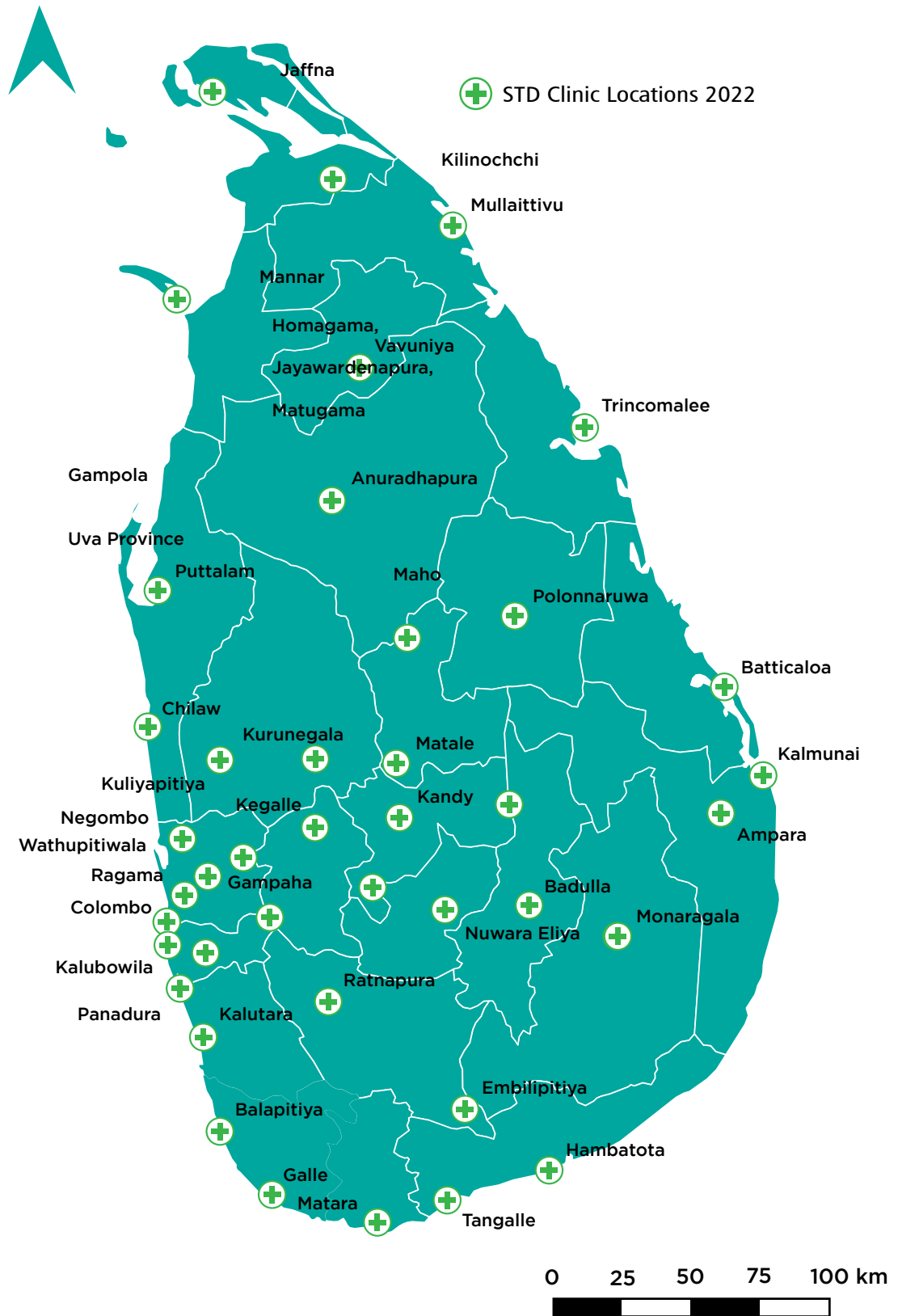
MISSION

“Quality sexual health services to prevent new HIV and sexually transmitted infections and provide comprehensive care and treatment services.”

Administratively NSACP comes directly under the Ministry of Health while most of the district STD clinics come under the administration of provincial health authorities. NSACP is responsible for training of staff, development of guidelines, preparation of strategic plans, generation of strategic information, procurement of antiretrovirals/other health products and management of grants such as Global Fund in collaboration with district STD clinics.

NSACP provides preventive, clinical and laboratory services for key populations as well as for the general population. National and district level strategic information management and surveillance are other important activities carried out by NSACP. In addition, it supports the National Institute of Infectious Diseases (NIID) of Sri Lanka to provide clinical care for people living with HIV. NSACP continue to work during the year 2022 to achieve goals even though it was another challenging year where recovery of the COVID-19 pandemic was further stressed by the worst economic crisis faced by the country but NSACP has taken all possible measures to provide best possible sexual health services in spite of the difficulties.

STD Clinics in Sri Lanka 2022



SENIOR MANAGEMENT TEAM



Seated

Dr. Sathya Herath, Dr. Geethani Samaraweera, Dr. KAM Ariyaratne, Dr. Janaki Vidhanapathirana, Dr. Thilani Ratnayake, Dr. Nimali Jayasuriya, Dr. Janaka Weragoda

Standing

Mr. S.K. Liyanage, Ms. M.G. R. N. Saman Kumari, Dr. Niluka Gunathilake, Mr. Chandana Senevirathne, Dr. Shan Muraliharan, Dr. Hemantha Weerasinghe, Dr. Piyumi Perera, Dr. Vino Darmakulasinghe, Dr. Champika Gunawardhana, Dr. Jayanthi Eliwigitigala

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Laboratory services	Dr Jayanthi Elvitigala (Consultant Microbiologist)
HIV testing	Dr Nimali Jayasuriya (Consultant Venereologist)
IEC Advocacy and Condom Promotion	Dr Thilani Ratnayake (Consultant Venereologist)
	Dr Janaka Weragoda (Consultant Community Physician)
Multisectoral Collaboration	Dr Janaka Weragoda (Consultant Community Physician)
Global Fund Project Implementation and KP Program	Dr Sathya Herath (Consultant Community Physician)
EMTCT Program	Dr Nimal Jayasuriya (Consultant Venereologist)

CHAPTER 01

CURRENT STATUS OF THE HIV EPIDEMIC, 2022

Ariyaratne Manathunge¹, Chathranie Gajaweera², Duleepa Weerasiri³

Human immunodeficiency virus (HIV) is a virus that attacks the body's immune system. It is spread through contact with infected blood, semen, vaginal fluids, and breast milk. HIV can lead to acquired immunodeficiency syndrome (AIDS), which is a condition in which the body's immune system is severely weakened. AIDS can lead to a variety of life-threatening infections and illnesses.

The HIV epidemic is a global health crisis that has affected millions of people worldwide. According to the World Health Organization (WHO), an estimated 39 million people are living with HIV globally as of 2022. The HIV epidemic has had a significant impact on health. In 2022, an estimated 630,000 people died from AIDS-related causes. The epidemic has also had a major impact on the economy.

- Adult HIV incident rate - <0.01%
- HIV incidence (per 1,000 uninfected) - <0.01
- New HIV infections - <200
- AIDS deaths - <100

The estimated number of people living with HIV (PLHIV) as of end 2022 is 4100 (3600-4700). The total number of PLHIV diagnosed and alive by the end of year 2022 is 3515. This figure was calculated by subtracting the total number of adjusted PLHIV deaths (1491) from the cumulative number of reported HIV infections (5016). Out of the total estimated number of people living with HIV, 2947 (72%) are linked with HIV treatment and care services as of end 2022 as illustrated by figure 1.1.

Status of the HIV Epidemic in Sri Lanka as of end 2022

These are the estimated figures for the Sri Lankan HIV epidemic for 2022.

- People living with HIV - 4,100
- HIV prevalence among adults - <0.1%

¹Consultant Venereologist

²Medical officer, epidemiology unit, NSACP

³Senior Registrar in Venereology

Figure 1.1 : Summary of the status of PLHIV as of end 2022

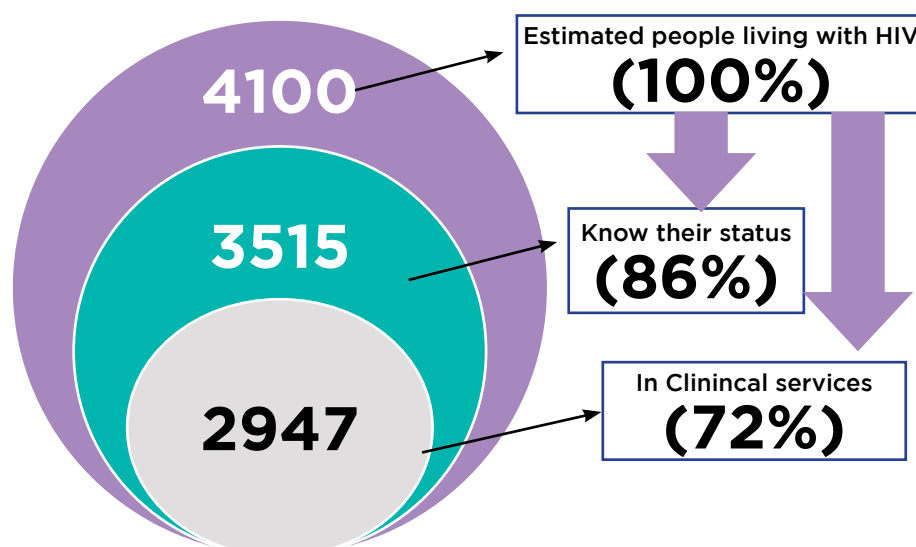
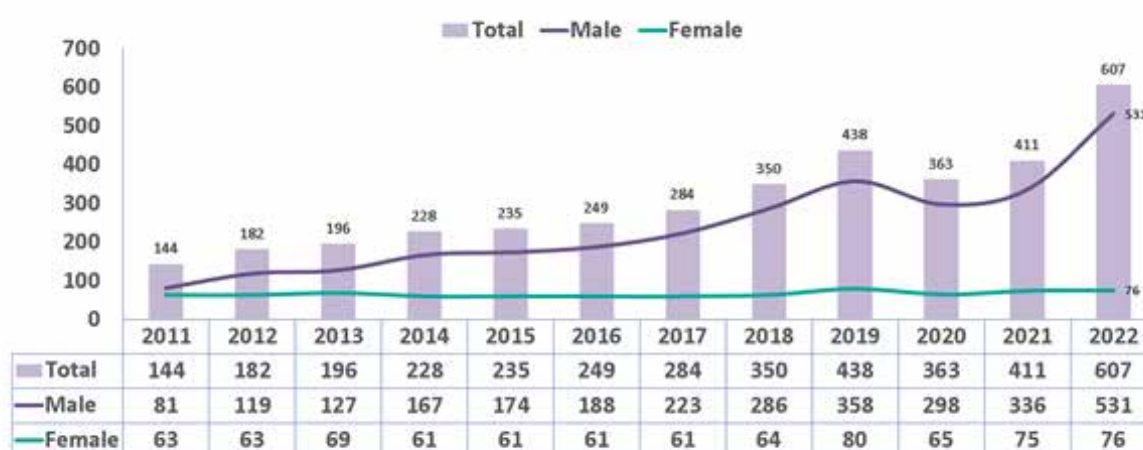


Figure 1.2 shows the trend of annually diagnosed and reported PLHIV for the period of 2011-2022. Even though men with HIV are increasing, the number of women with HIV is low and stable during this period. Out of the 607 PLHIV reported during 2022, 531 have

been men (88%). There is a 48% increase of reported HIV cases in 2022 compared to 2021. The decline in annual reported cases in 2020 and 2021 can be mainly attributed to Covid-19 related issues such as lockdowns etc.

Figure 1.2: Trends of annual reported number of PLHIV, 2011-2022



Annually diagnosed PLHIV could be newly infected people during the given year (new HIV infections) or those who infected in an earlier year but diagnosed and reported in the given year. During 2022, 82% of reported cases have been asymptomatic. However, the CD4 counts at the diagnosis gives a better and more objective measure regarding the possible duration of HIV infection in a newly reported person with HIV.

Below diagram shows the CD4 category of PLHIV newly enrolled in HIV care in 2022. This information is available only for 277 (54%) of the PLHIV who initiated ART during 2022 (515). This is due to the lack of testing facilities during the critical economic decline in 2022.

Figure 1.3: Trend of CD4 counts of PLHIV newly enrolled in HIV care, 2020-2022

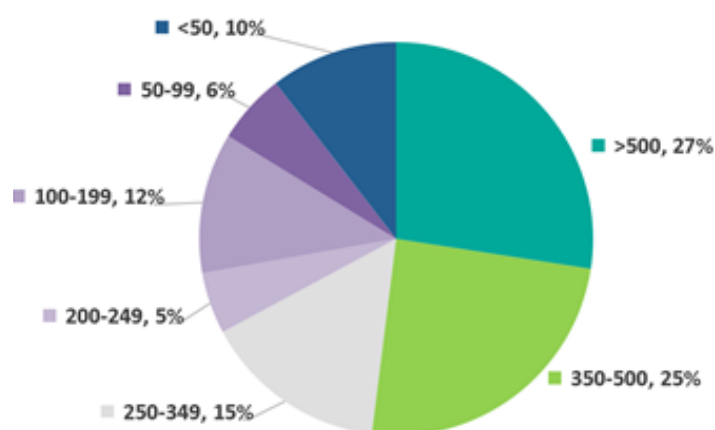


Figure 1.3 indicates that about 10% of PLHIV are in very late stage (with less than 50 CD4/mm³ count) and 28% have a CD4 count of less than 200. Only 27% has a CD4

count of over 500. This indicates that nearly three out of four newly diagnosed PLHIV have been infected in previous years.

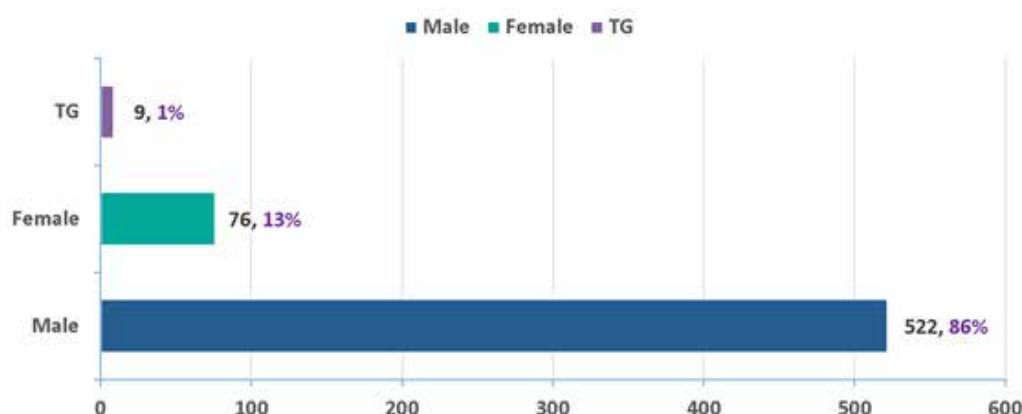
Figure 1.4: Trend of CD4 counts of PLHIV newly enrolled in HIV care, 2020-2022



The above graph indicates the trends of CD4 count categories of PLHIV newly enrolled in HIV care from 2020-2022. Almost similar trend can be seen during the period given. Only 27% of PLHIV have a CD4 count over 500. It should be noted that these percentages are calculated out of the PLHIV who has a CD4 test done in each year. In

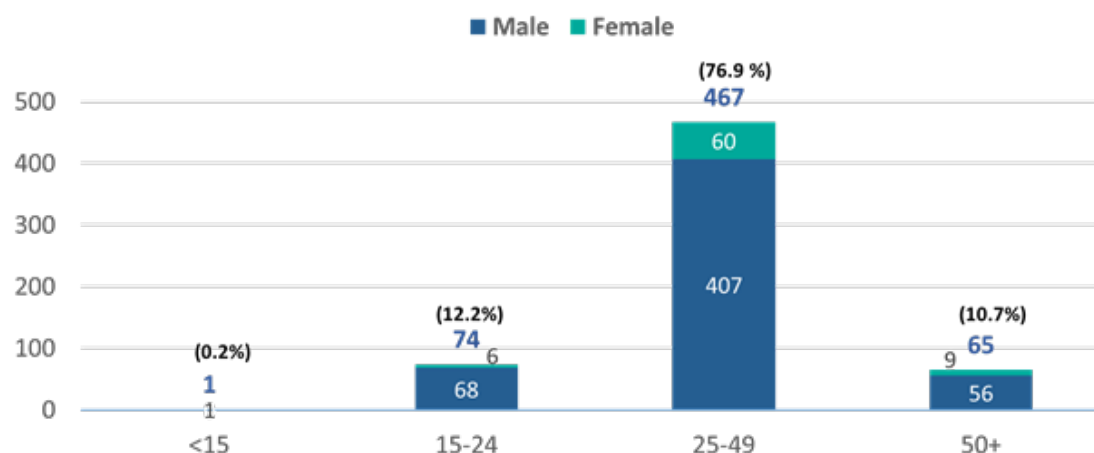
2020, 93% of PLHIV newly enrolled in HIV care had a CD4 count tested and reported. In 2021, this percentage was 90%, and in 2022, it was only 54%. The shortage of test reagents contributed to the less CD4 tests carried out during 2022.

Figure 1.5: Gender distribution of new HIV diagnoses 2022



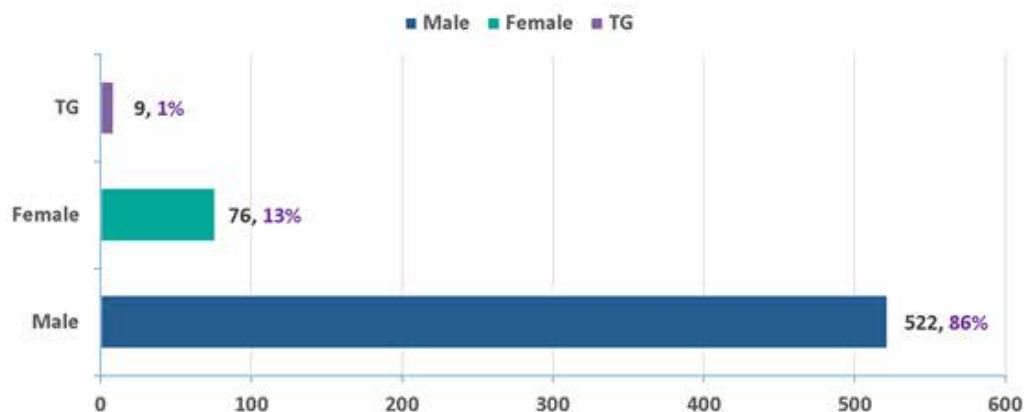
The majority of new HIV diagnoses done in 2022 are among men (86%). Thirteen percent are women and there are nine transgender persons among newly diagnosed HIV cases (1%).

Figure 1.6: Number and % of reported HIV diagnoses by age and sex at birth- 2022



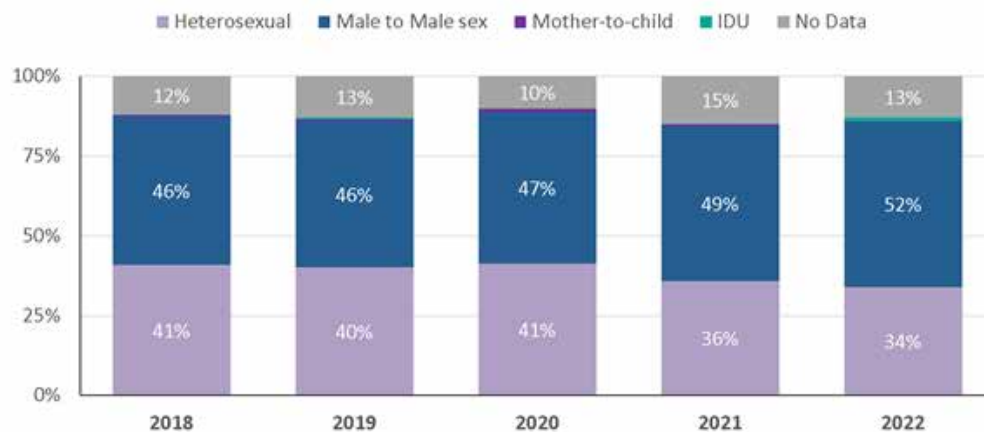
Above graph shows the sex (biological) and age category of HIV cases reported in 2022. More men are seen in all age categories. The majority of PLHIV are in the 25-49 age group.

Figure 1.7: Persons diagnosed with HIV by probable mode of transmission, 2022



More than half of all HIV cases reported in 2022 are due to male-to-male sexual transmission. Heterosexual transmission is responsible for 34% of the cases. There are 10 cases who give a history of injecting drug use. Of these eight persons also has a history of male-to-male sex.

Figure 1.8: Trend of reported PLHIV according to probable mode of transmission



The above figure shows the trend of the probable mode of HIV transmission among PLHIV who are diagnosed from 2018 to 2022. Male-to-male sex have been higher than heterosexual transmission throughout this period. Over 50% transmission in 2022 are due to male-to-male sex. About 10 -15% of cases probable mode of transmission has not been reported.

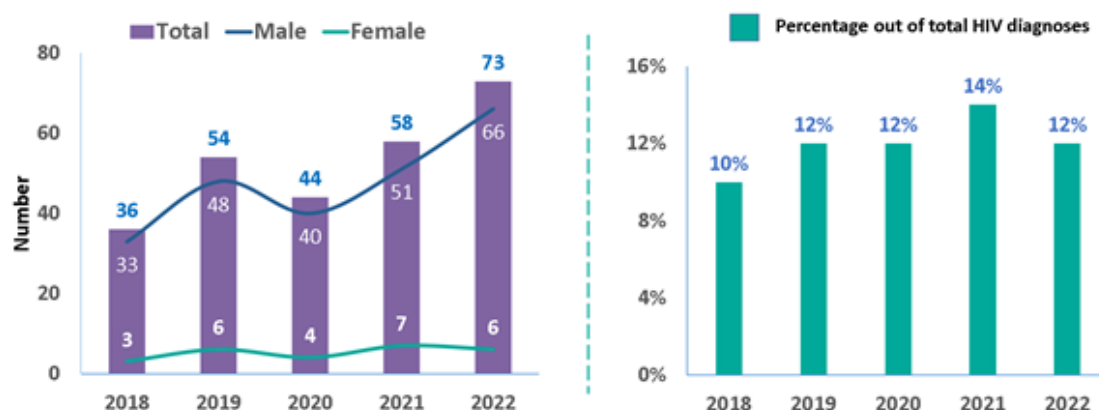
Figure 1.9: Number and percentage of young adults (15-24 years) 2018-2022

Figure 1.9 indicates the number and percentage of young adults (15–24-year age category) among PLHIV diagnosed from 2018-2022. The numbers are increasing from 36 to 73 with majority among men. Although the numbers have been increasing over the years, there is no clear trend seen in the proportion of young PLHIV among the total reported cases. It should be noted that young PLHIV are more likely to represent new HIV infections which occur during the same year.

Table 1.1: Persons diagnosed with HIV by risk categories, 2022.

	Sex				Total	
	Male		Female		No.	%
	No.	%	No.	%		
Male to male sex (MSM)	307	57.8%	0	0%	307	50.6%
Casual sex	101	19.0%	16	21%	117	19.3%
Client of FSW	27	5.1%	0	0%	27	4.4%
Spousal transmission	1	0.2%	24	32%	25	4.1%
MSM and Injecting drug use	8	1.5%	0	0%	8	1.3%
Beach boy	4	0.8%	0	0%	4	0.7%
Commercial sex worker	0	0.0%	4	5%	4	0.7%
Transgender	3	0.6%	0	0%	3	0.5%
Injection drug use	2	0.4%	0	0%	2	0.3%
Not reported	78	14.7%	32	42%	110	18.1%
Total	531	100.0%	76	100%	607	100.0%

The above table shows the reported HIV cases in 2022 according to the risk categories. Male to male sexual transmission (48%) and casual heterosexual transmission

(17%) predominate the risk categories. Clients of female sex workers (4%), spousal transmission (4%) and female sex worker (3%) are other notable risk categories. There

are 10 HIV cases with a history of injecting drug use. However, of them eight persons also gives a history of male-to-male sex.

Annual rates of newly reported HIV cases per 100,000 population

Figure 1.10 shows four maps which give the annual rate of newly reported HIV infections

per 100,000 population from 2019-2022. Colombo district shows the highest rate of HIV cases in all four years. Gampaha, Kalutara, Polonnaruwa, Galle and Matara also show higher rates of HIV infections over this period.

Figure 1.10: The rate of neroly reported HIV infections per 100,000 population, 2019-2022

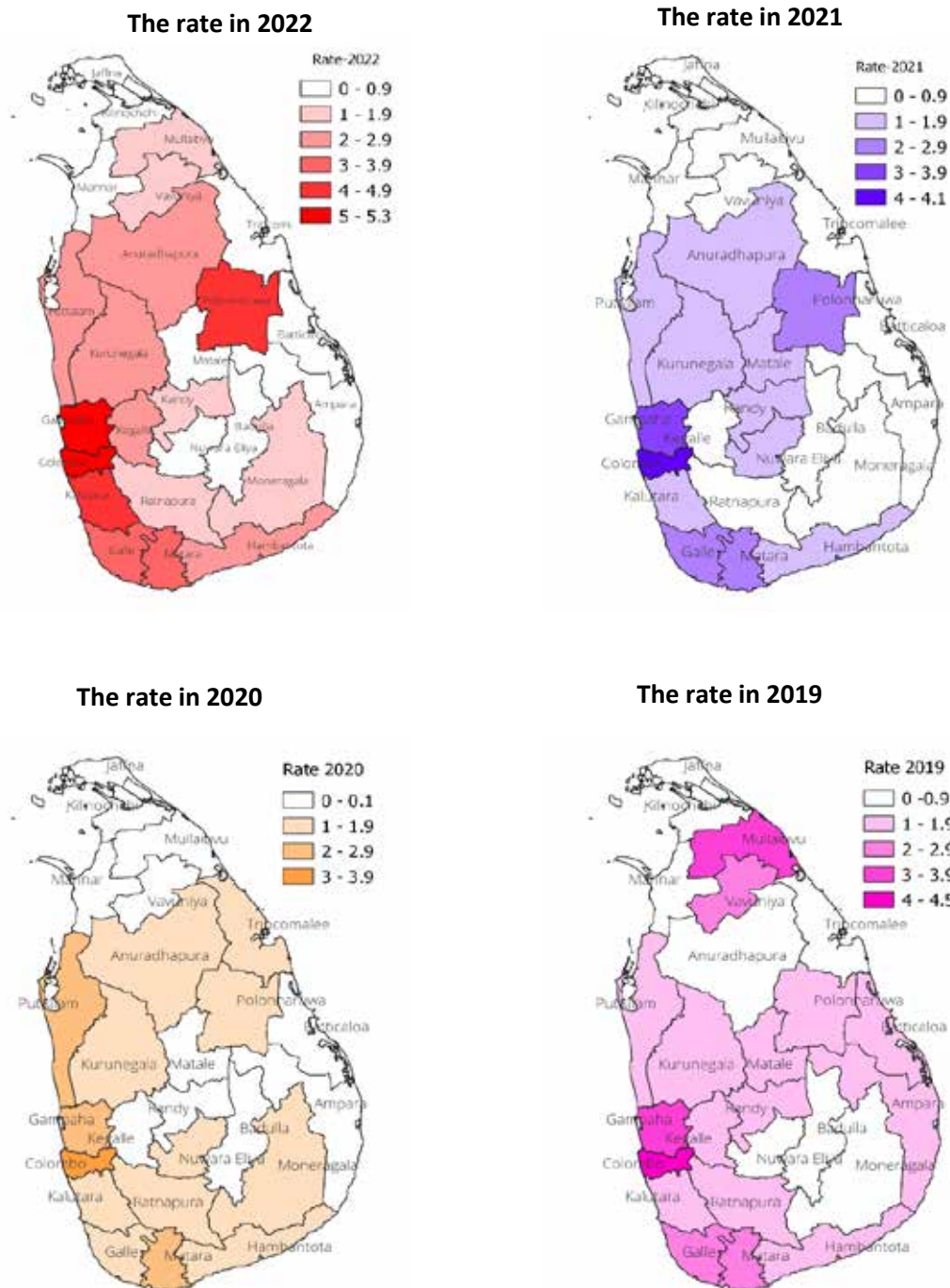
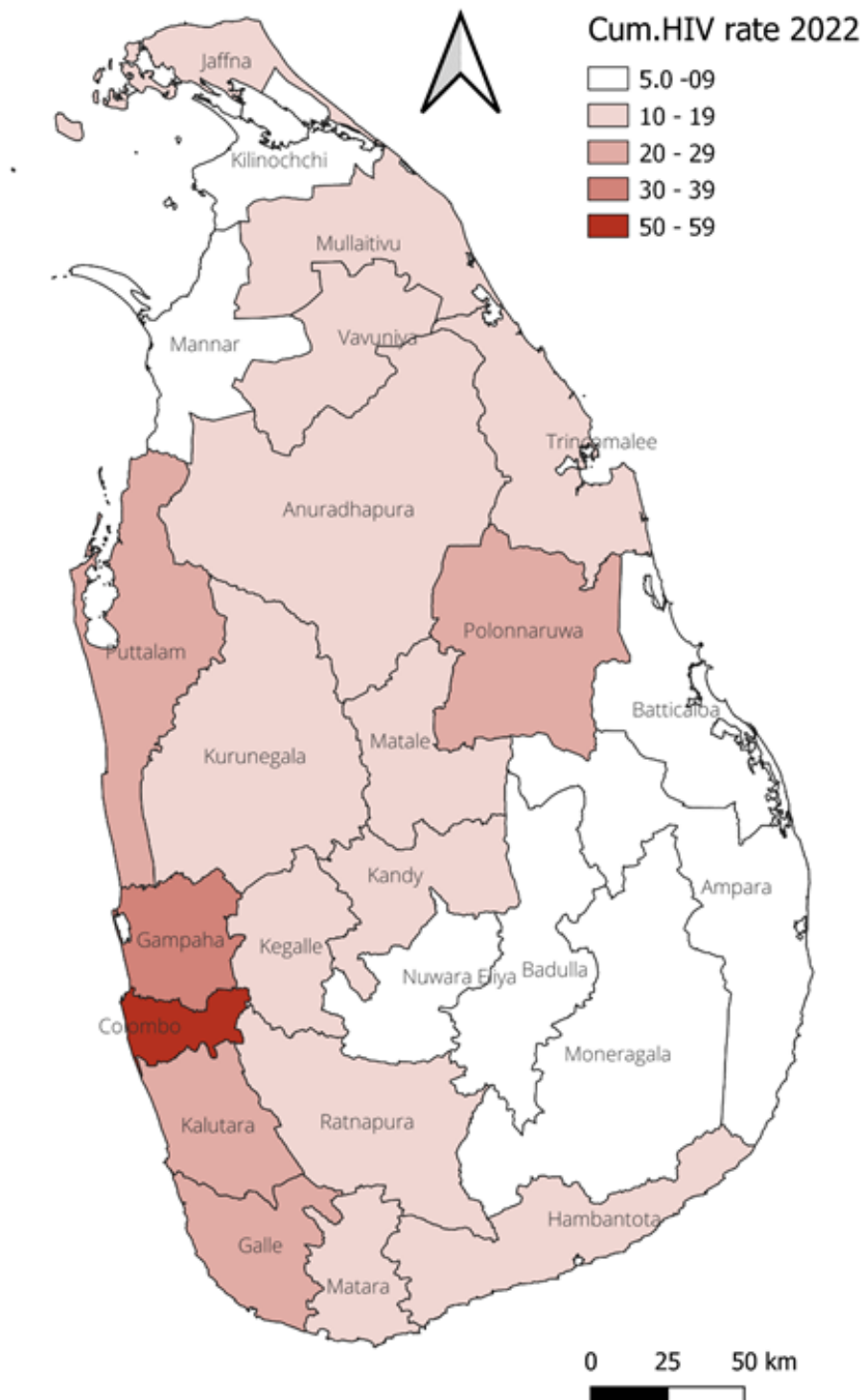


Figure 1.11: The cumulative rate of reported HIV cases per 100,000 population as of end 2022



Cumulative rate of newly reported HIV cases per 100,000 population

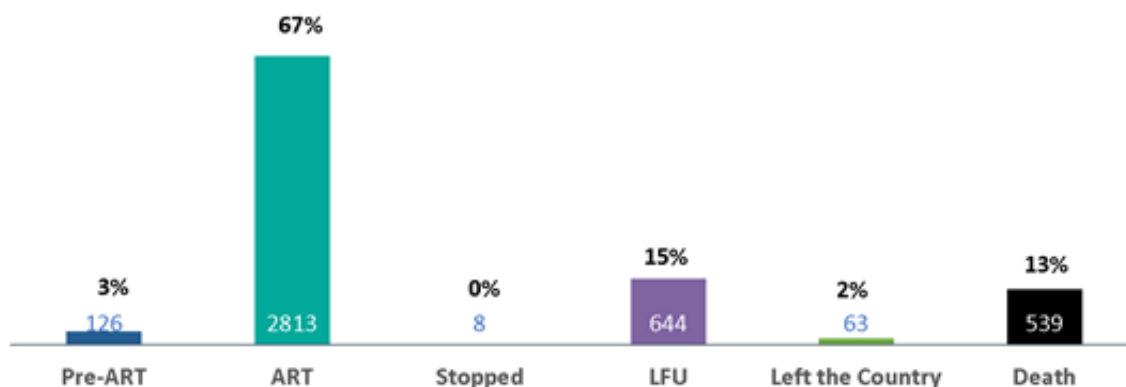
Figure 1.11 shows the cumulative rate of newly reported HIV cases per 100,000 population as of end 2022. Similar to the new HIV case rate, Colombo shows the highest rate of HIV rate per 100,000 population. In addition, Gampaha, Kalutara, Puttalam, Galle and Polonnaruwa districts show a higher cumulative HIV case rate per 100,000 population.

Current status of all PLHIV enrolled in HIV care up to 2022.

The figure given below shows the outcome of cumulative HIV cases enrolled in HIV care services since the late 1980's. Of the

total 4193, 67% (2,813) are on ART as of end 2022. Of the total, 15% are reported as lost to follow-up and 13% are reported as deaths. Only 3% are in Pre-ART stage.

Figure 1.12: Persons diagnosed with HIV by probable mode of transmission, 2022



Ending AIDS epidemic by 2030

Sri Lanka has committed to the goal of ending AIDS epidemic by 2030. According to UNAIDS, 95-95-95 testing and treatment targets should be achieved by 2025 to reach the goal of "Ending AIDS epidemic" by 2030.

The 95-95-95 treatment targets are described below.

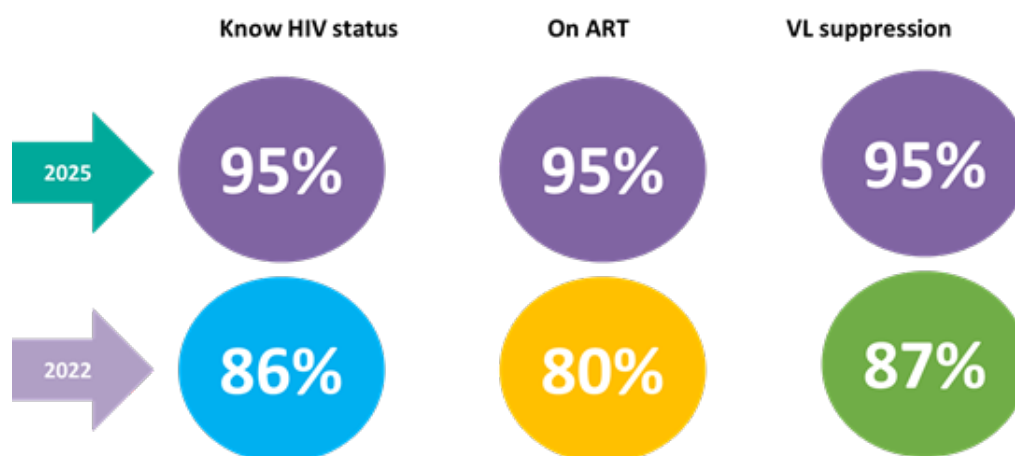
- i. 95% of all people living with HIV (PLHIV) know their HIV status.

- ii. 95% of all PLHIV who know their HIV status receive ART

- iii. 95% of all people on ART achieve viral suppression

According to UNAIDS, the ending AIDS epidemic is defined as reduction number of new HIV infections and AIDS-related deaths by 90% compared to the baseline year, 2010.

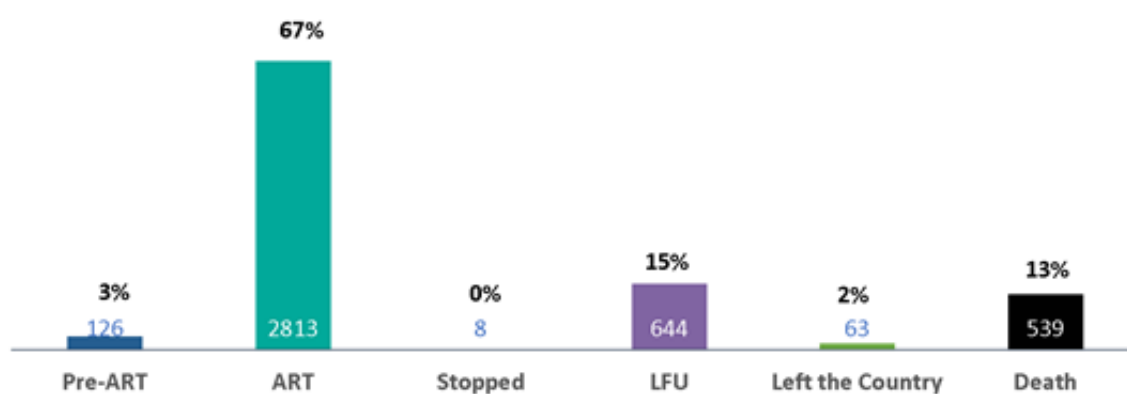
Figure 1.13: Status of 95-95-95 treatment targets as of end 2022



The above figure shows the status of knowing HIV diagnosis, on ART and viral load suppression targets as of end 2022. Currently these targets are 86-80-87. Sri Lanka needs to improve on all three targets to achieve 95-95-95 target by 2025.

Figure 1.14 indicates the cross-sectional HIV treatment cascade for 2022. The estimated number of PLHIV (4,100) taken as the denominator to calculate all these percentages of this graph.

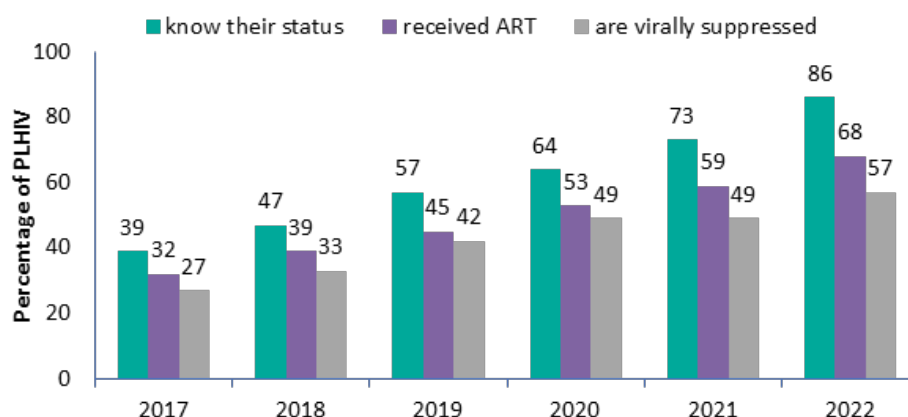
Figure 1.14: Cross sectional HIV treatment cascade as of end 2022



The below graph shows (figure 1.15) the trend of cross-sectional cascades from 2017 to 2022 based on the HIV estimation process done in early 2023. According to these the know the status (first 95%) has

increased from 39% to 86%, on treatment coverage has increased from 32% to 68% and percentage of viral load suppression has increased from 27% to 57% from 2017 to 2022.

Figure. 1.15 Trends of cross-sectional HIV treatment cascades 2017-2022



The trends of estimated number of new HIV infections and AIDS deaths

Figure1.16: Trend of estimated number of new HIV infections and AIDS deaths, 2010-2022

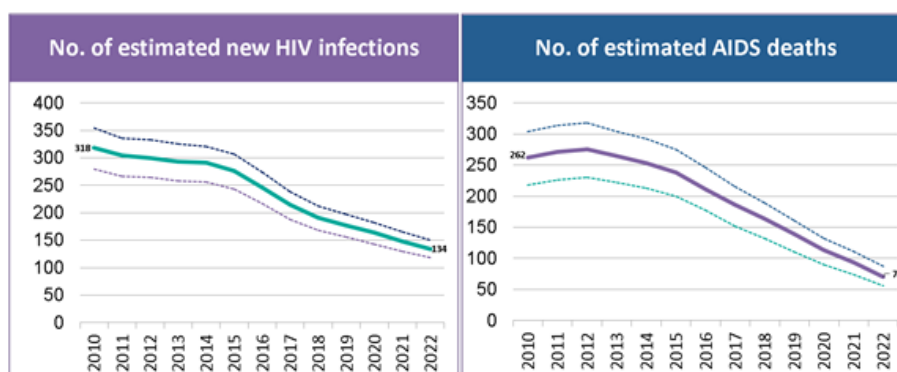


Figure 1.16 shows the estimated number of new HIV infections and AIDS deaths from 2010 to 2022. By the end of 2022, new HIV infections have reduced by 58% while AIDS deaths have reduced by 73% compared to the baseline year (2010). The goal for reduction of new HIV infections and AIDS deaths are 90% compared to the baseline values in 2010 to achieve ending AIDS epidemic.



CHAPTER 02

EPIDEMIOLOGY OF SEXUALLY TRANSMITTED INFECTIONS

Ariyaratne Manathunge¹
Chani Ratnayaka²

Sexually transmitted infections (STIs) continue to pose a significant public health challenge globally, and Sri Lanka is no exception. In 2022, the country experienced notable changes in the epidemiology of STIs, with a rising trend observed in most infections. This article provides an overview of the STI landscape in Sri Lanka during 2022, highlighting the factors contributing to this increase and the implications for public health. By understanding the epidemiological patterns and associated challenges, we can plan better

inform targeted interventions and prevention strategies to combat the spread of STIs in the country.

Similar to previous years, genital herpes, non-gonococcal infections and genital warts consist of main STIs reported during 2022. Details of number of STIs reported and male to female distribution is described in the table below. More females have reported with genital herpes and non- gonococcal infections whereas more males reported for all the other STIs.

Table 2.1: Diagnoses reported from STD clinics during 2022

	Sex				Total	
	Male		Female			
Diagnosis	No.	%	No	%	No	%
Genital herpes	1,094	27%	1,375	32%	2,469	30%
Non-gonococcal infections	659	16%	1,554	36%	2,213	27%
Genital warts	883	22%	860	20%	1,743	21%
Syphilis*	587	14%	183	4%	770	9%
Gonorrhoea	645	16%	109	3%	754	9%
Trichomoniasis	29	1%	27	1%	56	1%
Other STIs	171	4%	155	4%	326	4%
Total STIs	4,068	100%	4,263	100%	8,331	100%

¹Consultant Venereologist

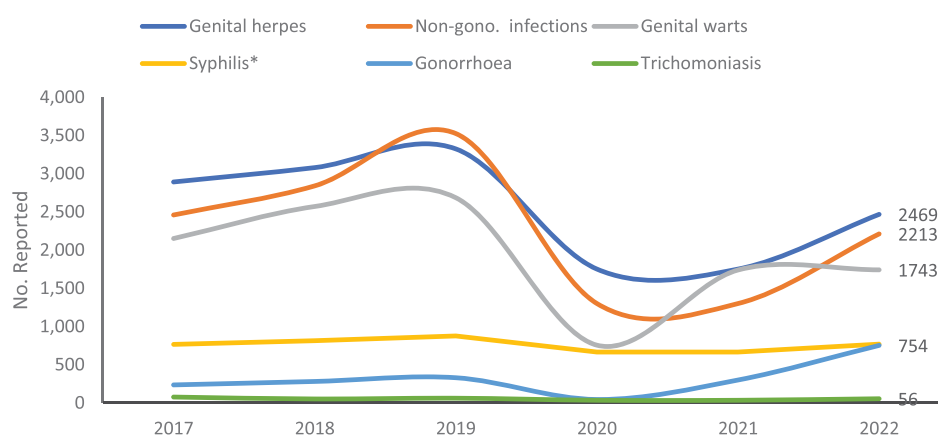
²Senior registrar in Venereology

Total number of STIs diagnosed in year 2022 was 8,331 and of these 51 % of them were viral infections respectively. Higher number of STI patients were females accounting for 51.2 %. Not clear

Most common STI was genital herpes accounting for 30% of the STI diagnoses,

while non gonococcal infections and genital warts were the next most common STIs during 2022. Most common infection among males was genital herpes (27%) while it was nongonococcal infections among females (36%).

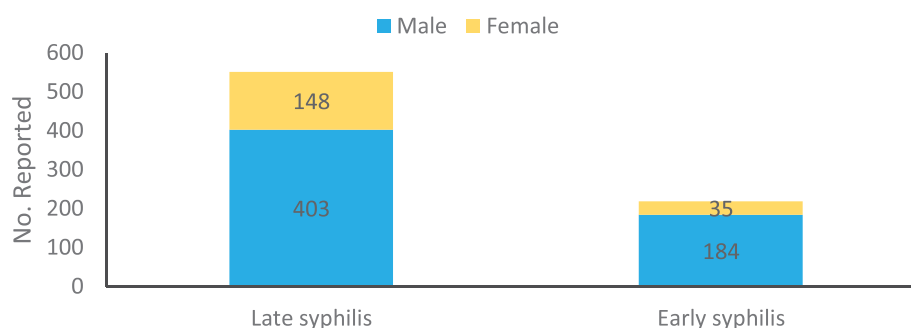
Figure:2.1 Trend of number of reported STIs, 2017-2022



The decline in STIs during 2020 and 2021 is due to Covid-19 epidemic related service accessibility issues. The same pattern was noticed in reported HIV diagnoses in this period. Nevertheless, a rising trend of STIs is seen in 2022.

A. Syphilis

Figure:2.2 Number of early and late syphilis cases by sex - 2022



Syphilis accounted for 9 % (n=770) of all STIs reported during 2022. Of these 28.4% were early infections. The majority of (76.2 %) syphilis infections were among males. Since 2018, a rising trend of early syphilis cases are seen even during the Covid-19 affected years.

Figure:2.3 The trend of syphilis cases, by stage, 2015 -2022

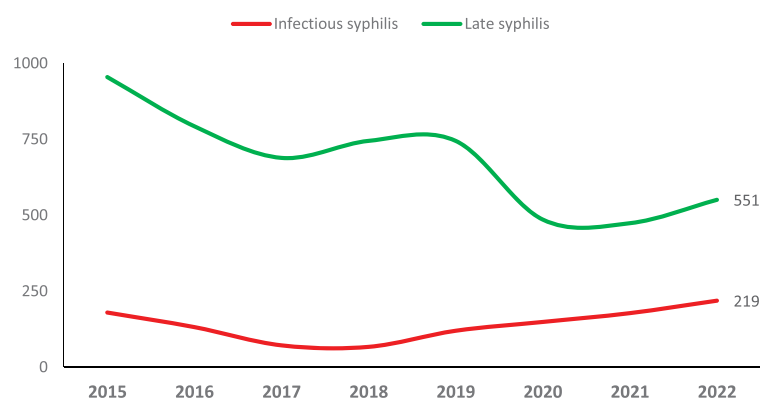
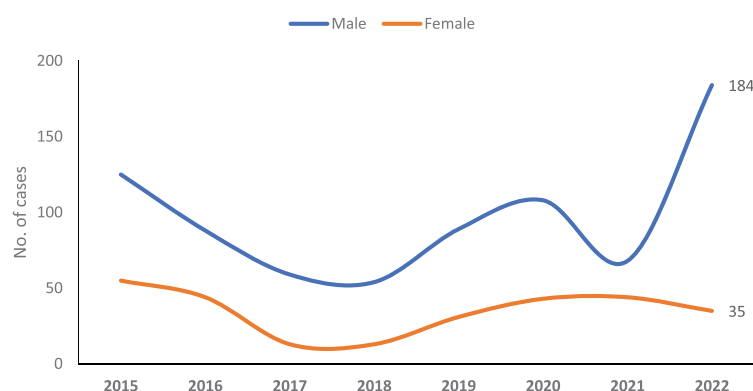


Figure:2.4 The trend of infectious syphilis cases, by sex, 2015 -2022



Infectious (early) syphilis is higher among males. A sharp raise of cases among males was noticed in 2022. Even though this may be partly due to normalization of reporting in 2022, this may also indicate the increase in unprotected sexual behaviour among males.

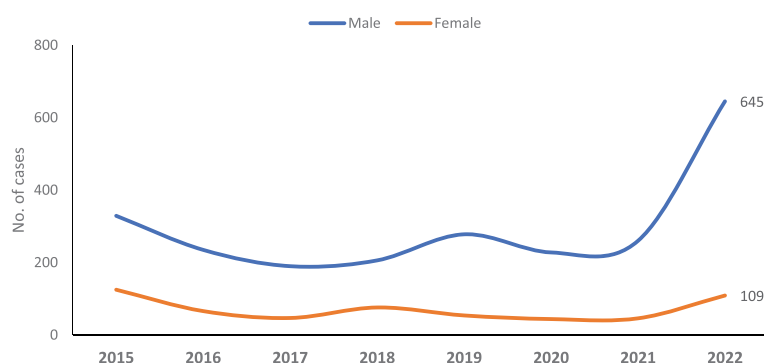
Table 2.2: Status of congenital syphilis, 2020 - 2022

Congenital syphilis per 100,000 live births	2020	2021	2022
Live births	301,706	284,848	275,321
Number of congenital syphilis cases	2	3	1
Annual rate of congenital syphilis per 100,000 live births	0.66	1.05	0.36

According to the global case definition for congenital syphilis, only one case of congenital syphilis was reported in the country during 2022. The number of live births has been declining during the past few years. The rate of congenital syphilis per 100,000 live births was 0.36 in 2022.

B. Gonorrhoea

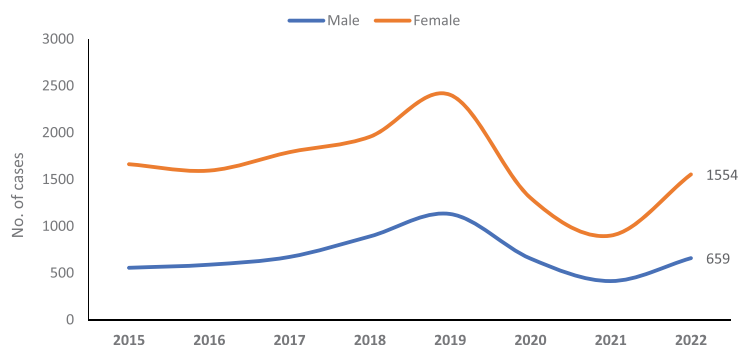
Figure:2.5 The trend of gonorrhoea cases by sex,2015-2022



There is a significant rise in reported cases of gonorrhoea infections among males is observed in 2022. This is worrying as gonorrhoea infections indicate incident (new) sexually transmitted infections. Of the total 754 gonorrhoea cases reported during 2022, the majority was males (85.5 %).

C. Non-gonococcal infections

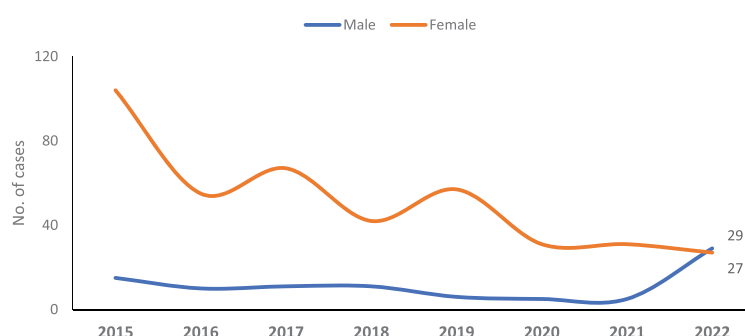
Figure:2.6 The trend of cases of non-gonococcal infections by sex,2015-2022



Non-gonococcal cervicitis and non-gonococcal urethritis have increased in 2022 compared to 2021. Among total number of non-gonococcal infections, 70.22% were among female STD clinic attendees, which was quite similar to the past trends. Non-gonococcal infections were more common among clinic attendees who are over 25 years of age.

D. Trichomoniasis

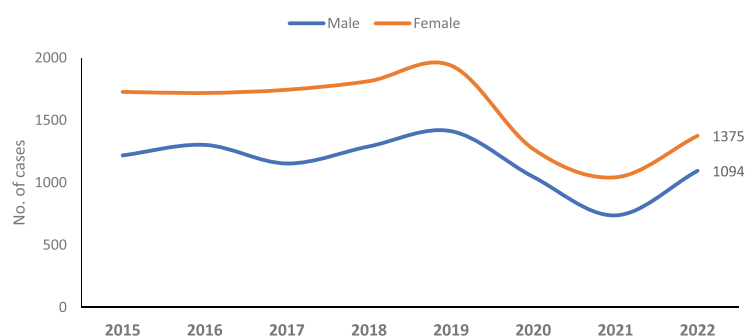
Figure:2.7 The trend of cases of trichomoniasis by sex, 2015-2022



Similar to most other STIs, trichomonas vaginalis infections have increased among STD clinic attendees in 2022 compared to the previous 2 years. In contrast to the previous years, trichomonas infections were slightly higher among males (51.78%) in 2022.

E. Genital herpes

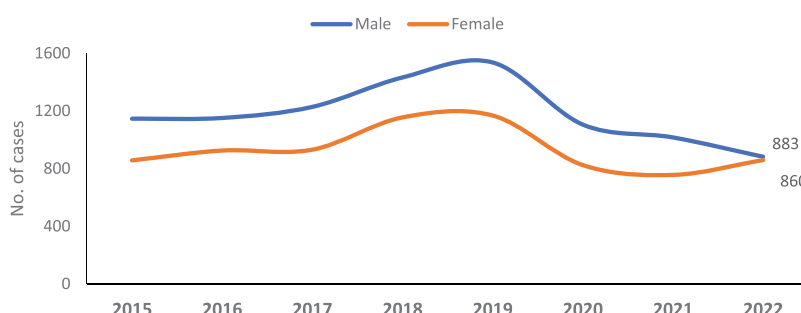
Figure:2.8 The trend of cases of genital herpes by sex, 2015-2022



The majority of genital herpes infections in Sri Lanka were observed among females, accounting for 55.7% of the total reported cases. Similar to other STIs, genital herpes (HSV) infections were more commonly reported among clinic attendees over the age of 25. In line with the overall rise in STI cases, genital herpes infections showed an increase during 2022, reflecting the general trend observed for STIs in the country during that period.

F. Genital warts

Figure:2.9 The trend of cases of genital warts by sex, 2015-2022



The majority of genital herpes infections in Sri Lanka were observed among females, accounting for 55.7% of the total reported cases. Similar to other STIs, genital herpes (HSV) infections were more commonly reported among clinic attendees over the age of 25. In line with the overall rise in STI cases, genital herpes infections showed an increase during 2022, reflecting the general trend observed for STIs in the country during that period.

It is noteworthy that all sexually transmitted infections (STIs), except for genital warts, have shown a rising trend in 2022. This increase can be partially attributed to issues related to the accessibility of services caused by the Covid-19 pandemic. In Sri Lanka, STIs are reported from public STD clinics, which may have been affected by the pandemic.

Furthermore, civil and political instability in 2021 disrupted the supply of essential services, including transportation, which further aggravated STD service accessibility in the country.

The rising number of STI cases suggests that public STI services are gradually returning to a near-normal level during 2022. However, the increase in certain bacterial STIs, such as infectious syphilis and gonorrhea among males, is concerning as it indicates the presence of unprotected risky sexual behaviors, especially among males. It is crucial to closely monitor higher-risk populations through public health surveillance to identify and address these behaviors. By doing so, actions can be taken to prevent the escalation of an HIV epidemic within these communities.



CHAPTER | 03

Sexual Health Clinic Services in Sri Lanka



CHAPTER 03

SEXUAL HEALTH CLINIC SERVICES IN SRI LANKA

Nimali Jayasuriya¹

Sexual health clinics in Sri Lanka are specialized clinics that focus on the diagnosis, treatment, and prevention of sexually transmitted infections (STIs) and other sexual health related issues. There are 41 clinics island wide and 30 branch clinics island wide. These clinics provide a wide range of services to individuals who are experiencing sexual health problems or concerns. Some of the services provided by sexual health clinics are given below.

- ▶ STI testing and treatment.
- ▶ HIV testing and counselling.
- ▶ Treatment for sexual dysfunction.
- ▶ Provision of Hepatitis B vaccine for high-risk groups.
- ▶ Conducting special clinics and services for key population groups.
- ▶ Condom promotion and condom provision.
- ▶ Partner notification.
- ▶ Defaulter tracing.
- ▶ Providing Information, Education and Communication.
- ▶ Contraception services.

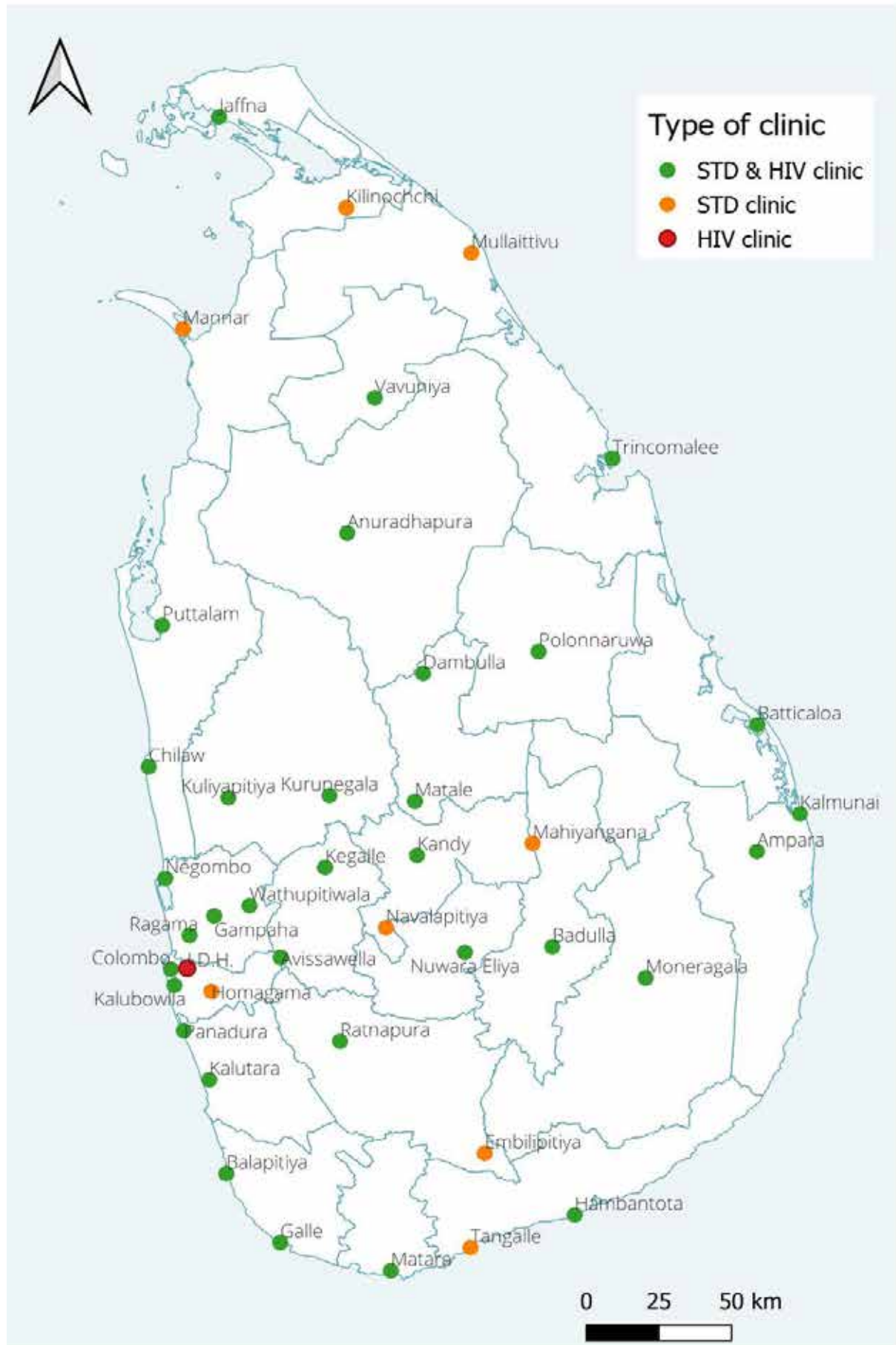
- ▶ Assessment for pre-employment and foreign employment medicals.
- ▶ Screening antenatal women for HIV and syphilis.
- ▶ Conducting special blood surveys and clinics at prisons.
- ▶ Outreach HIV testing and prevention activities for key population, vulnerable and general population.
- ▶ Conducting school health programmes
- ▶ Referrals: Sexual health clinics can refer patients to other healthcare providers or community resources for additional care or support.

Overall, sexual health clinics play a crucial role in promoting sexual health and preventing the spread of STIs. By providing comprehensive testing and treatment, education and counseling, and access to contraception and vaccination, these clinics help individuals maintain healthy sexual lives and prevent the transmission of infections to others. The way forward is to improve all currently available services with implementing services for hepatitis B/C positive individuals.

¹Consultant Venereologist

Distribution of STD clinics

Province	District	STD clinic	Branch clinic
Western province	Colombo	Colombo	Prison clinic in Welikada
		Kalubowilla	
		Awissawella	
		Homagama	
	Gampaha	Ragama	
		Gampaha	
		Negombo	
		Wathupitiwela	
Northwestern Province	Kurunegala	Kalutara	Horana, Pimbura
		Panadura	
	Kurunegala	Kurunegala	
Kuliyapitiya	Galgamuwa, Nikaweratiya, Dambadeniya		
	Puttalam	Chilaw	
Puttalam	Marawila, Dankotuwa, Kalpitiya, Anamaduwa		
Northern Province	Jaffna	Jaffna	
	Mannar	Mannar	
	Kilinochchi	Kilinochchi	
	Mullaitivu	Mullaitivu	
	Vavuniya	Vavuniya	
	Trincomalee	Trincomalee	
Eastern Province	Batticaloa	Batticaloa	BH Walachchenei, BH Kalawanchikudi
	Ampara	Ampara	Mahaoya, Dehiaththakandiya
	Kalmunai	Kalmunai	BH Kalmunai North, BH Pottuvil
Uva Province	Badulla	Badulla	Ella
		Mahiyanganaya	
	Monaragala	Monaragala	
Southern Province	Galle	Galle	
	Matara	Matara	
	Hambantota		
	Hambantota		
Sabaragamuwa Province		Tangalle	
	Ratnapura		
	Ratnapura		
Embilipitiya	BH Balangoda		
	Kegalle	Kegalle	Warakapola, Mawanella
Central Province	Kandy		
	Kandy		
Nawalapitiya	BH Teldeniya, Dumbara		
	Prison clinic		
	Matale		
	Matale	Wilgamuwa	
North Central		Dambulla	
	Nuwara Eliya	Nuwara Eliya	
	Anuradhapura	Anuradhapura	Thambuththegama
	Polonnaruwa	Polonnaruwa	Bakamuna , Medirigiriya, Welikanda, Aralaganwila, Siriputa





Pap smear

Pap smear for cervical cancer screening for eligible women attended for STI service is another service provided by STD clinics

where facilities are available. currently pap smear facility is available in twenty nine STD clinics including central clinic in Colombo and the number of pap's smears performed in 2022 by each clinic are listed below.

Table 8. Number of samples tested for cervical cytology	Number screened for cervical cytology during the quarter	Total number of reports received (Tested in any quarter)	Of them (8.2) number satisfactory for reporting	Of them (8.3) number reported as CIN-I or above
Ampara	1	0	0	0
Balapitiya	0	0	0	0
Anuradhapura	8	5	2	0
Avissawella	29	15	17	0
Badulla	7	8	8	0
Batticaloa	0	0	0	0
Chilaw	10	2	2	0
Colombo	338	270	259	11
Dambulla	0	0	0	0
Embilipitiya	0	0	0	0
Gampaha	18	18	16	0
Hambantota	7	3	3	0
Homagama	9	4	4	0
Jaffna	0	0	0	0
Kalmunai	1	1	1	0
Kalubowila	167	161	149	2
Kalutara	29	15	18	2
Kandy	61	61	53	0
Kegalle	9	9	7	0
Kilinochchi	0	0	0	0
Kuliyapitiya	7	7	6	1
Kurunagala	30	34	28	1
Mahamodara	61	42	41	1
Mahiyangana	0	0	0	0
Mannar	0	0	0	0
Matale	44	37	34	0
Matara	18	19	19	0
Monaragala	8	0	0	0
Mullativu	0	0	0	0
Nawalapitiya	4	4	4	0
Negombo	41	61	61	0
Nuwara_Eliya	0	0	0	0
Panandura	9	9	9	0
Polonnaruwa	10	5	5	0
Puttalam	0	0	0	0
Ragama	31	4	4	0
Rathnapura	5	1	1	0
Tangalle	0	0	0	0
Trincomalee	5	4	4	2
Vavuniya	12	12	11	1
Wathupitiwala	15	30	27	0
	994	841	793	21

CHAPTER 04

HIV TESTING SERVICES

Nimali Jayasuriya¹

The purpose of HIV testing services is to provide HIV tests to people in need, to give accurate and quick results, and to connect clients to relevant treatment and preventive services based on their serostatus. In Sri Lanka, the implementation of multiple HIV testing methodologies has contributed to high levels of HIV testing coverage.

The National STD/AIDS control program provides HIV testing services to clients through facility-based testing, community-based testing, and a self-testing strategy. Facility-based testing is primarily accessible for free at government STD clinics and hospitals, although it is also available at private laboratories. STD clinic employees or NGO partners provide community-based testing services. HIV testing services in prison are provided by trained prison employees or by STD clinic staff.

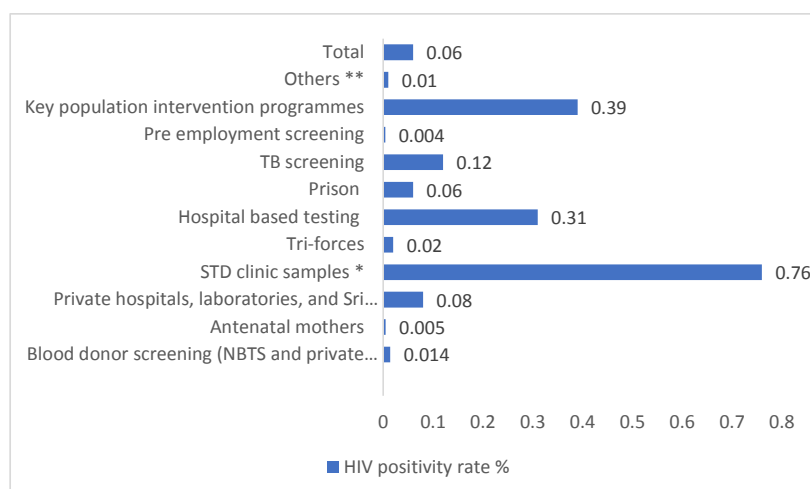
In 2022, a major limitation was the scarcity of HIV test kits and reagents. Due to the country's economic crisis, HIV screening and confirmation were not able to continue as usual due to a lack of test kit supplies.

HIV testing was interrupted in 2021 due to the COVID-19 pandemic. However, there was a modest decrease in overall HIV tests performed in 2022 when compared to 2021, owing primarily to a lack of screening of pregnant mothers through STD clinics.

Most STD clinics adopted to do HIV three-test strategy to avoid unnecessary delays in initiation of ART. HIV diagnosis was verified using the national HIV testing algorithm, and still HIV was confirmed through western blot or molecular testing.



¹Consultant Venereologist



The table below shows the number of HIV tests carried out in the country in 2022 according to different testing categories.

Table 4.1: HIV positivity rate by category of testing, 2022

Category of the samples tested for HIV	Number tested	Number confirmed positive	% HIV positivity rate
Blood donor screening (NBTS and private blood banks)	424,127	58	0.014
Antenatal mothers	185,569	9	0.005
Private hospitals, laboratories, and Sri Jayewardenepura GH	163,466	130	0.080
STD clinic samples *	37,423	286	0.764
Tri-forces	58,728	11	0.019
Hospital based testing	2,615	8	0.306
Prison	5,004	3	0.060
TB screening	6,686	8	0.120
Pre employment	25,477	1	0.004
Key population intervention programmes	22,115	87	0.393
Others **	72,045	6	0.008
Total	1,003,255	607	0.061

* (STD clinic samples include clinic attendees, outreach samples, hospital samples and testing of contacts)

* (others include rapid tests done by GPs and MOH etc.)

Despite intermittent stockouts of HIV test kits and reagents in 2022, a total of 1,003,255 HIV tests were performed throughout Sri Lanka, with 607 confirmed cases, a record high. In spite of the highest number of cases, HIV seroprevalence remains low, with a seropositivity rate of 0.06.

HIV testing is prioritized among key population groups through key population intervention programmes and people accessing STD clinics. With a seropositivity rate of 0.76, STD clinic attendees had the highest yield. Next among key population intervention programmes, which accounts for 0.4. All pregnant women are offered HIV testing, and they have the option to decline if they do not wish to be tested. HIV rapid testing is used in tertiary care hospitals, and now at base hospital level as well. It has reached the Base Hospital A level. In hospital set up rapid HIV tests are commonly used in occupational injuries: screen for symptomatic or high-risk categories; and during labour where providers need to take effective measures without delay to prevent perinatal HIV transmission. Not clear. The highest number of HIV-tests were done on donated blood samples, with 424,127 tests. Approximately 42% of HIV testing in the country was done on donated blood (by both the national blood transfusion service and private blood bank), resulting in 58 confirmed HIV positives in 2022. Private sector HIV testing contributes significantly to HIV testing, with a positive rate of 0.08. HIV testing at TB clinics or STD clinics was used to screen TB patients for HIV. Out of 6686 TB patients, 8 became confirmed positive for HIV.

HIV testing by STD clinics

All STD clinics in Sri Lanka have HIV testing facilities. Samples are mainly received from STD clinic attendees, hospital referrals, court referrals, pre-employment testing, prenatal screening, visa screening, and through population groups. HIV ELISA (Ag/Ab), HIV rapid blood test (Ag/Ab), particle agglutination, DUO HIV/Syphilis tests, and assisted HIV self-tests are used as HIV screening tests in STD clinics in Sri Lanka. Outreach HIV testing is carried out by STD staff and KP unit staff. A separate HIV rapid testing unit was established in the Colombo STD clinic to test key population groups and tests are carried out by NGO staff attached to FPA. This unit conducted 90 HIV tests in 2022, three of which were confirmed positives.

HIV testing at hospital settings

HIV rapid testing in hospitals are carried out by trained hospital staff. Due to a lack of test kits, expected number of HIV rapid tests could not be offered in 2022 in this setting. Despite the decreased number of HIV positives in hospital-based testing, the positivity rate among samples sent to STD clinics for HIV testing from hospitals remained significant. It is important to further strengthen hospital-based testing in order to diagnose symptomatic HIV patients without any delay and detect antenatal mothers who dropped out from initial antenatal screening.

HIV testing among TB patients

Tuberculosis is the leading cause of HIV-associated mortality, accounting for one of every five HIV-related deaths. The risk of developing TB is 30 times higher among

people living with HIV than among people who do not have HIV infection. NSACP collaborates well with the Tuberculosis Control Program. Blood samples from TB patients are routinely sent to STD clinics for HIV testing, while some TB clinics do it themselves at their own clinics. HIV testing should be made available to all TB patients to achieve 100% coverage.

HIV testing by private sector laboratories

Private laboratories made a substantial contribution to HIV testing in the country. HIV testing is mostly performed for clients who have symptoms, asymptomatic clients who have risk behaviors, pregnant women, insurance purposes, and visa screening.

HIV testing by National Blood Transfusion service

Sri Lanka will continue to screen all donated blood and blood products to avoid HIV transmission through infected blood. This makes the most significant contribution to the HIV screening program, detecting a

significant number of HIV-positive cases in the general community.

Key Population - HIV testing

KP HIV testing is carried out by both trained NGO workers and STD clinic staff through community-based testing or facility-based testing. Trained outreach workers primarily contributed to community-based HIV testing by using both HIV rapid testing and assisted self-testing, with a high positivity rate. In 2022, virtual HIV testing was also launched, mostly through HIV self-testing by online outreach workers.

Key population HIV testing by STD clinic staff

HIV testing among key populations is done through outreach testing and among those who directly come to the STD clinic. HIV testing for prison inmates was mostly provided as outreach testing by STD clinic staff. Few prisons have continued to conduct HIV testing on their grounds by trained prison employees.

Table 4.2: HIV testing among key population members conducted by STD clinics.

Type of key population	STD Clinics		
	Voluntary or By Outreach		
	No. tested	No confirmed positive	HIV positivity% rate
Men having sex with men	1725	245	14.203
Female sex workers	1591	3	0.189
Beach boys	35	1	2.857
Persons who Inject Drug	71	7	9.859
Trans gender women	155	0	0.000
Prison inmates	5004	3	0.060
Total	8581	259	3.018

Key population led HIV testing.

Both the case-finder model and the peer-led model were continued.

a) **The case finder approach** was continued in the districts of Colombo and Gampaha for female sex workers, men who have sex with men, and transgender women. A total of 6617 key population members were tested by this testing model, with 71 being newly diagnosed. The HIV positivity rate in this model was as high as 1.07. This model is functioned under the Family Planning Association.

b) **The peer-led targeted intervention model** was continued in 13 districts in 2022 under supervision by STD clinics. Altogether, sixteen HIV-positive cases were identified from Galle, Anuradhapura, Kalutara, Jaffna and Kurunegala districts through this model in 2022. Tables 3 and 4 show HIV testing by key population-led programmes.

Table 3 shows HIV testing through key population led HIV testing services in 2022.

Table 4.3: HIV testing through key population led HIV testing services in 2022.

Type of Key Population	NSACP KP Programme Peer-led targeted intervention model			KP Programme- Case finder model (Hybrid model)		
	No. tested	HIV confirmed positive	HIV positivity % rate	No. tested	HIV confirmed positive	HIV positivity % rate
Men having Sex with Men	6018	9	0.150	3086	65	2.106
Female sex workers	5838	0	0.000	2364	1	0.000
Beach Boys	2045	7	0.342	NA	NA	NA
Persons Who Inject Drug	734	0	0.000	525	0	0.000
Trans gender women	292	0	0.000	647	5	0.773
Total	14927	16	0.107	6622	71	1.072

The following table shows physical and virtual testing through key population led HIV testing services in 2022.

Table 4.5: Physical and virtual testing through key population led HIV testing services in 2022.

Type of Key Population	NSACP KP Programme		FPA KP Programme	
	No Tested Physically	No Tested Virtually	No Tested Physically	No Tested Virtually
Men having Sex with Men	5431	587	2913	173
Female sex workers	5397	441	1946	418
Beach Boys	1986	59	NA	NA
Persons Who Inject Drug	715	19	525	NA
Trans gender women	265	27	494	153
Total	13794	1133	5878	744

Total HIV tests carried out by STD clinics and key population led programmes were 30,130 and out of all 306 were confirmed positive.

The following table shows total HIV testing among key population groups in 2022.

Table 4.6: Total HIV testing among key population groups in 2022

Type of key population	Total tested	Total HIV confirmed positive	HIV positivity rate
Men having sex with Men	10829	319	2.946
Female sex workers	9793	4	0.041
Beach boys	2080	8	0.385
Persons who Inject Drug	1330	7	0.526
Trans gender women	1094	5	0.457
Total	30,130	346	1.148

To meet 95-95 targets by 2025 and end AIDS by 2030, the country needs to identify a range of innovative approaches to respond to contemporary needs. The new approach can help countries achieve maximum accuracy in HIV testing.

- Scale up HIV self-testing for people who are at higher HIV risk and are not testing in clinical settings.
- Adopt a standard HIV testing strategy to confirm an HIV-positive diagnosis.
- Scale up social network-based HIV testing to reach key populations who are at high risk but have less access to services.
- The use of peer-led, innovative digital communications such as short messages and videos which can build demand and increase uptake of HIV testing.
- Strengthen index case and network testing.

HIV self-testing

An HIV self-test is a test that allows an individual to test themselves for HIV in the privacy of their own home or other private setting. HIV self-tests are easy to use and involve obtaining a sample of blood or saliva, which is then tested for the presence of antibodies to the virus that causes HIV. There are different types of HIV self-tests available, including rapid diagnostic tests and oral fluid tests. Currently, NSACP uses oral fluid-based HIV self-tests. Positive results from an HIV self-test are always confirmed with a follow-up test at a healthcare facility. NSACP provides all services, including HIV self-test kits, free of charge to needy people. There were 7657 HIV STs distributed by NSACP and 5773 utilized by different categories, with 36 confirmed positives in 2022.

There are several delivery models for HIV self-testing that have been implemented in NSACP including:

- 1) Online ordering and delivery: This model involves users ordering a self-testing kit online or through Know 4 Sure web site and having it delivered to their doorstep by courier service.
- 2) STD clinic pharmacy-based distribution: In this model, self-testing kits are made available for purchase over the counter in pharmacies.
- 3) Community-based distribution: In this model, self-testing kits are distributed through community-based organizations, such as non-governmental organizations (NGOs), faith-based organizations, or public health workers.
- 4) Home-based distribution: In this model, self-testing kits are distributed through door-to-door campaigns or home visits by non-governmental organizations community health workers.
- 5) Mobile distribution: This model involves distributing self-testing kits through mobile clinics or vans that travel to different locations. A combination of delivery models may be necessary to reach the maximum number of individuals with HIV self-testing services.

This table shows HIV ST distribution, utilization and confirmed positive cases in 2022.

Table 4.7: Total HIV testing among key population groups in 2022

Category	Total tested	Total HIV confirmed positive
KP tested (assisted) by FPA	2911	27
KP tested (assisted) by STD clinics KP units	2155	9
STD clinic attendees	54	-
Pre-employment	1	-
MOH	52	-
Hospital-based testing	20	-
GPs	26	-
Migrant returnees	3	-
Prison testing	5	-
Others	546	36
Total	5773	36



CHAPTER 05

HIV TREATMENT AND CARE SERVICES

Geethani Samaraweera¹
Ariyaratne Manathunge², Himali perera³
K. Wirasinghe⁴, G D K N Karunaratne⁵

Comprehensive HIV treatment and care services include the provision of antiretroviral therapy (ART), counseling, and psychological support. They also involve support for partner disclosure and contact tracing, management of coinfections such as hepatitis B and C, including hepatitis B vaccination. Additionally, services encompass the diagnosis, management, and prophylaxis of opportunistic infections, as well as the screening and management of non-communicable diseases. Family planning, cervical cancer screening, and special services for pregnant mothers, including the prevention of mother-to-child transmission of HIV, are also provided for female PLHIV.

In Sri Lanka, these services are provided to people living with HIV (PLHIV) through HIV treatment and care centers attached to STD clinics island wide. By the end of 2022, there were 33 full-time ART centers covering all 25 districts of the country, providing comprehensive care including ART for all PLHIV who are registered for HIV care services. Currently, ART services are only available in the government sector.

Since 2016, antiretroviral therapy (ART) is offered to all PLHIV under care through the “Treat all” policy aiming to minimize mortality and morbidity associated with HIV infection. Combination ART works by blocking viral replication, leading to durable viral suppression, immune restoration, prevention of emergence of resistance, and prevention of onward transmission. ART does not completely eliminate the HIV virus, thus treatment needs to be continued for lifelong. ART has been proven to improve the quality of life of people living with HIV (PLHIV), and the progression to AIDS has become increasingly rare.

Newly registered PLHIV

A total of 594 people living with HIV (PLHIV) were newly registered in HIV care services during 2022, including males, females, and transgender individuals. The number of PLHIV who started ART during the year was 486. It is important to note that out of the total PLHIV newly registered, 78% were registered in ten ART centers: Colombo, Ragama, Kandy, Kalubowila, Kurunegala, Mahamodara, Gampaha, Kalutara, Anuradhapura, and Matara.

¹Consultant Venereologist ²Consultant Venereologist

³Consultant Venereologist ⁴Acting Venereologist

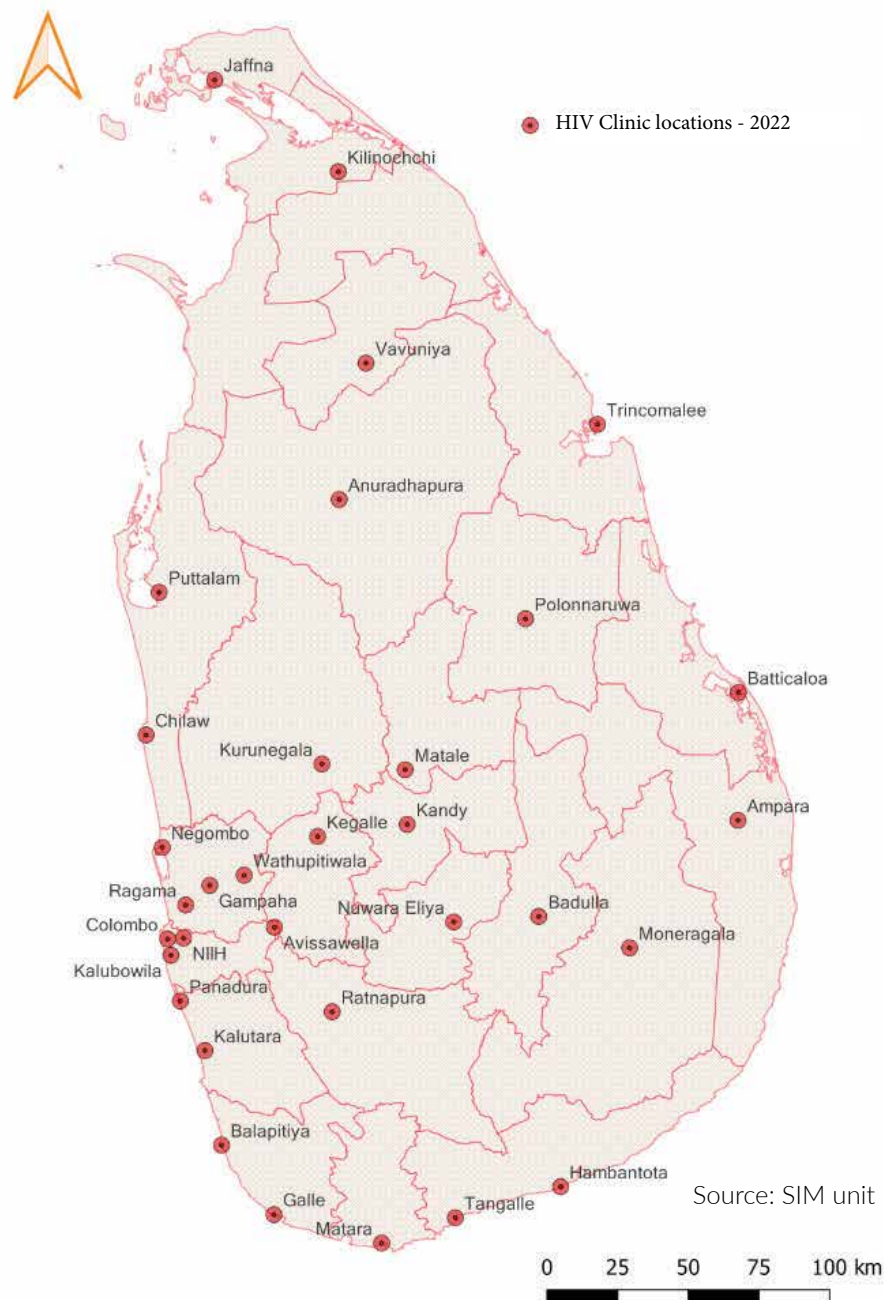
⁵Senior Registrar in Venereology

Cumulative number of people living with HIV by clinic and outcome as of end 2022

By the end of 2022, ART services were provided to PLHIV through 33 full-time ART centers across all 25 districts, with at least one ART center per district. Among the clinics, the Colombo Central HIV Clinic

located at No. 29, De Sarum Place, Colombo 10, had the highest number of PLHIV in care, with a total of 1847 individuals, accounting for approximately 44% of the total registered. Ragama had 424 individuals (10%) in care, while Kandy had 188 individuals (4.5%) in care. The total number of PLHIV registered across all ART centers by the end of 2022 was 4192.

ART Centers during 2022



	Pre-ART	ART	Stopped	Left the Country	LFU	Death	Grand Total
Colombo	30	1094		57	381	285	1847
Ragama	7	283			74	60	424
Kandy	5	143	5		12	23	188
Kalubowila	9	133		2	18	11	173
Mahamodara	12	93			34	21	160
Kurunegala	6	109	2		11	11	139
IDH	1	71			14	40	126
Anuradhapura	9	82			3	16	110
Gampaha	12	85		1	9	3	110
Kalutara	5	78			11	5	99
Negombo	2	75			12	5	94
Matara		76			7	4	87
Rathnapura	7	47			5	9	68
Chilaw	3	51			4	8	66
Jaffna		46		2	3	9	60
Polonnaruwa	4	44			9	2	59
Kegalle	1	46			5	4	56
Badulla	3	34			9	6	52
Hambantota	1	47			2	1	51
Matale		35			1	3	39
Awissawella		21			2	1	24
Vavuniya	2	18			1	2	23
Trincomalee		16			1	2	19
Wathupitiwala	1	11			7		19
Balapitiya	5	12				1	18
Nuwara Eliya		12			3	2	17
Batticaloa		13			2	1	16
Panandura		12	1	1	1	1	16
Monaragala		10				1	11
Puttalam		5			1	1	7
Ampara		5					5
Kuliyapitiya	1	2			1		4
Kalmunai		3					3
Dambulla		1					1
Kilinochchi						1	1
Grand Total	126	2692	8	63	643	539	4192

Number of people living with HIV ??? by age and gender as of end 2022

The total number of PLHIV on ART according to age group and gender was 2813 (Is this correct). The majority of PLHIV were males,

accounting for 76% of the total, with a male to female ratio of 3:1. Among the age groups, the largest proportion of PLHIV (54%) fell within the 25-49 age range. Only 0.7% of PLHIV were children under the age of 15.

Age group	Female	Male	Grand Total
5-9	5	6	11
10-14	11	13	24
15-19	6	12	18
20-24	14	106	120
25-29	43	258	301
30-34	69	324	393
35-39	86	353	439
40-44	114	340	454
45-49	83	242	325
50-54	88	185	273
55-59	75	134	209
60-64	47	101	148
65-69	24	39	63
70-74	10	16	26
75-79	5	3	8
80+		1	1
Grand Total	680	2133	2813

Number of people living with HIV by ART regimen as of end 2022

By the end of 2022, 24 different ART regimens are being used to manage PLHIV. The most commonly prescribed regimen was tenofovir, emtricitabine, and efavirenz, and 66% of PLHIV were on that regimen. The next most frequently used regimen was tenofovir, emtricitabine, and dolutegravir, comprising around 19% of the cases.

As a result of economic crisis prevailing in the country, the National STD/AIDS Control Programme (NSACP) faced challenges in procuring ART medications, resulting in a severe shortage of commonly

prescribed drugs such as dolutegravir and the combination pill of tenofovir and emtricitabine. Consequently, temporary measures were implemented, and the ART regimens were temporarily switched to DRV/r (darunavir/ritonavir) and LPV/r (lopinavir/ritonavir) protease inhibitor (PI)-based regimens, which also had limited stocks. As a result, patients had to make more frequent visits to healthcare facilities to obtain their ART medications. In order to overcome the shortage of domestic funds to procure ART, the Global Fund (GF) stepped in and provided financial support through a special emergency fund which will provide ART for latter part of 2022 and 2023. In addition, WHO also provided some special ART needed in small quantities.

ART regimen	Female	Male	Total
TDF+FTC+EFV	434	1435	1869
TDF+FTC+DTG	99	434	533
TDF+FTC+LPV/r	34	73	107
AZT+3TC+LPV/r	30	47	77
AZT+3TC+DTG	22	52	74
AZT+3TC+EFV	20	28	48
3TC+DTG	14	20	34
TDF+FTC+DRV/r	6	12	18
AZT+3TC+DRV/r	5	6	11
AZT+3TC+NVP	2	7	9
TDF+FTC+RAL	4	4	8
TDF+3TC+DTG		6	6
TDF+FTC+LPV/r+RAL	2	3	5
TDF+FTC+ATV/r	1	2	3
ABC+3TC+LPV/r		2	2
3TC+DRV/r+DTG		1	1
ABC+3TC+DTG	1		1
ABC+3TC+EFV	1		1
ATV/r+DTG	1		1
AZT+3TC+ATV/r		1	1
AZT+3TC+DRV	1		1
AZT+3TC+RAL	1		1
DTG+LPV/r	1		1
TDF+FTC+NVP	1		1
Grand Total	680	2133	2813

TB/ HIV Co-infection during 2022

People living with HIV are at a higher risk of tuberculosis (TB) infection. Thus identification and treatment of HIV – TB coinfection and treatment of latent TB is one of the most important aspects of HIV as well as TB management. Thus NSACP and NPTTCD work in hand in hand to identify and treat HIV -TB coinfecting and latent TB cases.

All newly registered PLHIV undergo TB screening and all TB patients are offered HIV

testing as part of their routine care. The chest clinics provide screening, diagnosis and treatment for TB according to the national guideline. A special chest clinic is conducted at NSACP on every Wednesdays, by medical officers from the Colombo chest clinic to assure patient convenience.

During the year, 978 HIV positive patients underwent TB screening and among them, 47 (0.05%) had pulmonary TB, and 22 (0.02%) had extrapulmonary TB. 47 individuals (0.05%) were newly diagnosed with latent

TB, It is worth noting that there were no cases of multi-drug resistant TB (MDR-TB), extensively drug-resistant TB (XDR-TB), or triple drug-resistant TB (TDR-TB) among the PLHIV. In 2022, a total of 170 PLHIV received IPT to prevent development of active TB disease.

It is noteworthy that despite the number screened for TB was nearly 23% less compared to the previous year, the number newly diagnosed with all forms of TB was much higher specially sputum negative pulmonary TB and extrapulmonary TB. In 2021 only 9 cases were being newly

diagnosed with smear negative pulmonary TB but it has gone up to 34 which is nearly a fourfold increase. Similarly in 2021 there were only 8 cases newly diagnosed with EPTB in contrast to the 22 cases reported in 2022. It is indeed a positive development that the number of reported cases of MDR (multi-drug-resistant) and XDR (extensively drug-resistant) TB (tuberculosis) was zero in 2022, compared to the four cases reported in 2021. This achievement reflects the collective efforts and progress made in combating the spread of these highly resistant forms of tuberculosis.

Status of TB and HIV Co-infection during year 2022	Number
Number of patients on anti-TB treatment at the time of diagnosis of HIV	25
Number of HIV positive patients having past history of TB	9
Number of HIV positive patients screened for TB during the year	978
Latent TB infection (This section not very clear)	47
Pulmonary TB (Sputum Smear +ve)	13
Pulmonary TB (Sputum Smear -ve)	34
Extra Pulmonary TB	22
MDR/XDR or TDR TB	0
Number of Patients on INAH prophylaxis (IPT)	170

Co-trimoxazole preventive therapy

Cotrimoxazole preventive therapy (CPT) is administered for the primary and secondary prophylaxis for *Pneumocystis pneumonia* (PCP) in all PLHIV with a CD4 count below 350 cells/ μ L. Additionally, for toxoplasma-seropositive patients with CD4 counts less than 100 cells/ μ L, CPT is also considered as primary prophylaxis against toxoplasma encephalitis. In 2022, a total of 382 PLHIV

received CPT as part of their comprehensive care.

HIV and Hepatitis co-infection during 2022

In 2022, only 229 PLHIV have undergone hepatitis B screening and, none were found to be positive for hepatitis B, resulting in a test positivity rate of 0%. The Hepatitis B

testing facility was not constantly available throughout the year due to lack of test kits. Therefore, some PLHIV were not screened for Hepatitis B though it was a part of routine baseline assessment. Similarly,

Hepatitis C screening was also not available continuously hindering routine screening of all newly diagnosed PLHIV for Hep C. Despite that four new cases of Hepatitis C was found during 2022.

Table 5.1: HIV and hepatitis co-infection during 2022

	Number tested	Number positive	Test positivity rate
Hepatitis B	229	0	0%
Hepatitis C	253	4	2%

Opportunistic infections (OI) among PLHIV in 2022

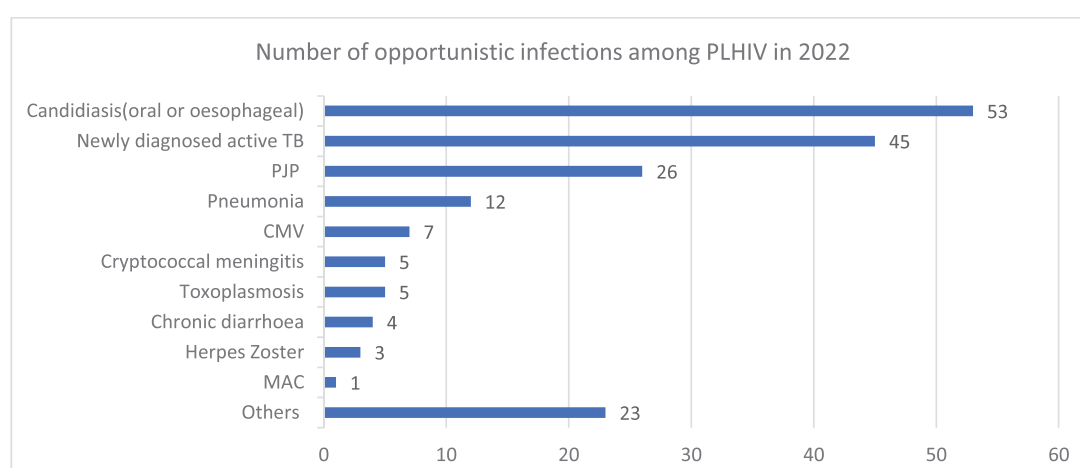
The below bar chart elaborates the number of opportunistic infections (OIs) reported among PLHIV during 2022. The commonest OI was candidiasis 57 (0.31%) followed by tuberculosis 40 (0.22%) and Pneumocystis jiroveci pneumonia 21 (0.12%). TB numbers are different in various tables

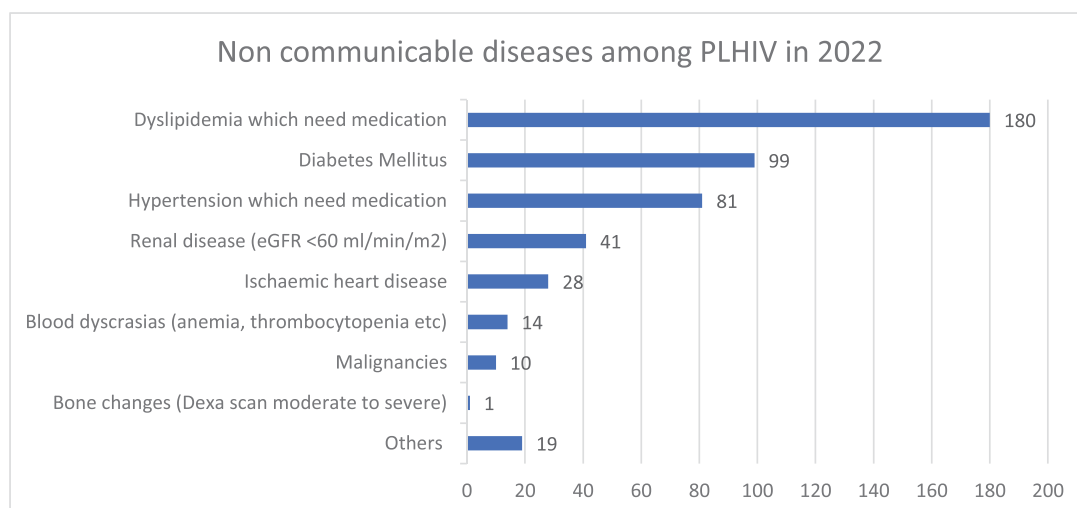
Lack of availability of OI diagnostic facilities and medications are the major challenges faced during the year 2022. Cryptococcal antigen testing was not available at MRI

throughout the year and CMV PCR was also not available time to time.

Non-communicable diseases (NCD) among PLHIV in 2022

The below bar chart elaborates reported number of non communicable diseases (NCDs) among PLHIV in 2022. Dyslipidemia was the commonest NCD reported among 180 (38%) PLHIV while 99 (20%) were reported to have diabetes mellitus and 81 (17%) had hypertension. The number of PLHIV with renal diseases (eGFR <60 ml/min/m²) were 41 (8.6%). Only one person had bone changes in Dexa scan.





However, it is identified that data reporting related to NCD is not up to the stranded thus the actual numbers are likely to be much higher than the reported numbers. In order to improve that staff was emphasized on importance of accurate data management.

Sexually transmitted infections among PLHIV in 2022

It is striking that during 2022 the number of PLHIV reported with early syphilis have increased by four-fold. Only 13 cases have been reported in 2021 compared to 57 cases reported in 2022. The number reported to

Table 5.2: HIV and hepatitis co-infection during 2022

Sexually transmitted infection	Number diagnosed in PLHIV
Non gonococcal infections	19
Newly diagnosed HSV	18
Newly diagnosed genital warts	19
Pap changes	3
Other STIs	58
Resistance to at least one NRTI	1
Resistance to at least one NNRTI	1
Resistance to at least one PI	0
Resistance to at least any other drug category	0
Resistance to more than one ARV	1
No Resistance to ARV	0

have genital warts also has doubled during 2022 as compared to 2021. On the other hand, there was no significant changes in reported number of other STIs such as GC and HSV.

HIV-I Drug resistance testing among patients with virological failure during 2022

During 2022, The number of samples sent for ART drug resistance was much less due to lack of funding. Only eight samples were been sent to National AIDS Research

Institution (NARI), India for ART resistance testing and three reports were received.

ART cohort analysis 2022

ART cohort analysis was done in 2022 for four cohorts of PLHIV who initiated ART in 2021 (12-month follow-up), 2020 (24-month follow-up), 2019 (36-month follow-up) and 2017 (60-month follow-up). The percentage of PLHIV alive and on ART at each follow-up period ranged from 83% to 96%. The percent of viral load suppression among PLHIV varied from 95% to 100%.



ART Cohort Analysis - 2022

Table1: Outcome of people with HIV who started ART in 2021 (12 months) by sex and age*

	Total	%	Female	Male	<15 yrs.	15+ yrs.
a. Number who initiated ART (N)	327	100%	52	275	1	326
Status (outcome) after 12 months of starting ART						
b. On 1st line regimen	310	95%	51	259	1	309
c. On 2nd line regimen	3	1%	0	3	0	3
d. Stopped for medical reasons (S)	0	0%	0	0	0	0
e. Lost to follow-up (F)	5	2%	0	5	0	5
f. Dead (D)	9	3%	1	8	0	9
g. Number alive and on ART(A) = (N - (S+D+F))	313	96%	51	262	1	312
h. Percentage of persons alive and on ART (A/Nx100)	313/327	96%	98%	95%	100%	96%
i. % of viral load suppression**	147/154	95%	100%	95%	100%	95%

Table2: Outcome of people with HIV who started ART in 2020 (24 months) by sex and age*

	Total	%	Female	Male	<15 yrs.	15+ yrs.
a. Number who initiated ART (N)	378	100%	72	306	3	375
Status (outcome) after 24 months of starting ART						
b. On 1st line regimen	336	89%	62	274	3	333
c. On 2nd line regimen	0	0%	0	0	0	0
d. Stopped for medical reasons (S)	0	0%	0	0	0	0
e. Lost to follow-up (F)	24	6%	5	19	0	24
f. Dead (D)	18	5%	5	13	0	18
g. Number alive and on ART(A) = (N - (S+D+F))	336	89%	62	274	3	333
h. Percentage of persons alive and on ART (A/Nx100)	336/378	89%	86%	90%	100%	89%
i. % of viral load suppression**	227/233	97%	98%	97%	100%	97%

Table 3: Outcome of people with HIV who started ART in 2019 (36 months) by sex and age*

	Total	%	Female	Male	<15 yrs.	15+ yrs.
a. Number who initiated ART (N)	342	100%	60	282	4	338
Status (outcome) after 36 months of starting ART						
b. On 1st line regimen	276	81%	49	227	3	273
c. On 2nd line regimen	7	2%	2	5	0	7
d. Stopped for medical reasons (S)	2	1%	0	2	0	2
e. Lost to follow-up (F)	37	11%	4	33	0	37
f. Dead (D)	20	6%	5	15	1	19
g. Number alive and on ART(A) = (N - (S+D+F))	283	83%	51	232	3	280
h. Percentage of persons alive and on ART (A/Nx100)	283/342	83%	85%	82%	75%	83%
i. % of viral load suppression**	226/229	99%	100%	98%	100%	99%

Table 4: Outcome of people with HIV who started ART in 2017 (60 months) by sex and age*

	Total	%	Female	Male	<15 yrs.	15+ yrs.
a. Number who initiated ART (N)	273	100%	64	209	1	272
Status (outcome) after 60 months of starting ART						
b. On 1st line regimen	222	81%	55	167	1	221
c. On 2nd line regimen	4	1%	1	3	0	4
d. Stopped for medical reasons (S)	0	0%	0	0	0	0
e. Lost to follow-up (F)	25	9%	5	20	0	25
f. Dead (D)	22	8%	3	19	0	22
g. Number alive and on ART(A) = (N - (S+D+F))	226	83%	56	170	1	225
h. Percentage of persons alive and on ART (A/Nx100)	226/273	83%	88%	81%	100%	83%
i. % of viral load suppression**	177/177	100%	100%	100%	100%	100%

*Age at ART initiation in years, ** VL <1000/ml among PLHIV whose VL is available in 2022

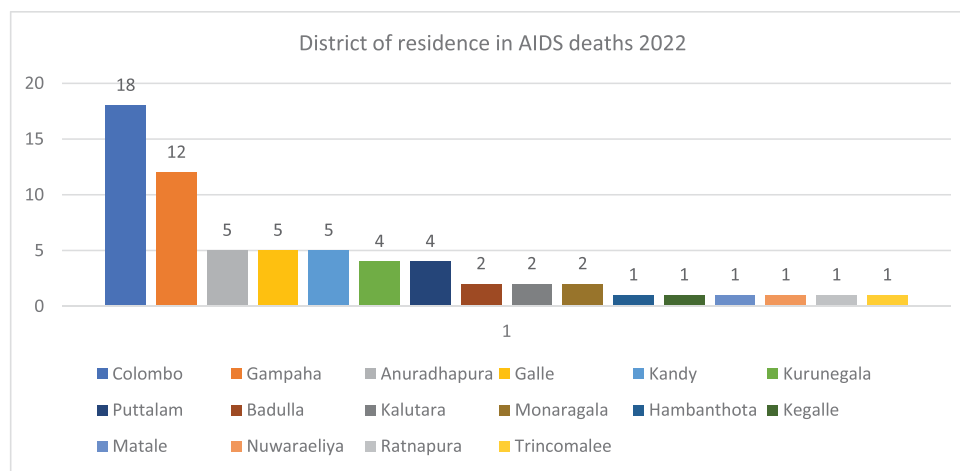
Prepared by,
Strategic Information Management Unit
National STD/AIDS Control Programme
Sri Lanka, 14th April 2023

Deaths among HIV positive cases 2022

Despite availability of ART centers in all districts of the country and practicing treat all policy, number of reported deaths among PLHIV are on rise over the years. In 2022, a total of 65 deaths were reported among PLHIV. Similar to previous years, Colombo

district had the highest number of deaths (18 deaths) followed by Gampaha district which reported 12 deaths.

The commonest cause of death was pneumonia followed by sepsis, meningitis and meningoencephalitis.



HIV/Psychiatry clinic

Psychiatric illnesses and psychological morbidities are much commoner among PLHIV compared to HIV negative population. Therefore, all PLHIV undergo comprehensive psychiatric assessment as part of their comprehensive HIV care and those who need special assessment and care are referred to special psychiatric clinics held at the central HIV clinic or to relevant psychiatric clinics in respective district hospitals.

This comprehensive approach aims to address the mental health needs of PLHIV and provide necessary support and interventions for their overall well-being.

HIV/Nutrition clinic

The clinical staff at HIV clinics regularly

assess the nutritional status of PLHIV during their clinic visits. If any nutritional problems are identified, patients are referred to the special nutritional clinic held at the central HIV clinic or the respective nutrition clinics in district hospitals.

Vaccination of PLHIV against Hepatitis B infection

All PLHIV received double doses of the Hepatitis B vaccine, totaling 40 micrograms, administered at 0, 1, 2, and 6 months. Their hepatitis B surface antibody levels are regularly monitored to assess the effectiveness of the vaccination. If the antibody levels are found to be below the required threshold (<100 IU), further doses of the vaccine are administered as per the guidelines.

The guidelines for administering the Hepatitis B vaccine and monitoring antibody levels are detailed in the publication titled “A Guide to HIV Care Services and Management of Opportunistic Infections”, second edition.

Guideline update

“A Guide to HIV Care Services and Management of Opportunistic Infections” was updated by a group of selected venereologist and other relevant specialists during 2022. The aim of this comprehensive guide is to provide up to date guidance on HIV care services and the management of opportunistic infections and NCDs. It will be finalized and published in 2023,

Training programmes conducted by HIV treatment and care unit

Monthly Webinars

The HIV care and treatment unit successfully organized monthly webinars as part of their continuous medical education initiative. These webinars covered a diverse range of topics related to sexually transmitted infections (STIs) and HIV. Renowned speakers, both internationally and nationally, delivered presentations on various subjects such as the revised National HIV Testing algorithm of Sri Lanka, updates on HIV treatment, the clinical significance of molecular analysis of HIV, joint manifestations in STIs, and ocular syphilis.

Meetings on HIV care and treatment services during 2022

HIV testing, laboratory, counseling and HIV care Subcommittee

The subcommittees on HIV testing, laboratory, counseling, and HIV care convened two meetings in 2022. The meetings

were chaired by Dr. S.M. Arnold, the Deputy Director General of Public Health Services I, and were attended by key stakeholders including, Consultants from NSACP, district Consultant Venereologists, representatives from National Programme for TB and Chest Diseases, World Health Organization (WHO), Family Planning Association (FPA), People Living with HIV (PLHIV) groups, and other supportive organizations.

The primary objective of the meetings was to enhance the quality and effectiveness of HIV care services provided by the NSACP. Discussions and deliberations were centered around HIV testing strategies, laboratory practices, counseling techniques, and overall HIV care management. By bringing together experts from different fields and engaging with relevant stakeholders, the subcommittees aimed to identify areas for improvement and implement strategies to enhance the delivery of HIV care services across Sri Lanka.

ART Drug estimation/procurement committee meetings

During 2022, number of ART Drug Estimate Committee meetings were conducted.

The working group of the ART Drug Estimate Committee comprised of Director of the National STD/AIDS Control Programme (NSACP), the National HIV Care Coordinator, Consultant Venereologists, Pharmacists from the NSACP, as well as officials from the Medical Supplies Division and the State Pharmaceuticals Corporation of Sri Lanka.

These meetings played a vital role in ensuring the timely and continuous supply of ART drugs by taking proactive measures.

By estimating the ART drug requirements in advance, the committee aimed to expedite the procurement process and maintain a consistent supply of medication for PLHIV (People Living with HIV) in Sri Lanka. This collaborative effort helped to address potential challenges related to the availability and distribution of ART drugs, ultimately improving the delivery of HIV care services in the country.

PLHIV groups and other supportive organization

PLHIV organizations as well as other support organizations provided their fullest support for continuation of quality care for PLHIV in Sri Lanka.

Three PLHIV organizations; Positive Women's Network, Positive Hopes Alliance and Lanka Plus provide various support services, such as counseling, peer support, financial support and educational programs, to enhance the well-being and quality of life for PLHIV. In addition, they actively participate in all subcommittees of NAC as important stakeholders and actively involve in decision making with regard to HIV prevention, treatment and care.

FPA (Family Planning Association of Sri Lanka): provide support for PLHIV especially financial support for travelling and laboratory investigations. In addition, National AIDS Foundation (NAF): continued its financial and other services for PLHIV throughout 2022 as well. Nest (Networking, Education, Support, and Treatment): offer counseling, peer support, educational programs, and advocacy initiatives to improve the quality of life for PLHIV and combat HIV-related stigma and discrimination.

These organizations provide important contribution for HIV/AIDS prevention, care,

support services in Sri Lanka and always work in hand in hand with NSACP.

Challenges

During 2022, the HIV program in Sri Lanka faced significant challenges. Human resource constrains, inadequate infrastructure and delays in ART procurement are some of them. During 2022, there were only 28 consultant Venereologists available to manage 41 STD clinics. 10 ART centers were managed without a fulltime consultant venereologist.

Though the number of PLHIV under care gradually increasing over the years neither infrastructure nor the human resources allocated to STD clinic had expanded adequately challenging delivery of comprehensive quality care for PLHIV.

Shortage of essential ART, delay in the ART procurement process further disrupts the continuum of HIV care.

Shortage of viral load and CD4 count testing, biochemical, hematological, and other necessary testing facilities is another challenge the programme faced during the year. This hindered the ability of the programme to provide standard quality care for PLHIV and increased the chance of development of drug resistance.

Despite these challenges, the programme has been able maintain an uninterrupted supply of ART and others essential services for PLHIV with minimum negative effects. However, it is important to address the above challengers timely and appropriately to ensure the effective delivery of HIV care and treatment in Sri Lanka.

CHAPTER 06

ELIMINATION OF MOTHER TO CHILD TRANSMISSION OF HIV AND SYPHILIS

Nimali Jayasuriya¹

EMTCT of HIV and syphilis programme

Sri Lanka became the fourth country in the Asia-Pacific region to eliminate mother-to-child transmission (EMTCT) of HIV and syphilis in 2019. First revalidation assessment was successfully achieved by the maintenance of the EMTCT program for in December 2021. The Global Validation Advisory Committee (GVAC) was given some recommendations for improvement in each area in the next maintenance validation report, which will be received in December 2024.

Due to the economic crisis, the year 2022 was critical for the country's STI screening program. The Ministry of Health in Sri Lanka has fully supported ANC screening for HIV and syphilis in the country; however, from August to December 2022, there were some stockouts of HIV test kits and reagents. Therefore, some STD clinics were not able to continue ANC screening as usual. Consultants at the NSACP have decided to delay ANC screening up to 28 weeks of POA and get the tests done from a recommended private laboratory for those who are capable. Anyhow, ANC screening was continued, and FHB data showed 98.1

and 98.5 percent coverage of HIV and syphilis testing at the time of delivery in 2022.

In 2022, WHO agreed to donate HIV/Syphilis DUO rapid test kits for the ANC screening program in Sri Lanka, and the NSACP had received 345,000 DUO test kits by mid-December 2022. Since then, ANC screening has continued with DUO HIV/Syphilis rapid test kits. The EMTCT unit with the Consultant Microbiologist has conducted a training program for all consultants, venereologists, MO STDs, and MLTs on DUO testing and the testing algorithm. MLTs do the DUO rapid tests for ANC blood samples at the STD clinic.

District and National reviews

National-level meetings and district reviews were held with all stakeholders and the district MCH team during 2022. District reviews were organized with the participation of all stakeholders in the district and with the support of the WHO. The national coordinator for EMTCT and convenor of the National Validation Committee (NVC) joined the review meetings virtually or physically.

¹Consultant Venereologist

Pregnant women with HIV in 2022

The total number of HIV positive women found to be pregnant in 2022 was 15 and of these, 9 pregnant women were identified as having HIV during antenatal screening. The other women were known to be women with HIV who became pregnant while on ART. There were total of 18 deliveries occurred and all infants were started on antiretroviral prophylaxis, and early diagnostic tests were arranged. All mothers who received EMTCT services for HIV delivered uninfected babies.

Paediatric HIV diagnoses in 2021

Paediatric HIV diagnoses in 2022 were zero.

MTCT of syphilis

During 2022, 61 pregnant women with syphilis delivered, and all these pregnant women received appropriate services. Three babies were treated for congenital syphilis and two pregnant women were treated with the non-penicillin regimen and other was due to inadequate treatment. The rest of the babies received prophylactic Benzathine Penicillin treatment. One child was diagnosed with late congenital syphilis at the age of 5 years.

Recommendations and Way Forward

In the Year 2022, the EMTCT program was further improved based on the recommendations of the GVAC. However, it was a challenging period due to the economic crisis in the country. It is important to maintain the interest shown by all stakeholders to achieve satisfactory impact and process indicators and to

sustain the success in the coming years. The support extended by all relevant authorities throughout the year, including the MoH, FHB, provincial authorities, secondary and tertiary care hospitals, as well as STD clinics, has to be commended. The Ministry of Health was the main sponsor of the program, and WHO and GFATM supported further strengthening the program in 2021 to achieve certification of the maintenance of validation status for the elimination of mother-to-child transmission of HIV and syphilis.

Pre-exposure prophylaxis (PrEP) is a highly effective HIV prevention strategy that involves taking antiretroviral medication daily or on demand to reduce the risk of HIV infection in people who are at high risk of HIV exposure. It is important to note that PrEP is not a replacement for other HIV prevention strategies, such as condom use. Rather, PrEP can be used in conjunction with other prevention methods to reduce the risk of HIV transmission. Additionally, PrEP is most effective when taken consistently, so individuals who are considering PrEP should be counselled on the importance of adherence to the medication regimen.

The PrEP pilot project was initiated at the STD clinic in Colombo in 2020, and it was continued in 2021 based on the pilot protocol. In 2022, both sexually transmitted disease (STD) clinics in Sri Lanka and community PrEP clinics offered PrEP as part of a comprehensive HIV prevention strategy. STD clinics provide PrEP services along with other sexual health services for PrEP clients. Both event-driven and daily PrEP were given based on the client's needs, and this PrEP service was made available during routine clinic hours at the STD clinic.

During 2022, there were 215 new clients who enrolled in PrEP services. 146 were on-demand PrEP, while the others were on daily PrEP. Most STD clinics carried PrEP medication in their pharmacies, but there was no demand from clients. Most of the PrEP was distributed through the Colombo STD clinic. There were few people who obtained PrEP services from, Ragama, Puttalam and Wathupitiwela STD clinics.

Challenges

- ▶ Lack of awareness among high-risk groups and health care workers.
- ▶ Poor Follow up.
- ▶ Adherence problem.
- ▶ Stock outs of medication.

- ▶ Unavailability of Sri Lankan PrEP guideline.

Way forward

- ▶ Expanding the PrEP service delivery in all STD clinics.
- ▶ Demand generation and scale up of PrEP among KPs.
- ▶ Increase awareness among health care workers.
- ▶ Development of PrEP guideline and staff training.
- ▶ Expanding service delivery models.



CHAPTER 07

PRE-EXPOSURE PROPHYLAXIS (PrEP)

Nimali Jayasuriya¹

Pre-exposure prophylaxis (PrEP) is a highly effective HIV prevention strategy that involves taking antiretroviral medication daily or on demand to reduce the risk of HIV infection in people who are at high risk of HIV exposure. It is important to note that PrEP is not a replacement for other HIV prevention strategies, such as condom use. Rather, PrEP can be used in conjunction with other prevention methods to reduce the risk of HIV transmission. Additionally, PrEP is most effective when taken consistently, so individuals who are considering PrEP should be counselled on the importance of adherence to the medication regimen.

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- Poor Follow up.
- Adherence problem.
- Stock outs of medication.
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Way forward

- Expanding the PrEP service delivery in all STD clinics.
- Demand generation and scale up of PrEP among KPs.
- Increase awareness among health care workers.
- Development of PrEP guideline and staff training.
- Expanding service delivery models.

¹Consultant Venereologist

CHAPTER 08

POST-EXPOSURE PROPHYLAXIS FOR HIV

Nimali Jayasuriya¹

Post-exposure prophylaxis (PEP) is a medical treatment that can help prevent HIV infection after someone has been exposed to the virus. PEP involves taking a combination of HIV medications for a period of 28 days after the potential exposure and it is important to start the treatment as soon as possible after exposure, ideally within 72 hours. Exposure can be occupational (o-PEP) or non-occupational (n-PEP), with non-occupational exposure being related to sexual or intravenous (IV) drug exposures. Recommended PEP regimens are TDF 300 mg daily + FTC 200mg daily + DTG 50mg daily or RAL 400mg bd. In the absence of first line regimens alternative regimens are used

Since 2017, the National STD/AIDS Control Programme and STD clinics have provided post-exposure prophylaxis following occupational exposure. This service was primarily for the healthcare workers following accidental exposure to potentially hazardous material at their workplace. It is guided by the health circular (01-19/2017) on the management of health care workers following occupational exposure to blood and other body fluids. Non-occupational post-exposure prophylaxis services were

launched in 2020, and a new protocol was introduced.

This treatment can reduce the risk of HIV infection after the virus has entered a person's body through sexual exposure. This service is available through the network of STD clinics island wide. Antiretroviral drugs are issued following detailed counseling on the importance of treatment along with their potential side effects. ART starter packs are available to use at predetermined 24-hour functioning units in hospitals (ie. emergency units and intensive care units) and security point of the Colombo STD clinic. These packs are helpful in initiating PEP during emergency situations specially out of STD clinic working hours. Post-exposure prophylaxis should be continued for 28 days with good adherence to prevent HIV transmission.

A total of 1983 clients attended STD clinics during 2022 for oPEP and services. Of these, PEP started with 37 clients, and 20 completed the full 28-day course of ART. Of the 77 clients who attended nPEP services, 58 started on ART and 40 completed the treatment. One of them tested positive for HIV.

¹Consultant Venereologist

This table shows details of HIV post exposure prophylaxis following provided post-exposure prophylaxis following occupational and non-occupational exposures.

Details of HIV post exposure prophylaxis following accidental injuries	oPEP	nPEP
Number of clients who attended for PEP services	1983	77
Number started on PEP	37	58
PEP 28 days completed	20	40
Number came for screening after 3 months	269	13
Became positive for HIV	0	1

Location information of ART for OPEP in Sri Lanka during 2022

District	Institution	Unit of location	Contact Number
Ampara	DGH - Ampara	PCU	632222261
	BH - Dehiaththakandiya	ETU	272250344
	BH- Mahaoya	PCU	632244061
Anuradhapura	TH – Anuradhapura	Medical ICU	0252222261 - Ext 700/701
	STD Clinic		025 2236461
	BH- Thambuththegama	Medical ward	025 2276262
	BH – Padaviya	Medical ward	025 2253261
	DH – Madavachchiya	Medical ward	025 2245661
Badulla	PGH – Badulla	ETU/ ICU	0552222261 Ext.322
	STD Clinic		
	BH – Welimada	Ward 04 – medical ward	055 2222578
	BH – Diyathalawa	ETU	057 2245161, 057 2229061
	BH – Mahiyanganaya	ETU	
	STD clinic		055 4936722 055 4936779

District	Institution	Unit of location	Contact Number
Batticaloa	TH Batticaloa	STD clinic	
	ETU		0652057078 0652222261-2
Colombo	National Hospital of Sri Lanka	OPD room number 08	0112691111, Ext. 2417
	Lady Ridgeway Hospital	Indoor Dispensary	011 2693711-2, Ext. 219, 242
	De Soysa Maternity Hospital	Emergency theatre (OT2)	0112696224-5 Ext.326
	Castle Street Hospital for Women	Intensive care unit (ICU)	0112696231-2 Ext.2230
	National Eye hospital	Room 4 (OPD)	0112693911-5 Ext.231
	Sri Jayawardenapura Teaching Hospital	Indoor pharmacy	0112802695-6 Ext.3032
	TH- Kalubowila	Infection Control unit (From 7am-4pm)	
	OPD room number 20 (after 4pm)		
	STD clinic		0112763261 Ext. 129
			0112763261 Ext. 218 0114891055
		Infection control unit	
		Ward 03	011 2411284 Ext.264
			011 2411284 Ext. 210
	DGH Avissawella	PCU	036 222279
	BH - Homagama	PCU	011 2855200 Ext. 224
Puttalam	BH - Puttalam	PCU	0322265496
		STD clinic	0322265496 Ext 132
	DGH Chilaw	PCU	032 2223261
		STD clinic	032 2220750

District	Institution	Unit of location	Contact Number
Galle	TH Mahamodara	STD clinic	
	Indoor Drug Dispensary		0912245998
			0912222261, 0912234951
	TH Karapitiya	ETU	0912232267, 0912232176
	BH Balapitiya	STD clinic	
		ETU	0912 258 261 Ext.
	BH Elpitiya	ETU	0912 291 261 Ext.

Gampaha	TH Ragama	SICU	
	STD Clinic		011 2960224 Ext 258
			011 2960224
	DGH – Gampaha	PCU	
	STD Clinic		033 2222261 Ext 200 033 2234383
	DGH Negombo	MICU	
	STD clinic		031 2222261 Ext 439
			031 2222261 Ext 144 031 2239016
	BH – Wathupitiwala	ICU	033 2280261
	STD Clinic		033 2280261-2 Ext 255

Hambantota	DGH – Hambantota	PCU	
	STD Clinic		047 2222247
			047 2222247
	BH - Tangalle	STD clinic, ICU infection control unit	047 3144194
			047 2240261 Ext.220

District	Institution	Unit of location	Contact Number
	BH - Tissamaharama	Pharmacy	0472237261
	BH - Walasmulla	ETU	0472245261
Jaffna	TH Jaffna	ETU	021 2222261
	STD Clinic		021 2217756
Kalmunai	DH - Sainthamaruthu	STD Clinic	672223660
Kalutara	GH - Kalutara	PCU	0342222261, Ext. 250
	STD Clinic - Kalutara		034-2236937
	BH - Panadura	ETU	038 2222261 Ext. 4298
	BH- Horana	PCU	0342261261 Ext.1135
	Kethumathi Maternity Hospital	ward 2	038 2232361 Ext.
Kandy	TH- Kandy	ETU	
	STD Clinic		081 2233338, 081 2234208, 081 2203622
	BH - Gampola	ETU	081 2352261
	BH- Teldeniya	ETU	081-2374055
	BH - Nawalapitiya	ETU	
	STD Clinic		054 222226, 0542222261 Ext. 230
Kegalle	TH- Kegalle	ETU	
	STD Clinic		035 2222261, 035 2231222
	BH - Mawanella	ETU	035 2247835
	BH - Karawanella	ETU	036 2267374
	BH - Warakapola	ETU	035 2267261

District	Institution	Unit of location	Contact Number
Kilinochchi	BH -Kilinochchi	ETU	021 2285329
	STD clinic		021 2283709
Kurunegala	TH - Kurunegala	STD Clinic	037 2224339
		A&E, ICU	037 2233909
BH - Kuliyaipitiya		A&E	
	STD clinic		037 2281261
	BH - Nikawaratiya	ICU	037 3378060
	BH - Dambadeniya	PCU	037 2266592
	BH - Galgamuwa	PCU	037 2253061
Mannar	DGH - Mannar	STD clinic	
Matale	DGH - Matale	STD clinic	066 2053746
Matara	DGH - Matara	ETU	
	STD Clinic		041 2222261 Ext.161 041 2232302
	BH - Deniyaya	ETU	0412273261
	BH - Kamburupitiya	ETU	0412292261
	Asiri Hospital Matara	ETU	0768728906
Monaragala	DGH Monaragala	Primary care unit	
	STD Clinic		055 2277024, 055 2276826
	BH Bibila	PCU	055 2265461 Ext 135
	BH Wellawaya	PCU	055 2274861 Ext 159
	BH Siyambalanduwa	PCU	055 2279460 Ext 109
Mullaitivu	DGH Mullaitivu	STD clinic	021 2061412

District	Institution	Unit of location	Contact Number
Nuwara Eliya	GH Nuwara Eliya	OPD	
		STD Clinic	052 2234393
			052 2223210
	BH Dickoya	OPD	051 2222261
	BH Rikillagaskada	OPD	081 2365261
Polonnaruwa	GH- Polonnaruwa	ETU	
	STD Clinic		027 2222384, 027 2225787
Ratnapura	PGH - Ratnapura	ICU	
	STD Clinic		0452225396, Ext.225, 337 045 2226561
	BH - Balangoda	Ward 02 (Medical ward)	045 2287261 Ext 273
	BH - Embilipitiya	ICU	
	STD Clinic		047 2230261 Ext 244
	GH Trincomalee	ETU	047 2230261 Ext 192
	STD Clinic		026 2222261
			026 2222563
Trincomalee	BH Kanthale	ICU, Wards	026 2234261
	BH Pulmoddai	Wards	026 2256161
	BH Muthur	Wards	026 2238261
	BH Gomarangadawala	Wards	026 3261073
	BH Kinniya	Wards	026 2236261
	DH Padavi Sripura	Wards	025 2255261
Vavuniya	DGH Vavuniya	ETU	
	STD Clinic		024 2222761, 024 2224575

CHAPTER 09

PREVENTION INTERVENTIONS FOR KEY POPULATION GROUPS AND VIRAL HEPATITIS B AND C

Sathya Herath¹

The key populations (known as KP) at high risk of contracting HIV are, Men who have sex with men (MSM), female sex workers (FSW), and people who use drugs specially focusing people who inject drugs (PWID), Transgender (TG) beach Boys (BB) and people in closed settings like prison inmates.

According to the strategic directions under the prevention intervention, prioritization was given to scale up Pre-exposure Prophylaxis (PrEP) for MSM and TG components and harm reduction services among PWID. Increasing testing among KPs using different approaches like the adoption of community-led testing, community clinics, and scaling up of self-testes were given priority too.

Special interventions launched in 2022

Year 2022 was a remarkable year as the KP intervention introduced and scaled up high impact interventions. The following interventions were scaled up in 2022.

1. Community Led Testing was launched on May 19, 2022. Community Led

Testing was launched in Heart-to – Heart (H2H) initially and in Strategic Alliance for Research and Development (SARD), National Transgender Network (NTN) & Community Strength Development Foundation (CSDF) subsequently in the same month.

2. Community PrEP clinics were launched in Colombo & Gampaha in 2022. Community PrEP clinic was launched on June 19, 2022 in the Heart-to-Heart organization, Colombo & on the August 26, 2022 in SARD organization.
3. The Needle Syringe Exchange Services were scaled up from January 2022 since it was launched on December 07, 2021.
4. Community Based Direct Observed treatment (DOT) Services for Hepatitis C infection was launched on June 09, 2022.
5. Regular monthly reviews with STD clinics, KP units & NGO/CBOs initiated since November 2021 and conducted each month during 2022.

¹Consultant Community Physician

6. Introducing innovative approaches “Virtual programs” to the NGO/CBO supported KP interventions. Target for virtual program reach and virtual mode of testing were integrated to the KP interventions. For the first time virtual outreach workers were recruited through the NGO/CBO

Role of Key Population Unit at the National STD/AIDS Control Program

- Technical guidance to implement the prevention interventions for the Key Population for HIV/STI and Viral Hepatitis at the National level, district level, field level, and for non-governmental organizations and Community-Based Organizations.
- Capacity building of the Central level Health Care staff and other field staff working with the Key Populations. Conducting refresher staff training and other training related to Key Populations programs.
- Support the National level and District level Advocacy programs and Multisectoral collaboration with relevant stakeholders (provincial and district level administrators, Police and Prison Officials, Community, and religious leaders) to reduce barriers to services and create a supportive enabling environment for Key Populations and their service providers.
- Engaged in community-based activities conducting community-based one-stop clinics for Pre-Exposure Prophylaxis clinics for MSM and Community-based Hepatitis C treatment clinics for PWID

- Monitoring and Evaluation of Key Population prevention activities by monthly progress reviews, district reviews monthly and National reviews of Key Population activities, and monitoring the target achievements by Annual and Mid-term reviews with the support of external consultancies.

Key Population Unit Services

Services at the District clinic level

There are district clinics to facilitate the KP engagement activities in the district and the Unit is headed by the district Consultant or the Medical officer in charge. The staff consists of a management assistant, an outreach worker per KP component, and 3 or more peer educators per component.

Services provided

- Health education, promotion, and communication on safe sexual practice, safe injection practice, and harm reduction services during drug use
- Condom and lubricant promotion and distribution and supply of needles for People Who Inject Drugs in a Needle Syringe Exchange Services (NSES) program.
- HIV testing, counseling, and linking for services (Community-based testing, Community Led Testing, and HIV self-testing)
- Referring clients for HIV/STI services: STI screening, treatment, PrEP, PEPSE, Hepatitis B vaccination, Community-based direct observed HCV treatment for PWIDs, and giving support for

treatment adherence and psychological support

- Applying innovative approaches in providing services, and reaching clients through online applications and mobile applications.

Community-Based activities

Community-based programs- Safe needle program and provide needles for PWID to prevent needle sharing among people who inject drugs and counseling done for harm reduction and treatment adherence at the community level. HIV self-testing promotion and Condom promotion and distribution are done at the field level with the support of outreach workers and peer educators.

Conducting Direct Observed Treatment Services for PWID (HCV clinic) - This was started in June 2022 for the PWID and screening for HBV and HCV done and treatment starting for HCV positive patients. Treatment adherence is supervised and assessed weekly in community based and outreach clinics. Counseling services are also provided.

PrEP clinics- This was launched in year 2022 mainly for the MSM and TG components. People are referred to clinics with the support of outreach workers and currently, Colombo and Gampaha districts PrEP clinics are conducted on weekends with the support of NGO/CBO. Screening for HIV and other STDs is done and same day PrEP initiation is done and follow-ups are done on a routine base.

In addition to these NGO/CBO supported KP interventions, routine passive integrated

Health care worker model reach KP in the clinics and field. Education, provision of condoms and lubricant, HIV/STI testing services are routinely provided.

Monitoring and Evaluation Activities

- KP interventions within the districts are directly monitored by the district clinics
- Monthly reviews are conducted by the KP/Unit/NSACP in collaboration with district clinics and NGO/CBO
- Indicators are monitored by the SIM unit quarterly and reported annually to National and International reporting entities.

Strengths

1. Efficient and supportive staff to conduct community-based activities and clinics
2. Dedicated Outreach and peer educator staff with a well-coordinated system with the government and NGO/CBO
3. Establishment of Community-based PWID clinics and PrEP clinics
4. Increase the coverage of services for the KP HIV and identify more HIV/STIs and Hepatitis C

Challenges

1. Establish tailored approaches to reach KP groups
2. Optimizing the Coordination with NGO/CBO KP and health staff
3. Referral Care pathway delays at the

diagnosis and initiation of treatment for HIV/STI among KP

4. Optimizing the quality of services
5. Inadequate sample size estimations of Key population

Future Action Plan

1. Enhancing the KP intervention coverage by scaling up virtual interventions, community-led testing, and home delivery of health commodities.
2. Conduct more training programs and capacity building of health care staff and NGO/CBO staff
3. Scaling up monitoring and evaluation of KP interventions with regular reviews and supervisory visits.

4. Expanding Prevention Intervention Management system (PIMS) and proper coordination among service providers
5. Conducting Integrated Bio Behavioral Surveillance and Population Size estimation for Key Population redesign the program based on the survey results.

Programmatic performance

Programmatic performance is monitored by set indicators against agreed targets for each KP group.



Below table describe indicator progress data for each KP category

Programmatic Indicators monitored and achievement					
Active Indicator used in the Programmatic Rating Calculation					
	Active Indicator Name	Country	Target Value	Result Value	Percentage
1	KP-1a Percentage of men who have sex with men reached with HIV prevention programs - defined package of services	Sri Lanka	N: 10,800 D: 73,800 P: 14.8%	N: 11,444 D: 73,800 P: 15.5%	106.0%
2	KP-Other 4 Percentage of "beach boys" reached with virtual HIV prevention programs - defined package of services	Sri Lanka	N: 700 D: 4,500 P: 15.6%	N: 94 D: 4,500 P: 2.1%	13.0% **
3	KP-1c Percentage of sex workers reached with HIV prevention programs - defined package of services	Sri Lanka	N: 7,200 D: 30,000 P: 24.0%	N: 9,385 D: 30,000 P: 31.3%	129.0% *
4	KP-1d Percentage of people who inject drugs reached with HIV prevention programs - defined package of services	Sri Lanka	N: 800 D: 2,700 P: 29.6%	N: 924 D: 2,700 P: 34.2%	116.0%
5	KP-1e Percentage of other vulnerable populations reached with HIV prevention programs - defined package of services	Sri Lanka	N: 2,010 D: 4,500 P: 44.7%	N: 1,840 D: 4,500 P: 40.9%	91.0%
6	KP-1f Number of people in prisons and other closed settings reached with HIV prevention programs - defined package of services	Sri Lanka	N: 17,396 D: 18,909 P: 92.0%	N: 19,027 D: 18,909 P: 100.6%	109.0%
7	HTS-3a Percentage of men who have sex with men that have received an HIV test during the reporting period and know their results	Sri Lanka	N: 16,000 D: 73,800 P: 21.7%	N: 8,768 D: 73,800 P: 11.9%	55.0% **
8	HTS-3b Percentage of transgender people that have received an HIV test during the reporting period and know their results	Sri Lanka	N: 880 D: 2,200 P: 40.0%	N: 939 D: 2,200 P: 42.7%	107.0%
9	HTS-3c Percentage of sex workers that have received an HIV test during the reporting period and know their results	Sri Lanka	N: 9,600 D: 30,000 P: 32.0%	N: 8,037 D: 30,000 P: 26.8%	84.0%
10	HTS-3d Percentage of people who inject drugs that have received an HIV test during the reporting period and know their results	Sri Lanka	N: 720 D: 2,700 P: 26.7%	N: 779 D: 2,700 P: 28.5%	107.0%
11	HTS-3f Number of people in prisons or other closed settings that have received an HIV test during the reporting period and know their results	Sri Lanka	N: 15,657 D: 18,909 P: 82.8%	N: 4,982 D: 18,909 P: 26.3%	32.0% **
12	HTS-3e Percentage of other vulnerable populations that have received an HIV test during the reporting period and know their results	Sri Lanka	N: 2,160 D: 4,500 P: 48.0%	N: 1,870 D: 4,500 P: 41.6%	87.0%
13	TCS-1.1 Percentage of people on ART among all people living with HIV at the end of the reporting period	Sri Lanka	N: 2,702 D: 3,663 P: 73.8%	N: 2,811 D: 4,111 P: 68.4%	93.0%
14	KP-Other 1 Percentage of men who have sex with men reached with virtual HIV prevention programs - defined package of services	Sri Lanka	N: 12,000 D: 73,800 P: 16.3%	N: 3,074 D: 73,800 P: 4.2%	26.0% **
15	KP-Other 2 Percentage of transgender people reached with virtual HIV prevention programs - defined package of services	Sri Lanka	N: 500 D: 2,200 P: 22.7%	N: 1,241 D: 2,200 P: 56.4%	120.0% *
16	KP-Other 3 Percentage of sex workers reached with virtual HIV prevention programs - defined package of services	Sri Lanka	N: 6,000 D: 30,000 P: 20.0%	N: 2,263 D: 30,000 P: 7.5%	38.0% **
17	KP-1b Percentage of transgender people reached with HIV prevention programs - defined package of services	Sri Lanka	N: 690 D: 2,200 P: 31.4%	N: 667 D: 2,200 P: 30.3%	96.0%

*Individual indicators should have a maximum score of 120%, when calculating the mean

**If an indicator is rated less than 60% then Final Quantitative Rating is downgraded to one rating unless indicator rating is C, D or E

Programmatic Indicators Progress on the activities implemented for the People who Inject Drugs (One stop service model) d and achievement

Hepatitis C treatment details	Number & Percent
Screened for using HCV rapid diagnostics tests	397
HCV Ab reactive	159 (40%)
HCV PCR tests done	119 (80%)
HCV confirmed	95 (74%)
Treatment Started	70 (74%)
Treatment completed	47 (65%)
Death	1 (1%)

Hepatitis C treatment details	Number & Percent
Hepatitis B detected through HBsAg	397
Hepatitis B treatment initiated	00
People gives needle safe boxes	153
People initiated on Needle Syring Exchange Services (NSES)	156
People on PrEP	00

Community PrEP program

Community PrEP District	Total PrEP attendees	PrEP initiated	HIV Diagnosed	STI Diagnosed
2022/Colombo	145	122	5	12
2022/Gampaha	29	26	3	7

CHAPTER | 10

Laboratory Services 2022



CHAPTER 10

LABORATORY SERVICES 2022

Jayanthi Elwitigala¹

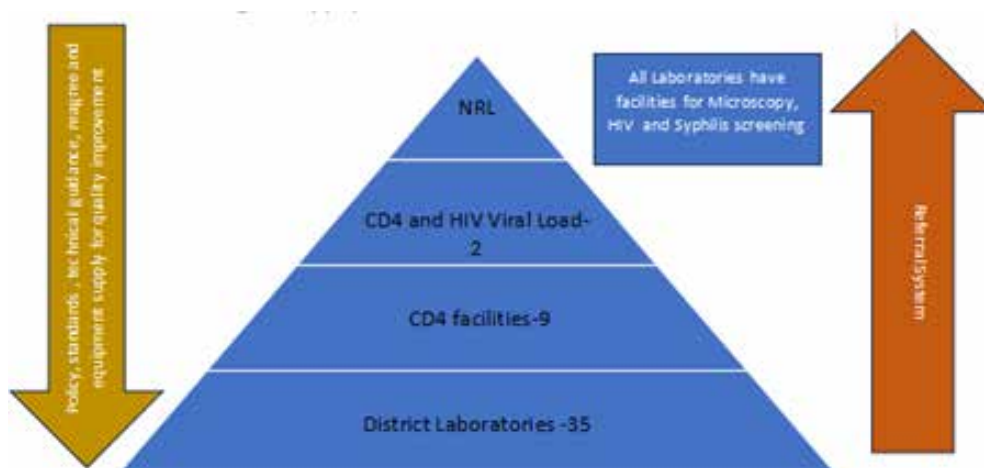
The laboratory services for HIV & STI spreads throughout the country covering all districts. There are 41 clinics which are capable of carrying out testing in varying degrees for STI & HIV through their laboratories and work in a network fashion. The laboratory network for STI & HIV in the country comprises of a National Reference Laboratory (NRL) and district clinic laboratories.

NRL serves as the apex body for the laboratory network and plays a crucial role in coordinating and standardizing laboratory testing and diagnostics for STIs and HIV, ensuring quality assured reporting from the laboratories in the network.

District STI/HIV Laboratories play an important role in the nationwide effort

to prevent sexually transmitted infections (STIs) and HIV. Each district is equipped with one or more laboratories to ensure easy access for testing services for the population. Anuradhapura & Galle are provided with HIV viral load testing facilities and all 9 provinces are provided with additional CD4 testing facilities for monitoring of HIV patients.

The district laboratories perform microscopy and routine testing for HIV & Syphilis for the clinic attendees and refer the specimens to the National Reference Laboratory (NRL) for further testing. All community testing programs for HIV and other STI's are supported by these district laboratories, ensuring widespread testing and increased detection of the infected people. Moreover, the district laboratories



¹Consultant Microbiologist

distribute HIV Rapid Diagnostic Test (RDT) kits to health institutions and community organizations within their respective districts. This distribution ensures that testing resources are available where they are needed the most, and local healthcare providers and organizations can contribute to the testing efforts.

As a whole year 2022 was disturbing to laboratory services due to social and political unrest of the country

Quality Assurance of laboratory service :

The laboratory system is monitored for quality assurance with a quality management system. Standards of laboratory services are maintained in the district laboratories by regular training and provision of technical guidance by NRL. The newly recruited staff of the laboratories undergo an initial training in NRL for testing.

The testing of all laboratories is subjected to quality with internal quality control and external quality assurance to ensure the accuracy and reliability of laboratory tests.

A NEQAs (national external quality assurance) is conducted by the NRL of NSACP to all the district laboratories

with proficiency testing for syphilis and HIV testing. Data management of the laboratories is technically guided by the NRL and supported by the EIMS.

The NRL collaborate with other national and international laboratories, organizations, and research institutions to exchange knowledge, share best practices, and participate in international EQAs for HIV & STIs.

Diagnosing HIV and STI

The peripheral laboratories across the island offer essential diagnostic services to address STIs and HIV effectively.

STI Screening:

The laboratories perform microscopy services to screen for sexually transmitted infections (STIs) syphilis, gonorrhoea, genital herpes, trichomoniasis, candidiasis and for bacterial vaginosis. The serological services are available for syphilis in all laboratories and for herpes in NRL. There is molecular testing available for chlamydia and N.gonorrhoea in NRL. Some of these laboratories are equipped with culture techniques to test for *Neisseria gonorrhoeae*.





HIV Testing:

For HIV testing, the ELISA technique and HIV rapid tests are employed in peripheral laboratories. ELISA is a laboratory based method and RDT is a point of care test. The confirmatory test for HIV is performed exclusively at the National Reference Laboratory (NRL) of NSACP. This central facility covers both public and private sectors, ensuring accurate and reliable HIV diagnosis. The NRL also extends its support to the National Blood Transfusion Service and private sector laboratories by providing free HIV confirmatory services.

Molecular Testing:

The National Reference Laboratory (NRL) is equipped with advanced molecular facilities, allowing it to perform testing for HSV (Herpes Simplex Virus) Gonorrhoea and Chlamydia. Molecular techniques are highly sensitive and specific for detecting these infections. GeneXpert viral load testing facilities are available at NRL, Galle, and Anuradhapura STD clinic labs. This advanced technology enables efficient monitoring of HIV viral load, guiding treatment decisions

for patients. The NRL possesses the capability to conduct HIV DNA PCR testing for infant diagnosis. This test is crucial for early detection of HIV in infants born to HIV-positive mothers. Further to this NRL has established testing for drug resistance in HIV with gene sequencing facility.

Establishment of drug resistance testing at the NRL

With the support of the Global Fund to Fight AIDS, Tuberculosis, and Malaria (GFATM) in 2021, the DR facility was established in NRL/NSACP. This facility plays a critical role in monitoring and managing drug resistance in HIV/AIDS and other infectious diseases.

Having a drug resistance testing facility at the NRL is a significant step in enhancing the country's capabilities in HIV/AIDS management and control. Drug resistance testing helps identify individuals who may require alternative treatment options due to resistance to standard drugs. This, in turn, enables healthcare providers to make more informed decisions about patient care and contributes to better disease management and prevention strategies.

CD4 Testing:

CD4 testing which is essential for monitoring the immune status of individuals living with HIV, is available at the NRL and eight other provincial laboratories.

Hepatitis B and Hepatitis C Testing:

In addition to STI and HIV testing, the NRL also performs testing for Hepatitis B and Hepatitis C including viral load test for hepatitis C. This is a significant improvement taken towards testing as these viral infections can have serious health implications.

Biochemical and Haematological Testing:

The NRL provides biochemical and haematological testing facilities for people living with HIV, ensuring comprehensive health monitoring and care.

Overall, this network of laboratories, ranging from peripheral facilities to the NRL, forms a robust and coordinated system to combat STIs and HIV effectively, supporting both public and private healthcare sectors.

Table 10.1: Island wide testing for STI -2022 (source - lab statistics- monthly return) Total Microscopy tests done in 2021 and 2022

Clinic	2021	2022
NRL	17279	22719
Peripheral	26227	34789
TOTAL	43506	57508

	Number done	No. positive	Positivity rate
Dark ground microscopy	335	6	1.8%
VDRL(Blood)	259303	5905	2.3%
VDRL(CSF)	244	3	1.2%
TPPA	30005	1612	5.3%
HIV ELISA	240797	972	0.4%

Table 10.2: NRL workload (% from total testing) - 2022

Tests	No. of tests done			NRL Workload (%)
	NRL	Peripheral	Total	
VDRL(Blood)	55,222	204,081	259,303	21.3 %
VDRL(CSF)	24	220	244	9.8 %
TPPA	9,841	20,164	30,005	32.8 %
HIV ELISA	63,575	177,222	240,797	26.4 %

Table 10.3: GC culture – 2021 and 2022

	NRL			Peripheral			Total		
	No. test done	No. Positive	Positivity rate%	No. test done	No. Positive	Positivity rate%	No. test done	No. Positive	Positivity rate%
2021	2571	36	1.4%	1025	10	1%	3596	46	1.3%
2022	5321	117	2.2%	1313	32	2.4%	6634	149	2.2%

Table 10.4: Monitoring of HIV 2021- 2022

Test	2021	2022
Viral load	2706	1695
CD4	3206	1767

Provision of Test Kits and Reagents to peripheral laboratories

The country has a uniformity in testing for HIV and STI. To ensure that all the laboratories use high quality test kits and reagents the, NSACP is procuring the supplies for syphilis and HIV serological diagnosis through a central procuring system and the NRL distributes it to the individual laboratories depending on their requirement.

Challenges for laboratory system

The challenges faced by the laboratory system, both long-term and short-term, can significantly impact the quality and effectiveness of healthcare services, particularly in the detection and management of HIV and other infectious diseases. Some of the specific challenges mentioned include:

Inadequate Space:

Limited space at the National Reference Laboratory (NRL) and certain district clinic laboratories poses a significant challenge in

scaling up and improving the quantity and quality of laboratory services. Insufficient space can limit the installation of advanced equipment, affect workflow, and potentially compromise biosafety measures.

Shortage of Medical Laboratory Technologists (MLTs):

The critical shortage of MLTs in certain district clinics severely impact the delivery of laboratory services. Without trained and qualified MLTs, the testing process may be delayed or compromised, leading to potential adverse effects on patient care.

Outdated Equipment:

The difficulty in obtaining machinery for testing is a concern in improving the laboratories. Automation and the use of up-to-date equipment are essential for handling high volumes of testing in an efficient manner as well as ensuring accurate and timely results. Nonfunctioning ELISA machines creates many issues in HIV testing if not replaced appropriately.

Addressing these challenges is crucial for strengthening the laboratory system's capabilities in detecting HIV and other infectious diseases in the country in order to prevent the diseases.

Potential solutions may include:

- Implementing capacity-building programs to train and recruit more MLTs, particularly in clinics that are facing a shortage.
- Upgrading laboratory infrastructure, including the provision of sufficient space

and modern equipment, to enhance testing capacity and maintain high-quality services.

- Advocating for increased funding and resources to support the expansion and improvement of laboratory services.

By fulfilling these gaps and addressing the challenges faced by the laboratory system, the country can improve its ability to detect HIV and other infectious diseases promptly and accurately. This, in turn, will contribute to better disease management, treatment outcomes, and public health overall.



CHAPTER 11

CONDOM PROMOTION

Dr Janaka Weragoda¹
Dr Thilani Ratnayake²

Correct and consistent condom use is a highly effective strategy to reduce the risk of acquisition and transmission of HIV and STIs. Also, a proven cost effective method of HIV/STI prevention with minimum side effects. Therefore, National program gives priority for condom promotion and included in the National Strategic Plan.

In Sri Lanka condoms are promoted and distributed by several different agencies including health department, community-based organizations (CBOs), non-governmental organizations (NGOs) and private organizations. NSACP plays a major role in distributing condoms among risk groups as a method of HIV/STIs prevention.

NSACP promotes condoms through its network of STD clinics and peer-led targeted intervention programmes which is conducted through district STD clinics and family planning association (FPA) among key populations. Peer-led targeted programmes help to distribute free condoms in diverse venues among key populations. Further, condoms are distributed online as part of the self-test kits which can be ordered through KNOW4SURE app.

NSACP has several responsibilities over condom promotion.

1 Evaluation of condoms and lubricants

The evaluation of condoms and lubricants prior to approval by National Medicines Regulatory Authority (NMRA) is carried out by the NSACP. An evaluation report is forwarded to NMRA stating the suitability or if not reason for rejecting the product.

2 Distribution of condom dispenser

The condom dispensers were distributed island-wide through peripheral STD clinics, 500 condom dispensers in 2019 and 44 more condom dispensers in 2020 were distributed as a measure to increase the availability and the accessibility of free condoms and lubricants to the public. It further promotes safe sex as well as a social venture to remove the taboo around purchasing condoms.

The main locations identified by the clinics were STD clinics, hospitals, police stations, supermarkets, filling stations, construction sites, hotels, SPAs, economic centres and MOH offices.

¹Consultant Community Physician

²Consultant Venereologist

In 2022 another forward step was made by getting the permission from the railway department to install condom dispensers at railway stations with the objective to make condoms available for those who take risks at casual sex during long distance travel.

3 Condom promotion through different media campaign

The National STD/AIDS Control Programme is promoting the importance of using condoms through targeted social media campaigns to most at risk populations also through mass media to reach the message to general public .

In 2022 two new video clips were developed to promote condoms and disseminated to relevant NGOs and peripheral STD clinics.

There was a request from high risk population groups for flavoured condoms and NSACP considered this request and ordered flavoured condoms for these groups in order to increase the condom use among these high risk, vulnerable and young population.

Condoms are considered as a medical device that prevent infections and included in the National Formulary list of the ministry of health. NSACP was able to convince the ministry to add lubricants to the National Formulary in 2022 which is another successful achievement. for effective use of condoms for prevention of HIV and STIs.

The Global fund and Ministry of health are the main condom suppliers for the NSACP. Condoms provided by global fund are used for key population targeted HIV prevention activities and condoms provided by Ministry of health are used for HIV prevention

activates for both key population groups as well as other people with high-risk behaviors.

NSACP promotes condoms all over the country through its network of STD clinics and peer-led targeted intervention programmes which is conducted through district STD clinics and via family planning association (FPA) among key populations with the help of many CBO/NGO. Peer-led targeted programmes help to distribute free condoms in diverse venues among key populations.

In addition to condoms the NSACP distribute lubricants mainly for groups with MSM behaviors and also for female sex workers. During the year 2022 there were many interruptions of distribution of condoms due to shortage of supply with economic crisis of the country.

Total number of condoms distributed in the year 2022 was 1,642,634 and this amount is little higher than the total number of condom distributed (1,436,679) in 2021.

There is a national target for the minimum distribution of condoms among key populations

FSW – 30 condoms per month

MSM – 15condoms per month

TG- 15 condoms per month

DU- 5 condoms per month

To prepare the annual condom estimates for the NSACP as well as for to prepare the distribution plan for each district, these norms are taken in to considerations.

The annual condom requirement for HIV prevention activities conducted by NSACP is around 13 million.

Table 11.1: Condoms and Lubricants received by NSACP from difference sources from 2017-2022

Condom				Lubricant	
Year	GoSL	GFATM	Total	GoSL	GFATM
2017	71,424	4,052,304	4,123,728	Nil	1,773,400
2018	200,000	2,289,600	2,489,600	Nil	1,775,000
2019	Nil	Nil	-----	Nil	NIL
2020	Nil	Nil	-----	Nil	80,000
2021	104,500	1,668,960	1,773,460	Nil	648,000
2022	Nil	770,400	770,400	Nil	702,000

Table 11.2: Condom and Lubricants distribution by STD clinics in 2022

	STD clinic	condoms	lubricants
1	Ampara	32072	3375
2	Balapitiya	30816	1800
3	Anuradhapura	55779	3620
4	Avissawella	1940	195
5	Badulla	26816	135
6	Batticaloa	4060	1700
7	Chilaw	40997	17460
8	Colombo	54,109	19,817
9	Dambulla	5,180	170
10	Embilipitiya	420	370
11	Gampaha	7,713	660
12	Hambantota	59,333	19,075
13	Homagama	544	0
14	Jaffna	44,549	10,249
15	Kalmunai	11,919	230
16	Kalubowila	28,297	5,960

	STD clinic	condoms	lubricants
17	Kalutara	41,304	5,970
18	Kandy	3,468	10
19	Kegalle	29,424	5,000
20	Kilinochchi	3,300	1,500
21	Kuliyapitiya	4,764	594
22	Kurunagala	69,977	21,819
23	Mahamodara	120,921	15,190
24	Mahiyangana	18,530	800
25	Mannar	1,550	2,000
26	Matale	49,333	1,180
27	Matara	46,807	4,656
28	Monaragala	7,512	595
29	Mullativu	1,200	700
30	Nawalapitiya	3,824	0
31	Negombo	21,096	3,632
32	Nuwara_Eliya	16,700	0
33	Panandura	1,783	625
34	Polonnaruwa	60,264	80
35	Puttalam	27,190	5,940
36	Ragama	6,427	690
37	Rathnapura	16,735	1,400
38	Tangalle	189	24
39	Trincomalee	1,529	300
40	Vavuniya	7,344	400
41	Wathupitiwala	7,803	425
	Total	973,518	158,346

Table 11.3: Condom and Lubricants distribution by SR (FPA) over the years

Year	Condoms	Lubricant
2018	3,654,030	Nil
2019	334,631	73741
2020	734,976	81260
2021	725,193	161,831
2022	669,116	194,459

Table 11.4: Condom and Lubricants distribution by SR (FPA) according to KP groups in 2022

KP	Condoms	Lubricant
MSM	161,127	133,448
TG	50,717	33,116
FSW	415,625	27,895
PWID	41,647	-----
	669,116	194,459

CHAPTER | 12

Multi-sectoral Collaboration



CHAPTER 12

MULTISECTORAL COLLABORATION

| Janaka Weragoda¹

The Multisectoral Unit (MSU) of National STD/AIDS Control Programme (NSACP) plans activities in liaising the National HIV/STI Strategic Plan Sri Lanka and focuses on interventions directed towards prison sector (key population), vulnerable groups and general population including young people (15-29 years).

The MSU of the NSACP is responsible for planning, monitoring, evaluation, and implementation of intervention strategies; provision of technical support for advocacy, risk communication, capacity building, awareness and training on behavior change, promotion of safe sex & enabling a conducive environment for prevention of HIV/STI and internalization of HIV/STI related activities in multi sectoral institutions.

The MSU coordinates and works in partnership with the public, private, and civil society organizations for enabling a conducive environment for the prevention of HIV/STIs in Sri Lanka.

Staff of multisectoral unit of NSACP

The MSU is headed by a Consultant Community Physician and the other staff

consists of medical officers, an assistant coordinator, a development officer (vacant now) and other supportive staff.

Multisectoral subcommittee

The subcommittee on “Prevention & Multisectoral coordination” also known as multisectoral subcommittee. This is a subcommittee under National AIDS committee. This subcommittee meets once a year. The objective of the subcommittee is to review activities carried out in the previous year, to plan HIV activities for the next year and to provide recommendations to National AIDS Committee.

All the institutions/organizations work with MSU are stakeholders of this subcommittee; the department of prison, tri forces, Sri Lanka police, education sector, National youth service council, National youth corps, National child protection authority, department of education, Sri Lanka bureau of foreign employment (SLBFE), tourist sector, vocational training authority, and many other directorates of the Ministry of health such as, family health bureau, health promotion bureau, directorate of mental health etc. In addition international agencies (UNFPA, WHO), local NGOs such as family planning

¹Consultant Community Physician

association of Sri Lanka (FPA), community-based organizations work for the welfare of PLHIV and for other key population groups are also stakeholders for this subcommittee.

For the year 2022 the subcommittee meeting was held in the month of September with the “country dialogue” for the development of NSP (2023-2030). Almost all stakeholders participated for the meeting and shared their concerns and views about the multisectoral approach for the prevention of HIV and STIs in Sri Lanka.

Prison sector HIV and STI prevention programme

Closed settings such as prisons are globally identified as high risks settings of HIV. The national HIV/STI strategic plan of NSACP has identified prison inmates as a group of key population in for HIV in Sri Lanka.

There are 30 prisons functioning all over the country. According to the prison statistics in 2021 on average around 25,000 prison inmates were occupying the prisons at any given time, and among them, around 1000 were female inmates. Annual total admissions to all prisons range from 100,000 to 125,000 and out of them the number of females ranges from 5500-6500.

The NSACP in collaboration with department of prisons developed a policy for prison sector known as “Prison HIV prevention, treatment and care Policy”. The purpose of this policy is to provide a framework for addressing HIV prevention and providing related health services in the prison sector, equivalent to the services available in the general community, and consistent with the national HIV/AIDS policy and guidelines.

In the year 2022, MSU conducted several activities in prisons in collaboration with the department of prison to minimize the HIV and STI burden within the prisons;

Advocacy for prison authorities, training of trainer programmes for prison officers on behavior change communication for HIV prevention and promotion of safe sexual practices, training of peer leaders in all prisons, life skills-based development activities for young offenders, are the main activities to prevent HIV/AIDS and STIs among the prison inmates. In addition to that many health promotion activities for prison inmates such as drama competitions, art competitions were conducted. These interventions were based on a communication strategy developed for the prison sector. The Global Fund provided financial support for the activities conducted prison programmes.

1.1. Prison Steering Committee

Prison Steering Committee for HIV prevention gathered quarterly. The objective of the prison steering committee is to review the HIV prevention activities carried out in prisons, identify barriers for HIV prevention activities in prisons, find solutions to overcome barriers and plan future activities.

In the year 2022, two steering committee meetings were held at Prison headquarters, Welikada with the participation of relevant stakeholders from NSACP, prison department and prison health authorities. Special attention was made to carry out HIV prevention activities in prisons such as training programmes, health promotion activities, HIV testing for prison inmates, management of HIV-infected prison inmates

while they are in prison. An important decision taken in the second steering committee meeting in the year 2022 was to internalize HIV testing in prison using trained prison staff and reaching high HIV testing targets.

1.2 Training of Trainer (TOT) programme

In the year 2022, NSACP conducted three TOT programmes as three-day training workshops at the prison training school in Welikada and trained fifty-eight (58) uniform and non-uniform officers as trainers representing all prisons island wide.

1.3 Training of peer leaders

The peer leaders selected from prison inmates are trained by prison welfare officers who have undergone training as master trainers for HIV and STI prevention. In the year 2022, thirty peer leader programmes were conducted in all 30 prisons in the country and 600 prison inmates as peer leaders were trained.

The trained peer leaders provide health education/ health messages on prevention of HIV/STI and promotion of sexual practices to other inmates through formal and informal ways. Number of peers educated by peer leaders during the year 2022 was around 19,000 in all prisons in the country. All peer leaders were given a peer leader badge after their training as a recognition of the service they provide to their peers to improve awareness on HIV and STIs in prisons.

1.4 Training programmes on Life Skills for young offenders

In the year 2022, four training programmes

were conducted in Wataraka, Ambepussa, Weerawila and Magazine prisons for young prison inmates aiming to improve their life skills in achieving goals of their life and to enhance knowledge on HIV/STI and prevention measures aiming promotion of safe sexual practices. One hundred and fifteen (115) young offenders participated for programmes. Resource persons from NSACP as well as from directorate of mental health also contributed to the training programmes conducted.

1.5 HIV testing in prison sector

NSACP along with district STD clinics provide HIV/STI testing services for prison inmates according to the HIV testing guidelines for the prison setup. The Multisectoral Unit promotes HIV testing in prison through regular health education programmes conducted for new inmates and through peer leaders by formal and informal discussions with inmates.

In the year 2022, number of HIV testing has dropped compare to 2021 due to the economic crisis and shortage of HIV test kits the country. In year 2022, only 4430 HIV tests were performed and that was almost 25% of the annual HIV testing targets in prison. Only three inmates found as HIV positive in year 2022.

In the year 2021, NSACP trained total of thirty-four (34) prison staff from Welikada, Kalutara, Kuruwita, Mahara, and Negombo to perform HIV testing for inmates and to reach high testing coverage.

As a task shifting activity of HIV testing, it was planned to internalize HIV testing in all prisons in the country in the year 2022 by training the prison staff to perform HIV

testing for inmates and thereby to reach high HIV testing targets. The prison authority granted the approval for this activity. However, due to the country situation in year 2022 and shortage of HIV test kits this activity was not implement.

1.6 Awareness programme for prison inmates

One day awareness campaigns on Behavior change communication and prevention of HIV/STI and safe sexual practices were conducted in twelve prisons by district STD clinics and more than 600 inmates participated.

1.8 Health promotion activities in prison

A poster art competition was conducted among all prisons island-wide parallel to the commemoration of World AIDS Day 2022. More than 300 posters were submitted representing all prisons for the competition. Winners were awarded certificates and gifts at the national World AIDS day commemoration ceremony.

1 HIV and STI prevention programme for Tri-Forces

Tri-forces; Sri Lanka Army, Sri Lanka Navy and Sri Lanka Air-force have been identified as of the vulnerable groups for HIV infection. HIV prevention activities for Tri-Forces have been identified as one of the important areas by NSACP considering their vulnerability for HIV. Many advocacy and training programmes were planned with the objective of internalizing preventive activities such as continuous training and testing for HIV in Tri-forces. However, the planned training programmes could not

be continued during the year 2022 due to situation in the country. However, already trained officers as master trainers were able to continue educational activities. 46,975 army personnel, 20,106 naval officers, and 1633 air force officers were tested for HIV during the year 2021.

2 HIV and STI prevention programme in the Police sector

The NSACP has identified police officers as a important population for HIV prevention, mainly because they are the first contact law enforcing officers when there is legal issue for the public. Creating a conducive environment for key populations to access preventive services is an essential element of all HIV prevention programmes. Stigma, discrimination and legal barriers can act as deterrents of this process.

Sri Lanka has certain laws that criminalize key population behaviors and these are often interpreted and enforced negatively on key populations due to inadequate understanding on HIV and related issues, LGBTQ community, people living with HIV (PLHIV) and laws related to KP groups. Therefore, these key populations groups as well as PLHIV and their families can be subjected to stigma and discrimination when police officers are implementing the law and order. Sensitizing the police officers on these legal issues and making them partners in the public health response towards HIV prevention is a priority in Sri Lanka.

In this background with the objective of reducing human rights-related barriers for HIV services, reduction of stigma and discrimination and creating supportive environment for HIV prevention activities in the country educational /advocacy

programmes for police officers were conducted in 11 districts in collaboration with district STD clinics (Galle, Matara, Hambanthota, Kegalle, Kurunegala, Kalutara, Monaragala, Mannar, Nuwaraeleiya, Polonnaruwa and Trincomalee). In addition to that 2 national level advocacy programmes were conducted for high-ranking police officers. Almost 600 police officer including high ranking officers participated for all these programmes.

3 HIV and STI prevention programmes for the youth

The MSU work closely with National Youth Services Council, National youth corps and many other youth organizations such as Youth Advocacy Network Sri Lanka (YAN Sri Lanka), Youth Action Network (YAN), Rotaract Club-IIT etc...

Youth corps and youth council officers collaborate with the district STD clinics and medical officers of health (MOH) for necessary support and guidance in delivering awareness programmes at the field level. In the year 2022, MSU conducted two training sessions for youth from National youth corps on behavior change for HIV/STI prevention, promotion and safe sexual practices. Forty five (45) youths participated for these

training sessions. Two TOT programmes on sexual and reproductive health including behavior change for HIV/STI prevention youths were conducted in collaboration with Family Health Bureau and around seventy (70) youth leaders participated reprinting representing youth organizations.

4 Education sector - HIV and STI prevention programme

The NSACP has identified the education sector as one of the important sectors for HIV prevention, especially with the gradual increase of new HIV diagnoses among 15 to 24-year-old individuals. The MSU continued to work on HIV prevention among school children in liaising with school health unit of Family Health Bureau and National Institute of Education.

5 HIV prevention and condom promotion programmes for key and vulnerable populations

One day workshops were conducted in Kalutara for 80 participants representing Vulnerable groups KP groups and young people on HIV/STI prevention. and participatory based skill development on proper use of condoms. The financial support for the programme was provided by UNFPA.



CHAPTER | 13

Training and Capacity Building



CHAPTER 13

TRAINING AND CAPACITY BUILDING

Geethani Samaraweera¹
Piyumi Perera²

The Training and Capacity Building Unit of the National STD/AIDS Control Programme of Sri Lanka plays a crucial role in the prevention and control of sexually transmitted diseases (STDs) and HIV in the country. The unit is responsible for designing and implementing comprehensive training programs for healthcare professionals, community workers, and other stakeholders involved in the management and control of STDs and HIV. By providing training and capacity building opportunities, the unit aims to enhance the knowledge, skills, and competencies of its target audience and empower them to deliver effective prevention and treatment services, thereby contributing to the overall goal of reducing the burden of STDs and HIV in Sri Lanka.

Historically this unit has been involved in training of undergraduate and postgraduate trainees in medicine and subspecialties, conducting preservice and Inservice/ refresher training for STD clinic staff and special training programs for NGO and CBO staff. During the recent past, going forward with the virtual platforms, webinar series, and some of the above-mentioned

trainings have been conducted as hybrid sessions, giving more access to participants island wide.

Preservice training

It is necessary for all healthcare workers at STD clinics to complete mandatory training within six months of enrolment. Medical officers assigned to the STD clinic must undergo two months of compulsory training, which includes both theoretical and practical components at NSACP in Colombo. Other important health personnel, including nursing officers, public health nursing sisters, matrons, medical laboratory technicians, pharmacists, public health laboratory technicians, dispensers, and public health inspectors, must complete a two-week training course that involves theory, practical work, case discussions, small group discussions, and outreach work. Supporting staff, such as attendants, Saukya Karya Sahayaka, and lab orderlies, must attend a one-week training program at NSACP. During the year 2022, two programmes were conducted virtually to provide preservice training for major staff. The first programme was conducted from 21.02.2022 to 05.03.2022 with the

¹ Consultant Venereologist

² Consultant Venereologist

participation of 98 participants and the second was conducted from 01.08.2022 to 13.08.2022 with 11 participants. The lectures were conducted virtually but the practical training was conducted onsite in their respective stations with the supervision of consultant venereologist.

Refresher training

Under refresher training, many programmes have been conducted with the funding support of the Global Fund. A special programme was conducted for supportive staff of STD clinics and government hospitals to reduce stigma and discrimination against key populations. The programme was conducted island wide covering all 19 districts. A case scenario based participatory training was conducted with many positive responses from the participants. A second refresher training was conducted to NGO and CBOs and other health staff categories aiming to improve their capacity in counselling and providing quality care for key populations. The programme mainly aimed at 15 districts where key population programmes are going on and it was carried out in 19 districts. It was a one-day training aiming to develop capacity of NGO, CBO in counselling services. It includes pre and post-test counselling risk reduction counselling and counselling for PrEP and PEP services. Participants were provided with basic knowledge on HIV STI and Counselling. Following that case scenario based practical skill building sessions were conducted.

In addition, 4 programmes have been conducted for NHSL two for nursing staff and two for supportive staff with each programme catered to 60 participants.

As Colombo has the highest population of PLHIV, the frequency of having multi-disciplinary involvement is high with the need of services from other specialties. Therefore, it is of utmost importance that the staff in the National Hospital of Sri Lanka are trained to minimize stigma and discrimination towards KPs as well as PLHIV. Hence, two such programs were conducted for both the major and minor staff members in this setting. A special Inservice programme was conducted to National dental institute following identification of some events of discrimination. The main aim of these programmes was to promote stigma free service delivery for PLHIV and key populations.

Strategic information management unit provided Inservice training programmes to introduce EIMS to

In addition to the NSACP all district STD clinics conduct regular Inservice programmes for health and non-health staff

Undergraduate training.

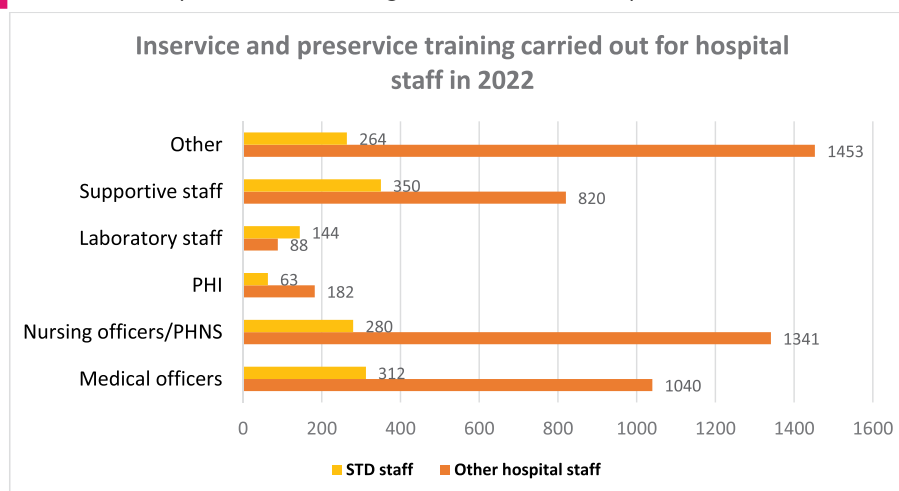
Training of medical students are being conducted as 1-to-2-week clinical rotations in the central STD/HIV clinic Colombo as well as in district STD clinics which are affiliated with medical faculties island wide. During the year 2022, a total of 1880 medical students have been trained on basics of STD/HIV care including the importance of having a non-discriminatory attitude when managing KPs and PLHIV. In contrast to the previous year, the training involved in-person attendance and included rotations in both the STD clinics and STD laboratories.

Post graduate training.

For the past two decades, the Training and Capacity Unit has been engaged in a significant endeavour of providing postgraduate training for various specialties including Venereology in Sri Lanka. This has been conducted with the collaboration of the Post Graduate Institute of Medicine,

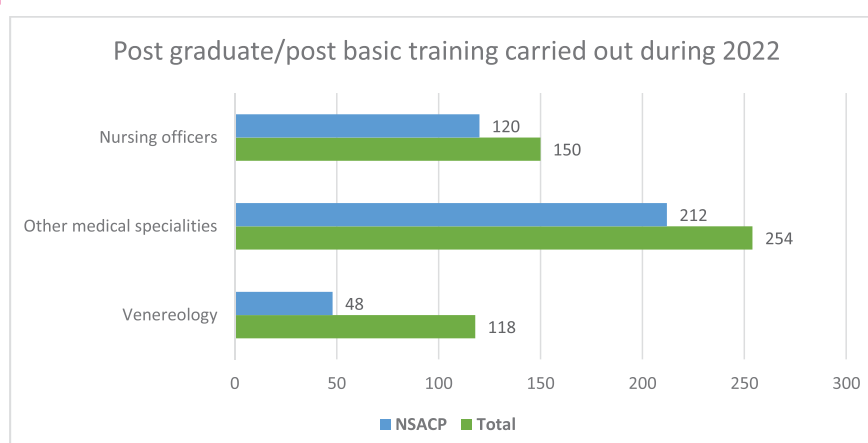
University of Colombo. A total of 372 post graduate trainees have been trained in STD/ HIV care provision during the year 2022. The central STD/HIV clinic, Colombo is the main centre of training for the postgraduate trainees in Venereology. A total of 48 venereology trainees have been trained at this setting in 2022.

Category of staff	Type of HCW	Number of programmes	Number of participants
STD clinic staff (pre-service & in-service)	Medical officers	42	312
	Nursing officers/PHNS	36	280
	PHI	27	63
	Laboratory staff	34	144
	Supportive staff	32	350
	Other	16	264
Other Health staff	Medical officers	83	1040
	Nursing officers/PHNS	53	1341
	PHI	35	182
	Laboratory staff	40	88
	Supportive staff	34	820
	Other	37	1453
Undergraduate/ basic HCW trainees	Medical students	295	1880
	Nursing officers/PHNS	82	1206
	PHI	0	0
	Laboratory staff	14	220
	Other	4	262
Postgraduate/ Post basic HCW trainees	Venereology	27	118
	Other medical specialities	69	254
	Nursing officers	3	150
	Other	0	0
	Other	33	4611
Other staff	NGO staff (MA, OW etc.)	36	563
	Peer educators	19	208
	Other	33	4611

Figure 13.1 : Inservice and preservice training carried out for hospital staff in 2022

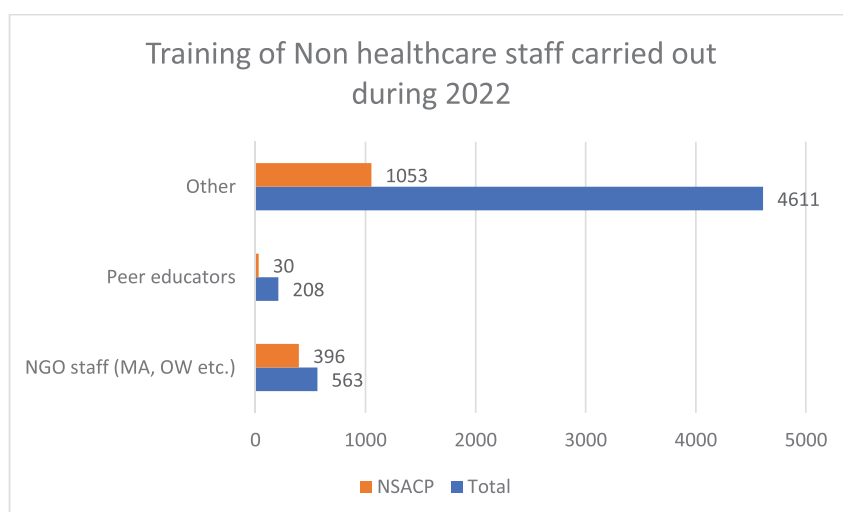
During the year 2022 a total of 469 training programmes have been carried by island wide STD clinics and was able to train 6337 health care workers. This includes STD clinic staff as well as other hospital staff. The topics covered by each programme are described

above. Out of the total training programmes nearly one fifth of the staff have been trained by NSACP. The NRL provide pre service and Inservice training for island wide MLTs and PHLTS.

Figure 13.2 : Post graduate/post basic training carried out during 2022

During the year 2022, 372 postgraduate trainees and 150 post basic nurses have been trained by the programme out of

which majority (73%) have been trained by the Central clinic (NSACP)

Figure 13.3 : Training of Non healthcare staff carried out during 2022

In addition to the health care staff, the programme provides training and capacity building for non-healthcare staff. Non-government organization (NGO) and community-based organization (CBO) staff working with the NSACP undergo training before starting their recruitment. HIV /STI knowledge, counselling, risk assessment, HIV testing monitoring and data management are some of the important areas covered

during pre-service and in service training programmes. In addition, police offices, triformes, prison staff, youth offices and various other categories of stakeholders undergo HIV/STI related training to facilitate HIV programmes within their sectors. Those programmes are conducted by training unit, KP unit, SIM unit and district STD clinics.

CHAPTER 14

IEC AND ADVOCACY PROGRAMS

Janaka Weragoda¹
Thilani Ratnayake²

National STD/AIDS control program recognizes the importance of making public as well as key population and vulnerable groups aware of STI/HIV as a cornerstone for HIV and STI prevention. As HIV is an evolving subject continuous effort for education with most updated information and novel methods of HIV treatments is essential to minimize the transmission of STIs/HIV as well as provide best patient care. Economic crisis during 2022 restricted most IEC and advocacy programs. In spite of limited resources following activities were completed during 2022.

1. IEC subcommittee meeting
2. Awareness program via text messages
3. Improve condom distribution through condom vending machines
4. Procuring flavored condoms
5. Adding lubricants to the National Formulary in Sri Lanka
6. Development of STD clinic promoting leaflet
7. Development of 6 video clips to promote

STD clinics for among KP and Vulnerable groups and promote condoms

8. Conduction of district and provincial AIDS Committee meetings
9. Advocacy programs for media and NGOs with the support of HPB

IEC subcommittee meetings

The information, Education, and Communication (IEC) subcommittee is one of the subcommittees of the national AIDS committee. It gathers every quarterly to discuss and develop national activities on HIV/STI prevention. The subcommittee consists of multiple stakeholders including health and non-health government sectors, private sectors, and Non -Governmental Organizations. It is chaired by Dr. Kapila Sooriyaarachchi, Head of health promotion unit; Department of public health-CMC and the secretary is the national coordinator for IEC, advocacy, and condom promotion unit. One subcommittee meeting was conducted physically at the auditorium of NSACP on 17th February 2022 and decisions were taken to change the name of the unit as Sexual Health Promotion Unit

¹ Consultant Community Physician

² Consultant Venereologist

Awareness program through text messages

Health information messages were developed and sent through text messages to make public aware of HIV and STI. The activity was supported by SLT Mobitel targeting 20-40 age group. But the activity was not continued because of difficulties with the support from the management team of SLT Mobitel which had concerns of the nature of the messages which they thought would have a negative impact on the marketing of other products.

Improve condom distribution through condom vending machines

Condom dispensers were placed at railway stations of main cities in the country mainly targeting KP groups and Tri-forces. Approval from the railway department was taken and refilling of dispensers is through PHIs and field workers of NGO.

Procuring flavored condoms and adding lubricants into the National Formulary

As there was a request for flavoured condoms, decision was taken to procure flavoured condoms from government as well as from GFATM funds. Lubricants were added to the National Formulary in Sri Lanka which is another successful achievement for prevention of HIV and STIs.

Development of leaflet to promote STD clinic services

STD and HIV services are provided through 41 island wide STD clinics covering all districts in Sri Lanka. But the availability of this service has not been reached to every

corner especially rural and marginalized population groups who are at risk of HIV and STI. This new leaflet on STD clinic was designed to promote STD clinics and services available to be disseminated among key population groups and other health staff to scale up uptake of sexual health services.

Development of video clips

Making STD clinics popular and user friendly for KP and vulnerable groups is very important them to come forward for services and correct information. Fear of stigma and discrimination prevent key population and vulnerable groups attending STD clinics where most facilities are available in the country. Therefore, four video clips developed targeting these population groups to promote services available at STD clinics.

Correct and consistent condom use is well proven method for prevention of HIV with minimum side effects. NSACP distribute free condoms targeting high risk and vulnerable population. To promote condoms two video clips were developed by the IEC unit in 2022.

Conduction of District AIDS Committees (DAC) and Provincial AIDS Committee (PAC) meetings

Prevention of new HIV and sexually transmitted infections and provision of comprehensive care and treatment services for all diagnosed patients is not an easy task and support from other health authorities and multisectoral involvement is required at national level as well as at district level.

The basis for multi sectoral commitment is through District AIDS Committee, Provincial AIDS Committee, where peripheral STD

CHAPTER 15

WORLD AIDS DAY 2022

Janaka Weragoda¹
Thilani Ratnayake²

As in previous years NSACP together with other stake holders commemorated the World AIDS Day on 1st of December 2022.

It was considered as an opportunity to increase public awareness on HIV prevention and recognize the role of stakeholders of the HIV response to reach the goal of ending AIDS. World AIDS on which people unite to support individuals who are living with HIV and to grieve those who have died from AIDS-related illnesses, is essential for enhancing the quality of life for those who are HIV positive

In line with global theme “Equalize” (සමානාත්මතාවය සුරකිමු) Sri Lanka commemorated World AIDS day on December 1st, 2022, with awareness-raising activities around the country, make a voice on issues of people living with HIV (PLHIV), with the support of district STD clinics, government institutions and non-government organization

Many activities were conducted island wide covering all districts to commemorate the world AIDS day 2022 emphasizing the theme of the year “equalize”.

- National level commemoration was jointly organized by the NSACP and Family Planning Association Sri Lanka,

at Colombo with the participation of all stake holders reprinting government organizations, nongovernment organizations and international organizations including representative from PLHIV and key population groups.

- Island wide awareness general public awareness campaign was carried out at national level and district level by means of using mass media (electronic and printed media), social media, conferences, workshops and commemorative walks etc....
- Two official media conferences were conducted at Health Promotion Bureau and the Department of Government Information
- Posters and leaflets were distributed on novel preventive measure pre-exposure prophylaxis (PrEP), post exposure prophylaxis (PEP), HIV self-test and online app “KNOW4SURE”.
- Special HIV testing campaigns were carried out targeting key population with support of FPA, NGO/CBOs.
- Many advocacy programmes were conducted for main stakeholders to make them aware of the theme of world AIDS day and update on HIV.

¹ Consultant Community Physician

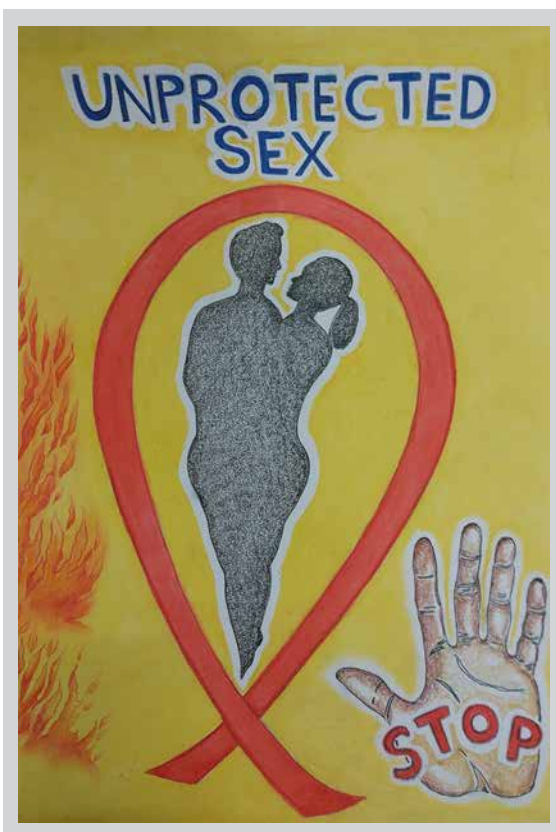
² Consultant Venereologist

Media conference at Department of Government information



Multi sectoral unit of the NSACP with the Department of prison organized poster competition among prison inmates with the participation of all the prisons island-wide. The main aim of the event was to increase the awareness and commitment of HIV and STI prevention among prison inmates and give them an opportunity to show their talents. More than 300 posters were presented for the competition and best poster were selected by independent group of judge panel. Certificates and gift items were awarded for the winner of the poster competition.

Winning poster (the 1st Place) of the Inter prisons poster competition – 2022



Prize awarding for the winners of the inter prisons poster competition at the National AIDS day Commemoration



Awareness program in Vavuniya





Advocacy and awareness programs were conducted by island wide STD clinics jointly with PLHIV and other stakeholders that work towards the same goal.



Kalmunei STD staff conducted an awareness program for hospital staff to address the stigma and discrimination



Awareness program in Vavuniya



Awareness program in Vavuniya

CHAPTER 16

UPDATE OF ELECTRONIC SYSTEMS OF NSACP

Lakshan Fernando¹

The National STD/AIDS Control Programme has taken significant steps towards improving the efficacy of HIV prevention services and the provision of clinical care for key populations, people affected by HIV and other sexually transmitted infections.

Current electronic and digital platforms of NSACP are listed below.

1. Electronic Information Management System (EIMS)
2. Prevention Information Management System (PIMS)
3. Know4sure.lk web site
4. NSACP website and dashboard
5. E-Learning platform for EIMS and know4sure.lk
6. Google sheet based Health Information Management System.

All above systems and platforms are monitored by the Strategic Information Management (SIM) unit of NSACP.

Electronic information management system (EIMS)

The Electronic Information Management System (EIMS) was introduced in 2019 as an electronic medical record system with modules for registration, STD Clinic Management, HIV care, Laboratory Information Management, and Pharmacy Management. User feedback led to modifications being made to the system. The IT team of the SIM unit visits STD clinics across the country to install the system and provide guidance on its use.

Despite these efforts, the implementation of EIMS has faced challenges, such as logistical issues faced by peripheral clinics, deficiencies in infrastructure, limited IT knowledge



Main electronic systems of NSACP

¹Consultant Microbiologist

among staff, and limited human resources in the SIM unit. These factors have particularly slowed down installation in remote areas.

However, the SIM unit is actively working to overcome these challenges and ensure the smooth functioning of the system. In 2022, supervisory visits and training programs were held on a smaller scale due to the country's economic crisis and instability.

Prevention information management system (PIMS)

The Prevention Information Management System (PIMS) was established to monitor key population-related HIV prevention programs conducted by STD clinics and NGO partners under the peer-led intervention model. The PIMS comprises a website and an Android mobile App. Its launch was held on February 22, 2022.

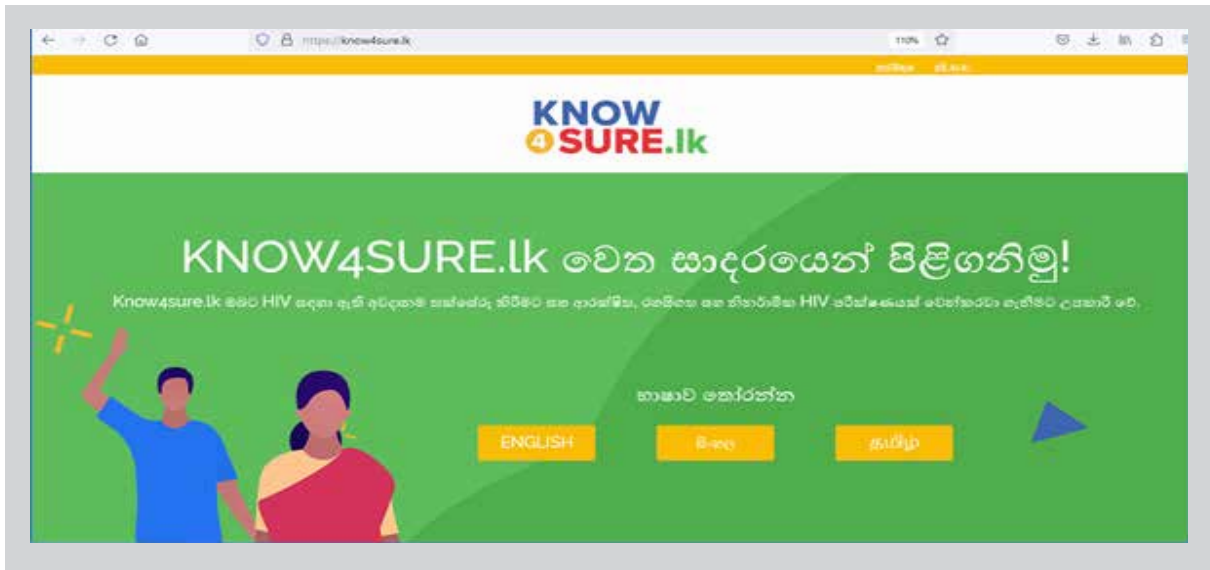
The system is currently being rolled out to all key population units of STD clinics since 2022 and is operational in eight out of the 15 KP intervention districts, including

KP units under STD clinics in Kandy, Jaffna, Kalutara, Negombo, Mahamodara, Matara, Hambantota, and the Colombo (Drug user component). The Family Planning Association (FPA-SL), which works in Colombo and Gampaha districts for MSM, TG, PWID, and FSW components, is also successfully utilizing the platform.

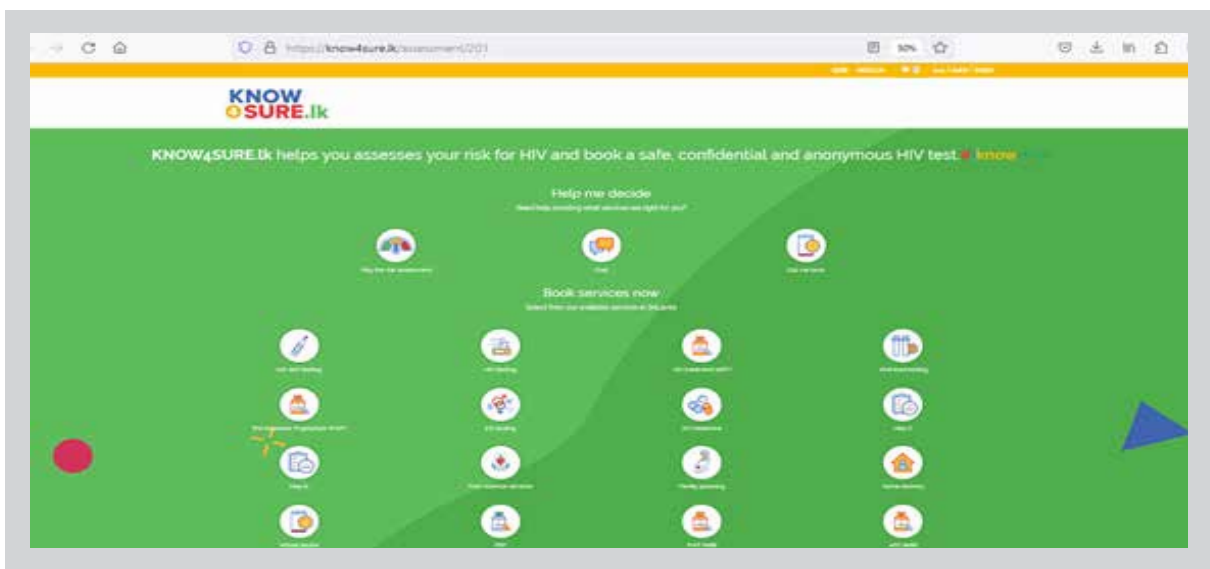
In 2023, PIMS will be rolled out to the rest of the KP units by the last quarter and will be taken to the field using the PIMS mobile application by outreach workers for data entry.

Know4sure.lk web site

Know4sure.lk is an online platform that aims to provide confidential STD services to high-risk and vulnerable populations who often find partners through virtual social media platforms and dating apps. The platform offers virtual HIV services, where clients can book online appointments and receive health products such as condoms, lubricants, and HIV self-test kits.



The online outreach workers are trained to help clients and manage inquiries in consultation with the clinic doctors. The Family Planning Association, a sub-recipient under the Global Fund grant, also offers virtual outreach services with their own online outreach workers.

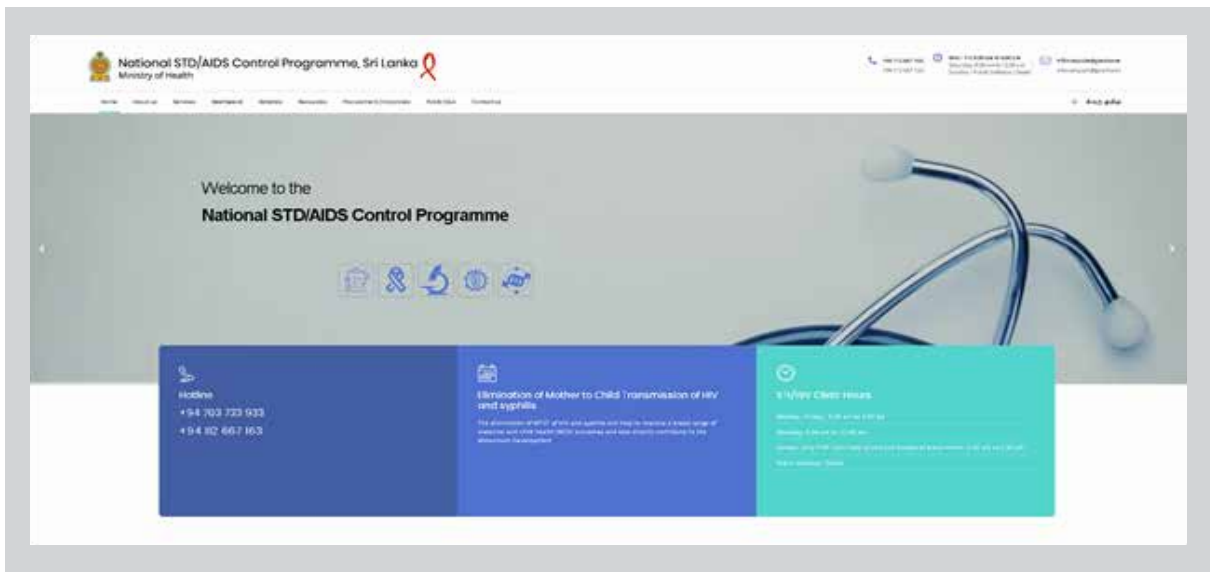


Services provided via the know4sure.lk website

To ensure the quality of service, all virtual outreach workers and clinic medical officers underwent training on September 27th, 2022, to handle the Know4sure.lk platform.

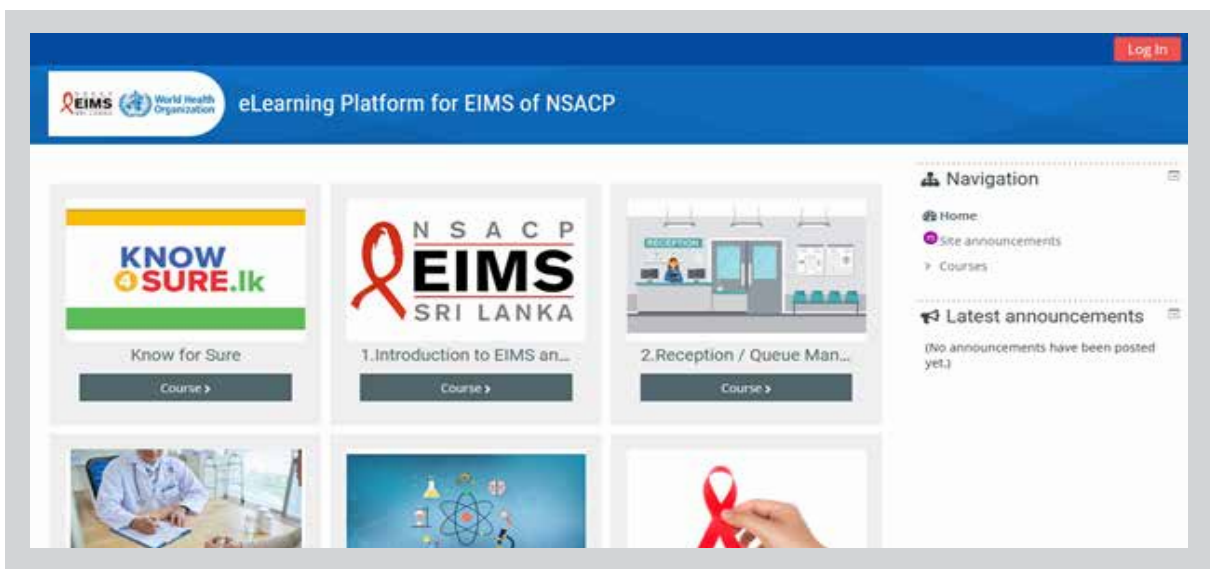
NSACP website and dashboard for key population interventions

Updated NSACP website with new features helps better support the stakeholders and key population interventions. The website can be accessed through this link: <https://www.aidscontrol.gov.lk>.



E-Learning platform

NSACP developed an e-Learning System for the EIMS (ELS) to ensure sustainability of training on EIMS and currently updated to access videos and demonstrations on know4sure.lk. This system is accessible through <http://eims.nsacp.headstartcloud.com/>



e-Learning platform EIMS of NSACP (including know4sure demo videos)

he e-learning platform currently consists of information and learning video demonstrations on EIMS and know4sure.lk

Quarterly return google-sheet

The SIM unit has introduced a new electronic data management system to replace the traditional paper-based data recording system. This new system includes three Google-sheets for capturing STD, HIV ART data and a cross-sectional database Google-sheet. The data entered into these Google-sheets are handled and analyzed by the SIM unit to present it to various stakeholders.

1. STD quarterly return google-sheet
2. ART quarterly return google-sheet
3. Cross sectional database google-sheet

These sheets are used by the peripheral STD clinic and KP staff to enter the data in real time. The SIM unit conducted training

sessions for users of these electronic systems to ensure that the quarterly return google-sheets are being utilized effectively.

The use of electronic data management systems like these Google-sheets allows for more efficient and accurate data collection, analysis, and reporting. It eliminates the need for paper-based records which are prone to errors, can be difficult to manage, and require additional storage space. Overall, these electronic systems help improve the management and delivery of STD and HIV ART services.

CHAPTER 17

UPDATE OF GF SUPPORTED ACTIVITIES 2022

Sathya Herath¹

Year 2022 had been the initial implementing year of the three-year (2022-2024) grant cycle. Total three-year grant value was US\$ 7, 377,649.77. Ministry of Health being the Principal

Recipient, contracted the Family Planning as the Sub Recipient. The Price water house Coopers (Private) Limited functions as the Local Funding agent.

Activities supported by the Global Fund in 2022

NSP Strategic Direction	Strategy/Activities supported by Global Fund
Prevention	NGO/CBO supported Peer led HIV prevention program for Key population (KP)- Operating Community Dropping in Center, Salaries & incentives
Prevention	Prison Peer led HIV prevention Program
Prevention	Biomedical Prevention - Pre-exposure prophylaxis (PrEP) for MSM & TG
Prevention	Community Based interventions & Harm Reduction Interventions for People who inject drugs (PWID)
Prevention	Differentiated HIV testing services for KP – Procurement, procurement supply chain managing of HIV, Syphilis, Hepatitis B & C Rapid Diagnostic Tests, HIV Self Tests,
Prevention	Condom programing to KP – Procurement of male & Female condoms, Lubricants.
Prevention	Addressing stigma, discrimination, violence, Initial, re fresher training programs, advocacy programs, KP interventions reviews, supervisions, program implementers meetings, information material printing, facilitate outreach activities.
Diagnosis, treatment and care	Improve Coverage of STI services, Improving ART adherence & monitoring – Training & meetings, regular meetings of subcommittees.

¹Consultant Community Physician)

Diagnosis, treatment and care	Scaling up quality laboratory Services- Procurement of CD4, Viral Load cartridges, Establishment and functioning of Drug Resistance Services in Sri Lanka.
Diagnosis, treatment and care	Emergency procurement of Anti Retro Viral medications, Procurement of PrEP medications.
Strategic information management system	Health management information systems and M&E – establishing and maintenance of Program monitoring & routine reporting – scaling up, maintenance of Electronic Information Management System (EIMS) & Prevention Information Management System (PIMS),
Health systems strengthening	Grant & Program Management costs (Human resources & other costs)
Supportive Environment	Programs to reduce human rights-related barriers to HIV services,

Major Activity Progress in 2022

While continuing and strengthening the routine global fund supported activities, NSACP focused scaling up new interventions. The following are significant among other routine interventions mentioned in the Table of Global Fund supported activities.

1. Scale up of Pre-Exposure Prophylaxis, establishing and expanding community PrEP clinics. Establishing Sunday PrEP clinics both in the STD clinics and within the communities.
2. Scale up of HIV self-Tests – establishing informal techniques such as sending test through outreach workers, placing orders and getting the tests through online and hotline, developing courier services for HIV self-test delivery.
3. Scaling up of Needle Syringe Exchange Services (NSES) for People who inject drugs
4. Scaling up and implementing Prevention Information Management System (PIMS)

5. Establishing ART Drug Resistance Services at the National Reference Services

6. Though, the program was continuously procuring ART and proportion of other health commodities using the domestic finances, due to country's economic situation, the program requests special support from Global fund. The Global Fund approved an emergency fund for the procurement of ART, Condoms, & HIV test kits.

Annual Performance rating 2022

The year 2022 physical and financial progress was reported through Progress Update and Disbursement Request for the financial year 2022. Physical progress was reported against agreed targets for each physical indicator.

Financial Progress

The financial absorption for the reporting period 1 January 2022 -31 December 2022

was 26% which is rated as “very poor” due to only a 23.6% absorption.

The total expenditure for the reporting period was US\$ 1,022,719, vis-à-vis the approved 2022 budget of US\$ 4,328,497.

Reason for poor performance was significant activities were not initiated due to lack of human resources available and delay in procurement processes.

Costing Dimension (Cost Grouping / Cost Input)	Reporting Period Budget	Global Fund Validated Expenditure for the Reporting Period	Cumulative period Budget	Global Fund Validated Cumulative Expenditure
Currency	USD			
Indirect and Overhead Costs	1,735,170.90	84,018.73	1,735,170.90	84,193.04
Human Resources (HR)	495,524.02	373,860.72	495,524.02	401,703.90
Communication Material and Publications (CMP)	15,768.21	8,135.39	15,768.21	989.37
Travel related costs (TRC)	90,241.20	43,520.83	90,241.20	22,649.66
Non-health equipment (NHP)	444,055.84	46,797.00	444,055.84	46,796.58
External Professional services (EPS)	293,289.52	14,802.04	293,289.52	14,802.04
Health Products - Non-Pharmaceuticals (HPNP)	429,654.94	206,733.98	429,654.94	206,733.98
Health Products - Pharmaceutical Products (HPPP)	493,814.00	133,281.00	493,814.00	133,281.00
Infrastructure (INF)	110,078.59	1,245.16	110,078.59	1,245.16
Procurement and Supply-Chain Management costs (PSM)	151,538.54	71,928.75	151,538.54	71,928.75
Health Products - Equipment (HPE)	35,799.60	7,631.42	35,799.60	7,631.42
Living support to client/ target population (LSCTP)	33,561.50	30,764.09	33,561.50	30,764.09
Payment for Results	0.00	0.00	0.00	0.00
Grand Total	4,328,497	1,022,719	4,328,497	1,022,719

Physical Progress

Programmatic rating was by using programmatic rating tool. The program obtained “Moderate rating” for the

programmatic rating. Below is the pictorial explanation of both financial and programmatic progress.

Performance Rating Tool

Grant: LKA-H-MOH

Rated Period: 1 Jan. 2022 to 31 Dec 2022

Country: Sri Lanka

Principal Recipient: Ministry of Health of the Democratic Socialist Republic of Sri Lanka

Implementation Period: 01 Jan 2022 to 31 Dec 2024

Annual performance rating



Management Adjustment: Not applied

Performance Rating Evolution

Performance Rating

Rated Period	Annual Rating Year 1 1 Jan. 2022 31 Dec. 2022	Annual Rating Year 2 1 Jan. 2023 31 Dec. 2023	Annual Rating Year 3 1 Jan. 2024 31 Dec. 2024
Programmatic Rating	C		
Rated Period	1 Jan. 2022 31 Dec. 2022	1 Jan. 2022 31 Dec. 2023	1 Jan. 2022 31 Dec. 2024
Financial Rating	5		

Programmatic Rating Summary

Grant: LKA-H-MOH

Rated Period: 1 Jan. 2022 to 31 Dec 2022

Country: Sri Lanka

Principal Recipient: Ministry of Health of the Democratic Socialist Republic of Sri Lanka

Implementation Period: 01 Jan 2022 to 31 Dec 2024

Programmatic Rating

Programmatic Rating		Legend	
Average Performance All Indicators:	82%	A	Excellent >=100%
All Indicators Rating:	C	B	Good 90% - 99%
Quantitative Indicator Rating:	C	C	Moderate 60% - 89%
Programmatic Technical Adjustment:	Not Applied	D	Poor 30% - 59%
Management Adjustment:	Not Applied	E	Very Poor <30%
Programmatic Rating	C		

CHAPTER | 18

Financial Summery



CHAPTER 18

FINANCIAL SUMMARY 2022

| **S. Muraliharan¹**

The National STD/AIDS Control Programme (NSACP) is responsible for managing funds and coordinating initiatives related to the prevention and control of sexually transmitted diseases (STDs) and HIV/AIDS in Sri Lanka.

The economic crisis that Sri Lanka experienced in 2022 had a significant impact on the overall utilization of funds allocated to the NSACP. During times of economic crisis, governments often face budget constraints and may prioritize spending in certain areas

over others. Consequently, the allocation for healthcare programs, including the NSACP, has been reduced, resulting in low utilization of funds during that year.

However, it is important to note that the funds allocated by the Ministry of Health of GoSL for peripheral STD clinics through the provincial allocations have not been included in the budget shown below. These funds are not directly managed by the NSACP, but they are an important source of funding for STD prevention and control activities in Sri Lanka.

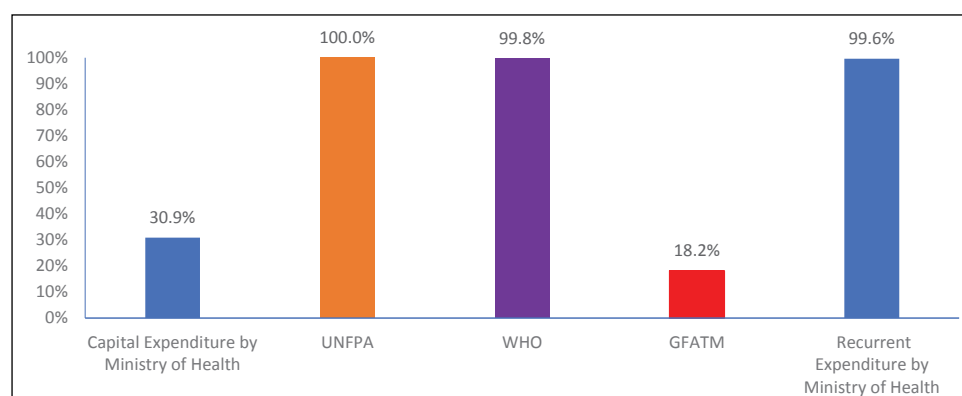
¹Medical officer, SIM unit

Table XX: Financial allocation against annual budget utilization - 2022

Financial Source	Description	Fund Allocation (LKR)	Fund Utilization (LKR)
I. Capital Expenditure			
Ministry of Health	Building Construction	20,000,000.00	1,798,947.51
	DDG (PH)I	10,000,000.00	6,651,584.00
	Service Agreement	1,163,573.64	1,163,573.64
	Subtotal	31,163,573.64	9,614,105.15
UNFPA	Consultative workshops, advocacy programmes, the printing of the publication	750,000.00	750,000.00
WHO	Consultative workshops, and review meetings. training module	304,204,638.03	303,499,000.89
GFATM	Human Resources (HR)	95,152,040.24	54,313,918.58
	Travel-related costs (TRC)	26,778,854.20	13,042,553.32
	External Professional Services (EPS)	104,726,990.37	8,164,177.28
	Health Products - Non-Pharmaceuticals (HPNP)	154,675,778.58	74,747,363.55
	Health Products - Equipment (HPE)	12,887,856.49	2,780,143.00
	Procurement and Supply-Chain Management (PSM)	54,553,874.40	26,168,049.29
	Infrastructure (INF)	39,628,291.90	450,086.00
	Non-health equipment (NHE)	158,135,095.87	15,173,740.65

Figure XX: Percentage of fund utilization, 2022

Financial Source	Description	Fund Allocation (LKR)	Fund Utilization (LKR)
	Communication Material and Publications (CMP)	5,240,741.34	2,647,342.64
	Antiretroviral medicines	177,773,040.00	48,596,849.55
	Office-related costs	2,534,543.17	103,923.93
	Indirect and Overhead Costs	603,604,258.61	15,215,214.55
	Subtotal	1,435,691,365.19	261,403,362.34
Total Capital Expenditure		1,771,809,576.86	575,266,468.38
Financial Source	Description	Fund Allocation (LKR)	Fund Utilization (LKR)
2. Recurrent Expenditure			
Ministry of Health	Personal Emoluments	169,807,200.25	170,870,927.73
	Traveling Expenses	372,800.00	284,110.00
	Supplies	5,942,500.00	4,441,004.41
	Maintenance Expenditure	3,500,000.00	1,624,969.77
	Services	10,530,000.00	15,006,716.02
	Transfers	1,361,000.00	309,128.93
	Reagents	24,177,575.87	23,955,991.72
	Training Allowances	14,500,000.00	12,630,430.40
	Antiretroviral drugs	47,717,138.22	47,717,138.22
Total Recurrent Expenditure		277,908,214.34	276,840,417.20
Grand Total (LKR)		2,049,717,791.20	852,106,885.58



CONTACT INFORMATION OF STD CLINICS 2023

A CENTRAL PROVINCE

1. Dambulla STD clinic

Address	STD clinic, District Base Hospital, Dambulla.
Email	stdclinicdambulla@gmail.com
Telephone	066-2284761 (GH)
Contact person	Dr. Anuradha Perera (Acting Venereologist)

2. Kandy STD clinic

Address	STD clinic, P.O. Box 207, Kandy.
Email	stdclinic.kandy@gmail.com
Telephone	081-2203622
Fax	081-2203923
Contact Persons	Dr. Jagath Ranawaka (Acting Venereologist) Dr. M.I.M. Lareef (MO/IC)

3. Matale STD clinic

Address	STD clinic, District General Hospital, Matale.
Email	stdclinic.matale@gmail.com
Telephone	066-2053746
Contact persons	Dr. Iruka Rajapaksha (Venereologist) Dr. K.W.K.K.A. Bandara (MO/IC)

4. Nawalapitiya STD clinic

Address	STD clinic, District General Hospital, Nawalapitiya
Email	NA
Telephone	054-2222261 (GH)
Contact persons	Dr. Kasun Ekanayake

5. Nuwara Eliya STD clinic

Address	STD clinic, District General Hospital, Nuwara Eliya.
Email	stdnuwaraeliya@gmail.com
Telephone	052-2223210, 052-2222261 (GH) Ext: 345
Fax	052-2223476 (GH)
Contact persons	Dr. Chathurika Wickramaratne (Acting Venereologist) Dr.A. Sampath (MO/IC)

B | EASTERN PROVINCE

6. Ampara STD clinic

Address	STD clinic, District General Hospital, Ampara.
Email	stdclinicampara1@gmail.com
Telephone	063-2224239
Fax	063-2222988 (RDHS Office)
Contact person	Dr. Piyumi Perera (Venereologist) Dr. Sakunthala Zoysa (MO/IC)

7. Batticaloa STD clinic

Address	STD clinic, Health Friendly Center, 1st floor of Chest Clinic, Hospital Rd, Batticaloa.
Email	stdbatti@gmail.com
Telephone	065-2057078
Fax	065-2224401 (TH)
Contact persons	Dr Tharmaratnam Thivakaran (MO/IC)

8. Kalmunai STD clinic

Address	STD clinic, District General Hospital, Sainthamaruthu, Sainthamaruthu.
Email	stdkalmunai@gmail.com
Telephone	067-2223660
Fax	067-2223660
Contact person	Dr. M.N.M.Thilshan (MO/IC)

9. Trincomalee STD clinic

Address	STD clinic, District General Hospital, Trincomalee.
Email:	shctrinco@gmail.com
Telephone	026-2222563
Fax	026-2222563
Contact person	Dr.A. Devarajah (MO/IC)

C NORTH CENTRAL PROVINCE**10. Anuradhapura STD clinic**

Address	STD clinic, Room No 11, Teaching Hospital, Anuradhapura
Email	stdclinic.anuradhapura@gmail.com
Telephone	025-2236461, 071-8103001
Fax	025-2225616 (TH)
Website	https://sites.google.com/view/sexual-health-anuradhapura/home
Contact person	Dr.Ajith Karawita (Venereologist) Dr Thilani Rathnayake (Venereologist)

11. Polonnaruwa STD clinic

Address	STD clinic, District General Hospital, Polonnaruwa.
Email	stdclinicpolonnaruwa1@gmail.com
Telephone	027-2225787
Fax	027-2225787
Contact Persons	Dr. Prageeth Premadasa (Venereologist) Dr. Indra Peris (MO/IC)

D NORTH WESTERN PROVINCE

12. Chilaw STD clinic

Address	STD clinic, General Hospital, Chilaw.
Email	std.rdhspu@gmail.com
Telephone	032-2220750
Fax	032-2223200 (GH)
Contact persons	Dr. Subashini Jayasuriya (Acting Venereologist)

13. Kuliapitiya STD clinic

Address	STD clinic, Teaching Hospital, Kuliapitiya.
Email	stdclinicKuliapitiya@gmail.com
	032-2220750
Telephone	037-2281261
Contact person	Dr. Vino Dharmakulasinghe (Venereologist)

14. Kurunegala STD clinic

Address	STD clinic, Teaching Hospital, Kurunegala.
Email	stdclinic.kurunegala@gmail.com
Telephone	037-2224339
Fax	037-2224339
Contact persons	Dr. C. Hathurusinghe (Venereologist) Dr. Nihal Edirisinghe (MO/IC)

15. Puttalam STD clinic

Address	Unit 13, Base Hospital Puttalam, Puttalam.
Email	stdputtalama@gmail.com
Telephone	0322-265 261 (GH)
Contact person	Dr. Geethika Namarathna (MO/IC)

E NORTHERN PROVINCE

16. Jaffna STD clinic

Address	STD clinic, Teaching Hospital, Jaffna.
Email	stdclinic.jaffna@gmail.com
Telephone	021-2217756
Fax	021-2222262 (TH)
Contact persons	Dr.A. Rohan (MO/IC)

17. Kilinochchi STD clinic

Address	STD clinic, District General Hospital, Kilinochchi.
Email	stdkilinochchi@gmail.com
Telephone	021-2283709, 021-2285329 (GH) Ext: 194
Fax	021-2285327 (GH)
Contact persons	Dr. Elankumaran Velayathapillai (MO/IC)

18. Mannar STD clinic

Address	STD clinic, District General Hospital, Mannar.
Email	stdclinic.mannar@gmail.com
Telephone	023-2250573
Fax	023-2250748 (RDHS Office)
Contact persons	Dr.Thakshagini Mahendranathan (Acting Venereologist) Dr. Dayani Dias (MO/IC)

19. Mullaitivu STD clinic

Address	STD clinic, District General Hospital, Mullaitivu.
Email	stdaidscontrolprogramme.mtv@gmail.com
Telephone	021-2061414
Contact person	Dr.A.Thayalaseelan (MO/IC)

20. Vavuniya STD clinic

Address	STD clinic, District General Hospital, Vavuniya.
Email	stdclinic.vavuniya@gmail.com
Telephone	024-2224575
Fax	024-2222892 (RDHS Office)
Contact persons	Dr. Priyantha Batagalla (Venereologist) Dr. D.Arulmoly (MO/IC)

F SABARAGAMUWA PROVINCE

21. Kegalle STD clinic

Address	STD clinic, District General Hospital, Kegalle.
Email	stdunit.kegalle@gmail.com
Telephone	035-2231222
Fax	035-2231222
Contact persons	Dr. Shyama Somawardana (Venereologist) Dr. D.K. Nilanga Jeewani Jayasinghe (MO/IC)

22. Ratnapura STD clinic

Address	STD clinic, Teaching Hospital Ratnapura.
Email	stdclinic.rathnapura@gmail.com
Telephone	045-2221561 (Venereologist)
	045-2226561
Contact persons	Dr. Upuli Abeyrathna (Acting Venereologist) Dr. H.A.K.A. Jayarathne (MO/STD)

23. Embilipitiya STD clinic

Address	STD clinic, District General Hospital, Embilipitiya.
Email	stdclinic.embilipitiya@gmail.com
Telephone	047-2230261 (GH)
Fax	047-2230141
Contact persons	Dr. Sachini Mendis (Acting Venereologist)

G SOUTHERN PROVINCE

24. Balapitiya STD clinic

Address	STD clinic, Base Hospital, Balapitiya.
Email	stdbalapitiya@gmail.com
Telephone	091-2256822
Fax	091-2256410 (BH)
Contact persons	Dr Heshani Colombage (Acting Venereologist), Dr. H.D. Fernando (MO/IC)

25. Galle STD clinic

Address	STD clinic, Teaching Hospital, Mahamodara, Galle.
Email	stdclinic.mahamodara@gmail.com
Telephone	091-2245998
Fax	091-2232088
Contact persons	Dr. Darshani Wijewickrema (Venereologist)

26. Tangalle STD clinic

Address	STD clinic , Base hospital, Tangalle.
Email	stdclinictangalle@gmail.com
Telephone	091-2245998
	047-2240261 Ext: 220
Contact person	Dr. K.A.S. Nilanka (MO)

27. Hambantota STD clinic

Address	STD clinic, General Hospital, Hambantota.
Email	stdclinic.hambantota@gmail.com
Telephone	047-2222247
Fax	047-2222247
Contact persons	Dr. Gayan Mahakumbura (Venereologist) Dr. L.K.H.M. Jayaruwan (MO/IC)

28. Matara STD clinic

Address	STD clinic, No 43, District General Hospital, Matara.
Email	stdclinic.matara@gmail.com
Telephone	041-2232302
Fax	041-2232302
Contact persons	Dr. Umedha Nilakshi Jayasinghe (Venereologist) Dr. Sunethra Kandambi (MO/IC)

H UVA PROVINCE

29. Badulla STD clinic

Address	STD clinic, Room No 73, Daya Gunasekara Mawatha, Badulla.
Email	stdclinic.badulla@gmail.com
Telephone	055-2222578
Fax	055-2222578
Contact persons	Dr. Niroshan Jayasekara (Venereologist) Dr. R. Crishan Vedanayagam (MO/IC)

30. Mahiyanganaya STD clinic

Address	STD clinic, Room 22, Base Hospital, Mahiyanganaya.
Email	stdclinicmyg@gmail.com
Telephone	055-4936779
Fax	055-2223750
Contact person	Dr. Udari Gallage (Acting Venereologist)

31. Monaragala STD clinic

Address	STD clinic, District General Hospital, Monaragala.
Email	monaragalastd@gmail.com
Telephone	055-2276826
Fax	055-2276700 (RDHS Office) 055-2276912 (GH)
Contact person	Dr. D. R. K.W. M. R. S. Kaushalya (MO/IC)

I WESTERN PROVINCE

32. Avissawella STD clinic

Address	STD clinic, Room 5, OPD Complex, Base Hospital, Avissawella.
Email	stdavissawella@yahoo.com
	055-2222578
Telephone	036-2222003
	036-2222261/62 (BH) Ext: 228
Contact person	Dr. Gayani Nanayakara (Venereologist) Dr. Ayesha Rupasinghe (MO)

33. Colombo Central STD clinic (NSACP)

Address	29, De Saram Place, Colombo 10.
Web link	www.aidscontrol.gov.lk
Email	info@aidcontrol.gov.lk
Telephone	011-2667163 (Exchange)
Hot lines	011-2695420 (Female clinic), 011-2-695430 (Male clinic)
Fax	011-2665277
Contact persons	Dr. Janaki Vidanapathirana (Acting Director) Dr. K.A. Manathunge Ariyaratne (Venereologist) Dr. Jayanthi. P. Elwitigala (Microbiologist) Dr. Sathya Herath (Community Physician) Dr. Geethani Samaraweera (Venereologist) Dr. Janaka Weragoda (Community Physician) Dr Nimali Jayyasuriya (Venereologist)

34. Gampaha STD clinic

Address	STD Clinic, District General Hospital, Gampaha.
Email	stdclinic.gampaha@gmail.com
Telephone	033-2234383
Fax	033-2222179 (GH)
Contact persons	Dr Manjula Rajapakshe (Venereologist) Dr. Jayantha Amarasinghe (MO/IC)

35. Homagama STD clinic

Address	OPD Building, Base Hospital, Homagama.
Telephone	011-2855200
Contact person	Dr. Shanika Jayasena (Acting Venereologist)

36. Kalubowila STD clinic

Address	STD Clinic, District General Hospital, Gampaha.
Email	stdclinic.gampaha@gmail.com
Telephone	033-2234383
Fax	033-2222179 (GH)
Contact persons	Dr Manjula Rajapakshe (Venereologist) Dr. Jayantha Amarasinghe (MO/IC)

37. Kalutara STD clinic

Address	STD clinic, General Hospital, Nagoda, Kalutara.
Email	stdclinic.kalutara@gmail.com
Telephone	034-2236937
Fax	034-2236937
Contact persons	Dr. Lasanthi Siriwardena (Venereologist)

38. Negombo STD clinic

Address	STD clinic, District General Hospital, Negombo.
Email	stdclinic.negombo@gmail.com
Telephone	031-2239016, 031-2222261 (GH)
Contact persons	Dr Priyantha Weerasinghe (Venereologist) Dr. Shriyantha De Silva (MO/IC)

39. Panadura STD clinic

Address	STD clinic, Base Hospital, Panadura.
Telephone	038-2232261
Contact person	Dr. Kokilanthi Dharmaratna (Acting Venereologist)

40. Ragama STD clinic

Address	STD clinic, Room 70, Teaching Hospital, Ragama.
Email	stdclinic.ragama@gmail.com
Telephone	011-2960224
Fax	011-2960224, 011-2959266 (TH)
Contact persons	Dr. Jayadari Ranatunga (Venereologist) Dr. Chamantha Wijerathna (MOIC)

41. Wathupitiwala STD clinic

Address	STD clinic, Base Hospital, Wathupitiwala.
Email	stdcampaign.bswathupitiwala@yahoo.com
Telephone	033-2280261 Ext: 255
Fax	033-2280927
Contact person	Dr. S.P.P. Gunawardhana (MO/IC)

ANNEX - 1

ANNEX-I

Reported infectious syphilis cases, 2020-2022

Province	Clinic	2020			2021			2022		
		Male	Female	Total	Male	Female	Total	Male	Female	Total
Central Province	Dambulla	1	1	2	1	0	1	1	0	1
	Kandy	1	0	1	3	1	4	6	2	8
	Matale	0	0	0	0	0	0	1	1	2
	Nawalapitiya	0	0	0	0	0	0	1	0	1
	Nuwara Eliya	2	0	2	0	1	1	0	0	0
Eastern Province	Ampara	0	0	0	1	0	1	3	0	3
	Batticaloa	7	6	13	8	4	12	2	1	3
	Kalmunai	0	0	0	0	0	0	2	2	4
	Trincomalee	4	4	8	1	1	2	1	0	1
North Central province	Anuradhapura	0	0	0	0	0	0	0	0	0
North Western Province	Polonnaruwa	3	0	3	1	1	2	1	0	1
	Chilaw	0	0	0	0	0	0	2	2	4
	Kuliapitiya	0	0	0	1	0	1	2	0	2
	Kurunegala	0	0	0	4	0	4	2	0	2
	Puttalam	0	0	0	1	0	1	2	2	4
Northern Province	Jaffna	1	1	2	6	3	9	0	0	0
	Kilinochchi	1	0	1	1	1	2	0	0	0
	Mannar	0	0	0	0	0	0	0	0	0
	Mullaitivu	0	0	0	2	1	3	1	0	1
	Vavuniya	4	3	7	1	1	2	1	0	1
Sabaragamuwa Province	Embilipitiya	0	0	0	1	0	1	0	0	0
Southern Province	Kegalle	0	0	0	0	0	0	1	0	1
	Ratnapura	1	0	1	2	1	3	2	0	2
	Balapitiya	12	1	13	1	1	2	2	0	2
	Hambanthota	3	1	4	3	0	3	6	1	7
	Mahamodara	3	0	3	7	3	10	12	2	14
	Matara	9	2	11	5	1	6	3	1	4
	Tangalle	0	0	0	0	0	0	1	0	1
UVA Province	Badulla	0	6	6	5	5	10	6	2	8
	Mahiyanganaya	1	0	1	0	0	0	1	1	2
	Monaragala	-	-	-	9	5	14	1	1	2
Western Province	Avissawella	1	1	2	3	0	3	6	1	7
	Colombo	17	7	24	31	4	35	49	8	57
	Gampaha	5	2	7	4	0	4	10	1	11
	Homagama	0	0	0	0	0	0	4	0	4
	Kalubowila	11	0	11	12	0	12	19	0	19
	Kalutara	3	2	5	4	3	7	3	2	5
	Negombo	3	1	4	1	2	3	11	3	14
	Panadura	0	0	0	2	0	2	5	0	5
	Ragama	15	4	19	11	3	14	12	2	14
	Wathupitiwala	0	0	0	3	2	5	2	0	2
Total		108	42	150	135	44	179	184	35	219

ANNEX-I Reported late syphilis cases, 2020-2022

Province	Clinic	2020			2021			2022		
		Male	Female	Total	Male	Female	Total	Male	Female	Total
Central Province	Dambulla	0	0	0	0	2	2	2	0	2
	Kandy	17	7	24	14	2	16	18	8	26
	Matale	6	1	7	3	3	6	1	3	4
	Nawalapitiya	0	0	0	0	0	0	0	0	0
	Nuwara Eliya	3	4	7	7	7	14	9	4	13
Eastern Province	Ampara	2	4	6	3	2	5	2	0	2
	Batticaloa	12	1	13	11	4	15	9	6	15
	Kalmunai	1	0	1	1	0	1	4	0	4
	Trincomalee	4	0	4	8	5	13	9	3	12
North Central province	Anuradhapura	10	5	15	3	1	4	9	4	13
	Polonnaruwa	7	6	13	6	6	12	16	1	17
	Chi law	9	4	13	11	5	16	16	8	24
	Kuliyapitiya	1	0	1	0	1	1	3	3	6
	Kurunegala	14	10	24	21	11	32	17	8	25
	Puttalam	2	1	3	1	1	2	2	3	5
Northern Province	Jaffna	16	10	26	11	7	18	21	7	28
	Kilinochchi	1	1	2	0	2	2	1	0	1
	Mannar	1	1	2	1	1	2	3	2	5
	Mullaitivu	0	0	0	0	0	0	0	0	0
	Vavuniya	2	2	4	2	3	5	2	3	5
Sabaragamuwa Province	Embilipitiya	0	0	0	3	2	5	1	0	1
	Kegalle	10	3	13	7	0	7	4	2	6
	Ratnapura	13	4	17	9	8	17	3	9	12
	Balapitiya	22	3	25	1	0	1	3	0	3
	Hambanthota	14	7	21	11	3	14	20	3	23
	Mahamodara	16	9	25	19	12	31	17	8	25
	Matara	11	3	14	6	4	10	7	3	10
	Tangalle	1	0	1	1	0	1	0	0	0
UVA Province	Badulla	6	1	7	10	4	14	7	1	8
	Mahiyanganaya	0	0	0	4	2	6	2	1	3
	Monaragala	—	—	—	1	3	4	0	0	0
Western Province	Avissawella	2	1	3	5	0	5	3	2	5
	Colombo	68	29	97	75	30	105	109	37	146
	Gampaha	6	2	8	14	4	18	12	1	13
	Homagama	0	0	0	0	3	3	3	0	3
	Kalubowila	22	7	29	20	3	23	24	4	28
	Kalutara	9	8	17	9	6	15	7	4	11
	Negombo	12	4	16	10	7	17	10	4	14
	Panadura	0	1	1	6	2	8	6	0	6
	Ragama	23	3	26	15	5	20	10	3	13
	Wathupitiwala	7	1	8	3	0	3	11	3	14
Total		350	143	493	332	161	493	403	148	551

ANNEX-I

Reported non gonococcal cases, 2020-2022

Province	Clinic	2020			2021			2022		
		Male	Female	Total	Male	Female	Total	Male	Female	Total
Central Province	Dambulla	3	2	5	0	8	8	4	44	48
	Kandy	19	38	57	18	42	60	19	53	72
	Matale	7	84	91	4	48	52	13	74	87
	Nawalapitiya	0	0	0	0	0	0	2	2	4
	Nuwara Eliya	2	5	7	2	5	7	1	5	6
Eastern Province	Ampara	1	2	3	23	16	39	17	31	48
	Batticaloa	7	3	10	1	0	1	5	14	19
	Kalmunai	1	2	3	2	0	2	2	1	3
	Trincomalee	4	4	8	4	8	12	0	4	4
North Central province	Anuradhapura	21	20	41	13	29	42	36	29	65
	Polonnaruwa	16	30	46	17	25	42	13	10	23
	Chilaw	0	16	16	2	14	16	3	21	24
	Kuliyapitiya	1	2	3	2	0	2	8	7	15
	Kurunegala	76	151	227	24	76	100	18	89	107
	Puttalam	1	1	2	2	4	6	0	11	11
Northern Province	Jaffna	8	0	8	11	2	13	11	0	11
	Kilinochchi	9	10	19	0	1	1	3	0	3
	Mannar	2	0	2	0	0	0	1	1	2
	Mullaitivu	1	0	1	0	0	0	0	0	0
	Vavuniya	7	3	10	3	7	10	8	9	17
Sabaragamuwa Province	Embilipitiya	13	39	52	15	22	37	15	33	48
	Kegalle	7	23	30	4	3	7	4	3	7
	Ratnapura	5	2	7	3	1	4	6	6	12
	Balapitiya	16	14	30	5	10	15	1	3	4
	Hambanthota	8	23	31	5	10	15	2	5	7
	Mahamodara	10	33	43	14	18	32	16	37	53
	Matara	18	29	47	4	18	22	10	14	24
	Tangalle	0	2	2	7	6	13	3	0	3
UVA Province	Badulla	8	25	33	7	30	37	9	23	32
	Mahiyanganaya	11	12	23	12	111	123	19	104	123
	Monaragala	—	—	—	4	43	47	4	42	46
Western Province	Avissawella	6	0	6	3	0	3	3	0	3
	Colombo	112	326	438	82	156	238	181	366	547
	Gampaha	12	65	77	7	33	40	11	65	76
	Homagama	2	1	3	1	5	6	24	32	56
	Kalubowila	171	188	359	78	58	136	107	85	192
	Kalutara	8	17	25	8	24	32	14	205	219
	Negombo	10	46	56	6	58	64	8	50	58
	Panadura	6	13	19	11	10	21	6	9	15
	Ragama	49	79	128	19	46	65	44	54	98
	Wathupitiwala	4	4	8	3	11	14	8	13	21
Total		662	1314	1976	426	958	1384	659	1554	2213

ANNEX-I Reported non gonococcal cases, 2020-2022

Province	Clinic	2020			2021			2022		
		Male	Female	Total	Male	Female	Total	Male	Female	Total
Central Province	Dambulla	3	2	5	0	8	8	4	44	48
	Kandy	19	38	57	18	42	60	19	53	72
	Matale	7	84	91	4	48	52	13	74	87
	Nawalapitiya	0	0	0	0	0	0	2	2	4
	Nuwara Eliya	2	5	7	2	5	7	1	5	6
Eastern Province	Ampara	1	2	3	23	16	39	17	31	48
	Batticaloa	7	3	10	1	0	1	5	14	19
	Kalmunai	1	2	3	2	0	2	2	1	3
	Trincomalee	4	4	8	4	8	12	0	4	4
North Central province	Anuradhapura	21	20	41	13	29	42	36	29	65
	Polonnaruwa	16	30	46	17	25	42	13	10	23
	Chilaw	0	16	16	2	14	16	3	21	24
	Kuliyapitiya	1	2	3	2	0	2	8	7	15
	Kurunegala	76	151	227	24	76	100	18	89	107
	Puttalam	1	1	2	2	4	6	0	11	11
Northern Province	Jaffna	8	0	8	11	2	13	11	0	11
	Kilinochchi	9	10	19	0	1	1	3	0	3
	Mannar	2	0	2	0	0	0	1	1	2
	Mullaitivu	1	0	1	0	0	0	0	0	0
	Vavuniya	7	3	10	3	7	10	8	9	17
Sabaragamuwa Province	Embilipitiya	13	39	52	15	22	37	15	33	48
	Kegalle	7	23	30	4	3	7	4	3	7
	Ratnapura	5	2	7	3	1	4	6	6	12
	Balapitiya	16	14	30	5	10	15	1	3	4
	Hambanthota	8	23	31	5	10	15	2	5	7
	Mahamodara	10	33	43	14	18	32	16	37	53
	Matara	18	29	47	4	18	22	10	14	24
	Tangalle	0	2	2	7	6	13	3	0	3
UVA Province	Badulla	8	25	33	7	30	37	9	23	32
	Mahiyanganaya	11	12	23	12	111	123	19	104	123
	Monaragala	—	—	—	4	43	47	4	42	46
Western Province	Avissawella	6	0	6	3	0	3	3	0	3
	Colombo	112	326	438	82	156	238	181	366	547
	Gampaha	12	65	77	7	33	40	11	65	76
	Homagama	2	1	3	1	5	6	24	32	56
	Kalubowila	171	188	359	78	58	136	107	85	192
	Kalutara	8	17	25	8	24	32	14	205	219
	Negombo	10	46	56	6	58	64	8	50	58
	Panadura	6	13	19	11	10	21	6	9	15
	Ragama	49	79	128	19	46	65	44	54	98
	Wathupitiwala	4	4	8	3	11	14	8	13	21
Total		662	1314	1976	426	958	1384	659	1554	2213

ANNEX-I

Reported herpes cases, 2020-2022

Province	Clinic	2020			2021			2022		
		Male	Female	Total	Male	Female	Total	Male	Female	Total
Central Province	Dambulla	0	2	2	3	8	11	6	20	26
	Kandy	26	72	98	19	53	72	41	78	119
	Matale	19	29	48	15	16	31	17	26	43
	Nawalapitiya	0	0	0	2	7	9	3	7	10
	Nuwara Eliya	4	16	20	7	14	21	3	8	11
Eastern Province	Ampara	20	23	43	23	25	48	17	34	51
	Batticaloa	7	12	19	3	11	14	0	12	12
	Kalmunai	1	7	8	3	5	8	1	9	10
	Trincomalee	8	13	21	9	6	15	14	17	31
North Central province	Anuradhapura	36	64	100	25	47	72	38	55	93
	Polonnaruwa	29	46	75	24	33	57	24	39	63
	Chilaw	35	34	69	28	17	45	42	47	89
	Kuliyapitiya	10	8	18	8	10	18	12	21	33
	Kurunegala	47	71	118	54	58	112	48	74	122
	Puttalam	1	6	7	1	16	17	8	11	19
Northern Province	Jaffna	12	7	19	5	11	16	18	25	43
	Kilinochchi	7	12	19	4	1	5	10	15	25
	Mannar	3	0	3	0	0	0	0	1	1
	Mullaitivu	0	1	1	3	7	10	4	4	8
	Vavuniya	13	11	24	9	13	22	5	15	20
Sabaragamuwa Province	Embilipitiya	21	37	58	17	30	47	31	36	67
	Kegalle	48	61	109	41	45	86	34	29	63
	Ratnapura	33	59	92	25	51	76	47	78	125
	Balapitiya	43	27	70	11	19	30	9	12	21
	Hambanthota	30	36	66	16	24	40	33	37	70
	Mahamodara	40	57	97	23	34	57	40	55	95
	Matara	27	50	77	11	21	32	33	31	64
	Tangalle	1	2	3	6	8	14	8	12	20
	Badulla	21	38	59	23	39	62	15	46	61
UVA Province	Mahiyanganaya	8	23	31	16	23	39	17	40	57
	Monaragala	—	—	—	7	22	29	10	28	38
Western Province	Avissawella	40	27	67	19	21	40	32	34	66
	Colombo	161	106	267	109	86	195	205	115	320
	Gampaha	34	32	66	19	26	45	26	28	54
	Homagama	0	9	9	1	5	6	15	16	31
	Kalubowila	95	93	188	63	62	125	89	75	164
	Kalutara	44	78	122	17	62	79	23	42	65
	Negombo	29	41	70	25	42	67	28	36	64
	Panadura	8	17	25	13	19	32	14	19	33
	Ragama	65	44	109	31	40	71	55	55	110
	Wathupitiwala	18	30	48	14	14	28	19	33	52
Total		1044	1301	2345	752	1051	1803	1094	1375	2469

ANNEX-I Reported warts cases, 2020-2022

Province	Clinic	2020			2021			2022		
		Male	Female	Total	Male	Female	Total	Male	Female	Total
Central Province	Dambulla	5	7	12	6	13	19	11	9	20
	Kandy	40	28	68	37	40	77	35	31	66
	Matale	22	14	36	17	13	30	23	26	49
	Nawalapitiya	0	0	0	0	6	6	1	5	6
	Nuwara Eliya	5	7	12	1	4	5	7	3	10
Eastern Province	Ampara	23	18	41	23	15	38	16	20	36
	Batticaloa	7	3	10	4	4	8	8	6	14
	Kalmunai	2	0	2	6	2	8	9	8	17
	Trincomalee	6	9	15	16	10	26	11	6	17
North Central province	Anuradhapura	50	51	101	40	31	71	38	37	75
	Polonnaruwa	31	21	52	22	25	47	20	18	38
	Chilaw	27	22	49	20	14	34	24	25	49
	Kuliyapitiya	17	10	27	14	13	27	25	27	52
	Kurunegala	46	54	100	105	71	176	33	28	61
	Puttalam	8	6	14	27	12	39	17	13	30
Northern Province	Jaffna	22	16	38	33	11	44	25	13	38
	Kilinochchi	3	2	5	1	1	2	1	2	3
	Mannar	0	1	1	0	1	1	2	0	2
	Mullaitivu	0	0	0	3	0	3	3	0	3
	Vavuniya	8	5	13	12	9	21	11	7	18
Sabaragamuwa Province	Embilipitiya	24	15	39	15	11	26	27	9	36
	Kegalle	54	36	90	37	28	65	21	23	44
	Ratnapura	34	32	66	23	43	66	52	55	107
	Balapitiya	29	23	52	12	7	19	2	8	10
	Hambanthota	42	29	71	29	20	49	32	25	57
	Mahamodara	27	20	47	30	34	64	34	27	61
	Matara	48	32	80	26	19	45	25	22	47
	Tangalle	5	3	8	10	7	17	4	11	15
UVA Province	Badulla	26	32	58	21	25	46	30	11	41
	Mahiyanganaya	7	19	26	12	12	24	5	12	17
	Monaragala	—	—	—	5	11	16	14	21	35
Western Province	Avissawella	26	30	56	17	17	34	21	23	44
	Colombo	160	70	230	164	65	229	239	92	331
	Gampaha	49	41	90	24	30	54	37	28	65
	Homagama	7	9	16	3	6	9	12	10	22
	Kalubowila	86	44	130	67	36	103	66	47	113
	Kalutara	50	45	95	48	33	81	50	51	101
	Negombo	36	30	66	24	15	39	36	29	65
	Panadura	9	15	24	12	10	22	23	10	33
	Ragama	66	43	109	48	31	79	52	49	101
	Wathupitiwala	10	16	26	9	8	17	19	13	32
Total		1117	858	1975	1023	763	1786	1121	860	1981

ANNEX-I Reported trichomonosis cases, 2020-2022

Province	Clinic	2020			2021			2022		
		Male	Female	Total	Male	Female	Total	Male	Female	Total
Central Province	Dambulla	0	0	0	0	0	0	0	0	0
	Kandy	0	1	1	0	0	0	0	1	1
	Matale	0	1	1	0	0	0	0	0	0
	Nawalapitiya	0	0	0	0	0	0	0	0	0
	Nuwara Eliya	0	0	0	0	1	1	0	0	0
Eastern Province	Ampara	0	0	0	0	0	0	0	0	0
	Batticaloa	0	0	0	4	0	4	0	0	0
	Kalmunai	0	0	0	0	0	0	0	0	0
	Trincomalee	0	0	0	0	0	0	1	0	1
North Central province	Anuradhapura	0	0	0	0	1	1	0	0	0
	Polonnaruwa	0	0	0	0	1	1	0	0	0
	Chilaw	0	0	0	0	0	0	0	0	0
	Kuliyapitiya	0	1	1	0	0	0	0	1	1
	Kurunegala	0	1	1	0	0	0	0	3	3
	Puttalam	0	0	0	0	0	0	0	0	0
Northern Province	Jaffna	0	0	0	0	0	0	0	0	0
	Kilinochchi	2	1	3	0	0	0	0	0	0
	Mannar	0	0	0	0	0	0	0	0	0
	Mullaitivu	0	0	0	0	0	0	0	0	0
	Vavuniya	0	0	0	0	1	1	0	0	0
Sabaragamuwa Province	Embilipitiya	0	0	0	0	0	0	0	0	0
	Kegalle	0	2	2	0	1	1	0	1	1
	Ratnapura	0	2	2	0	2	2	0	0	0
	Balapitiya	0	2	2	0	0	0	0	0	0
	Hambanthota	0	0	0	0	4	4	0	5	5
	Mahamodara	0	0	0	0	0	0	0	0	0
	Matara	0	0	0	0	0	0	0	0	0
	Tangalle	0	0	0	0	1	1	0	0	0
UVA Province	Badulla	0	8	8	0	3	3	0	2	2
	Mahiyanganaya	0	1	1	0	0	0	0	0	0
	Monaragala	-	-	-	0	0	0	1	1	2
Western Province	Avissawella	0	0	0	0	0	0	0	2	2
	Colombo	1	9	10	0	10	10	3	9	12
	Gampaha	0	1	1	0	1	1	0	0	0
	Homagama	0	0	0	0	0	0	0	0	0
	Kalubowila	2	1	3	0	3	3	0	0	0
	Kalutara	0	0	0	0	1	1	0	0	0
	Negombo	0	0	0	0	0	0	1	0	1
	Panadura	0	1	1	1	1	2	0	0	0
	Ragama	0	1	1	0	0	0	0	2	2
	Wathupitiwala	0	0	0	0	0	0	0	0	0
Total		5	33	38	5	31	36	6	27	33

ANNEX - 2

ANNEX-2

Number of clinic attendees and details of clinic attendances for STD clinics during 2022

		New patients registered			Total new patients with STIs	Total clinic visits by STD patients	Total visits by others
		Male	Female	Total			
Central Province	Dambulla	96	117	213	118	497	899
	Kandy	439	444	883	444	2,509	6,603
	Matale	238	322	560	259	532	1,894
	Nawalapitiya	28	45	73	38	104	100
	Nuwara Eliya	95	110	205	64	514	1,448
Eastern Province	Ampara	221	224	445	441	1,085	742
	Batticaloa	92	194	286	85	561	2,911
	Kalmunai	48	61	109	47	380	404
	Trincomalee	126	131	257	118	357	5,654
North Central Province	Anuradhapura	357	365	722	358	1,339	10,690
	Polonnaruwa	286	312	598	261	1,226	1,229
North Western Province	Chilaw	214	254	468	253	1,850	1,040
	Kuliyaipitiya	70	130	200	122	431	476
	Kurunegala	313	370	683	800	1,798	3,384
	Puttalam	128	196	324	177	1,284	1,476
Northern Province	Jaffna	230	120	350	202	761	7,012
	Kilinochchi	63	57	120	68	249	2,121
	Mannar	86	91	177	19	290	2,427
	Mullaitivu	100	65	165	15	216	1,980
	Vavuniya	100	88	188	109	554	3,384
Sabaragamuwa Province	Embilipitiya	120	105	225	113	566	665
	Kegalle	164	176	340	234	1,233	2,468
	Ratnapura	437	441	878	359	1,474	3,682
Southern Province	Balapitiya	163	174	337	81	362	1,359
	Hambanthota	372	304	676	385	2,131	4,072
	Mahamodara	304	225	529	247	2,092	2,961
	Matara	332	219	551	251	1,673	6,025
	Tangalle	101	62	163	48	227	115
UVA Province	Badulla	872	796	1,668	313	3,249	1,114
	Mahiyanganaya	213	371	584	215	825	792
	Monaragala	148	164	312	152	744	4,836
Western Province	Avissawella	141	160	301	175	1,068	1,694
	Colombo	3,527	1,393	4,920	2,508	15,062	4,125
	Gampaha	355	224	579	283	2,051	3,102
	Homagama	139	119	258	149	609	724
	Kalubowila	639	392	1,031	488	4,254	3,190
	Kalutara	355	387	742	472	2,141	1,080
	Negombo	234	278	512	303	1,747	1,982
	Panadura	233	151	384	129	1,167	276
	Ragama	466	305	771	492	2,716	1,334
	Wathupitiwala	138	121	259	190	510	1,037
Total		12,783	10,263	23,046	11,585	62,438	102,507

ANNEX-2

Reason for attendance among New STD clinic attendees in 2022

Province	Clinic	Contact of patients	Voluntarily	Referral from magistrate/ court	Others
Central Province	Dambulla	18	60	62	80
	Kandy	70	301	112	399
	Matale	27	148	88	295
	Nawalapitiya	3	26	18	22
	Nuwara Eliya	14	11	68	112
Eastern Province	Ampara	14	143	87	220
	Batticaloa	6	19	61	200
	Kalmunai	8	13	27	65
	Trincomalee	15	24	65	157
North Central province	Anuradhapura	50	314	48	310
	Polonnaruwa	23	250	116	208
North Western Province	Chilaw	44	171	50	213
	Kuliyapitiya	16	36	71	77
	Kurunegala	39	496	122	410
	Puttalam	17	120	126	61
Northern Province	Jaffna	10	68	21	255
	Kilinochchi	8	4	11	97
	Mannar	4	8	60	105
	Mullaitivu	1	9	16	140
	Vavuniya	17	39	54	78
Sabaragamuwa Province	Embilipitiya	3	96	72	51
	Kegalle	14	290	59	131
	Ratnapura	36	240	277	330
Southern Province	Balapitiya	6	100	45	186
	Hambanthota	21	192	216	254
	Mahamodara	53	133	76	267
	Matara	21	198	131	194
	Tangalle	7	18	52	38
UVA Province	Badulla	11	163	59	1435
	Mahiyanganaya	12	91	28	294
	Monaragala	10	57	105	140
Western Province	Avissawella	7	70	46	178
	Colombo	197	3092	38	1593
	Gampaha	40	224	32	282
	Homagama	29	60	17	152
	Kalubowila	43	566	98	324
	Kalutara	24	869	191	256
	Negombo	38	138	120	216
	Panadura	13	118	39	214
	Ragama	26	268	84	393
	Wathupitiwala	10	78	27	145
Total		1025	9321	3095	10577

ANNEX-2

Number of patients with confirmed syphilis diagnoses completing treatment during 2022

Province	STD clinic	Number of diagnosed with syphilis	Number completed treatment	No. of pregnant women diagnosed with syphilis	No. of pregnant women completed treatment
Central Province	Dambulla	3	2	0	0
	Kandy	34	31	0	0
	Matale	6	6	2	3
	Nawalapitiya	1	1	0	0
	Nuwara Eliya	13	12	3	1
Eastern Province	Ampara	3	3	0	0
	Batticaloa	18	18	4	4
	Kalmunai	8	8	0	0
	Trincomalee	14	14	2	3
North Central province	Anuradhapura	12	11	1	4
	Polonnaruwa	18	12	0	0
North Western Province	Chilaw	22	20	1	1
	Kuliyapitiya	8	7	0	0
	Kurunegala	8	8	3	3
	Puttalam	9	9	0	2
Northern Province	Jaffna	28	27	3	8
	Kilinochchi	1	1	0	0
	Mannar	4	3	1	1
	Mullaitivu	1	1	0	0
	Vavuniya	6	6	0	0
Sabaragamuwa Province	Embilipitiya	1	1	0	0
	Kegalle	1	1	0	0
	Ratnapura	14	14	0	0
Southern Province	Balapitiya	5	4	0	0
	Hambanthota	29	28	2	1
	Mahamodara	39	36	2	2
	Matara	14	12	2	4
	Tangalle	1	1	0	0
UVA Province	Badulla	11	5	0	0
	Mahiyanganaya	3	3	0	0
	Monaragala	3	3	0	0
Western Province	Avissawella	11	11	0	0
	Colombo	196	165	8	8
	Gampaha	24	23	0	0
	Homagama	6	6	0	1
	Kalubowila	45	39	4	4
	Kalutara	16	11	0	0
	Negombo	28	27	2	2
	Panadura	11	9	1	1
	Ragama	26	23	1	1
	Wathupitiwala	12	6	1	1
Total		713	628	43	55

ANNEX-2

Samples screened for HIV infection during 2022

Province	Clinic	No. screened for HIV (ELISA,PA,RAPID)				No. confirmed HIV positive				
		STD pt. samples	Ante- natal samples	Pre emp. samples	Other samples	STD pt. samples	Ante- natal samples	Pre emp. samples	Other sam- ples	Total
Central Province	Dambulla	275	15	311	76	1	-	-	-	1
	Kandy	2,510	4,287	-	2,394	17	-	-	5	22
	Matale	600	6,806	813	1,654	2	-	-	-	2
	Nawalapitiya	37	1	-	25	-	-	-	-	-
	Nuwara Eliya	326	3,874	443	574	3	1	-	-	4
Eastern Province	Ampara	522	2,249	134	158	-	-	-	-	-
	Batticaloa	286	7,004	1,054	3,140	1	-	-	-	1
	Kalmunai	147	6,101	160	619	1	-	-	-	1
	Trincomalee	283	3,811	1,112	712	5	-	-	-	5
North Central province	Anuradhapura	1,442	9,119	2,545	2,640	23	1	1	-	25
	Polonnaruwa	898	3,795	433	9,579	2	1	-	3	6
North Western Province	Chilaw	1,410	6,877	350	607	7	-	-	2	9
	Kuliyaipitiya	267	-	-	67	-	-	-	-	-
	Kurunegala	2,061	9,613	-	198	20	-	-	-	20
	Puttalam	207	45	457	46	2	-	-	-	2
Northern Province	Jaffna	346	4,920	1,749	3,880	2	-	1	-	3
	Kilinochchi	117	1,473	577	150	1	-	-	-	1
	Mannar	118	1,525	792	180	-	-	-	-	-
	Mullaitivu	165	1,233	435	37	-	-	-	-	-
	Vavuniya	188	2,090	637	548	2	-	-	-	2
abaragamuwa Province	Embilipitiya	570	22	541	283	1	-	-	-	1
	Kegalle	959	4,750	32	1,282	8	-	-	-	8
	Ratnapura	2,026	9,632	2,379	2,777	10	-	-	1	11
Southern Province	Balapitiya	313	3,778	909	1,829	1	-	-	-	1
	Hambanthota	715	5,751	690	2,099	8	1	-	1	10
	Mahamodara	1,340	5,254	283	1,434	21	1	-	13	35
	Matara	1,032	5,754	1,692	1,149	9	1	-	2	12
	Tangalle	261	16	84	154	-	-	-	-	-
UVA Province	Badulla	2,557	6,198	2,487	1,623	2	-	-	-	2
	Mahiyanganaya	588	1,709	453	243	1	-	-	-	1
	Monaragala	384	6,796	1,442	512	-	-	-	2	2
Western Province	Avissawella	338	25	495	157	4	-	-	-	4
	Colombo	7,551	32,140	291	32,409	136	1	-	582	719
	Gampaha	747	7,501	674	264	16	2	-	-	18
	Homagama	283	3	-	7	3	-	-	-	3
	Kalubowila	1,215	156	790	1,196	22	-	1	3	26
	Kalutara	1,212	11,642	23	841	7	-	-	1	8
	Negombo	727	4,410	206	699	17	-	-	-	17
	Panadura	355	5	3	107	9	-	-	-	9
	Ragama	1,482	4,052	-	1,245	48	1	-	15	64
	Wathupitiwala	375	136	-	321	8	-	-	-	8
Total		37,235	184,568	25,476	77,915	420	10	3	630	1,063

ANNEX-2

Samples screened for HIV infection during 2022

Province	Clinic	Number screened for syphilis				Number TPPA/TPHA tested			
		STD	ANC	Pre-Emp.	Other	STD	ANC	Pre-emp.	Other
Central Province	Dambulla	267	14	366	77	207	14	0	56
	Kandy	1427	4342	0	761	420	6	0	39
	Matale	541	6703	663	1249	213	54	3	331
	Nawalapitiya	10	0	43	0	1	0	0	0
	Nuwara Eliya	454	5348	821	555	239	162	11	52
Eastern Province	Ampara	729	2249	382	51	447	0	0	0
	Batticaloa	286	7004	1773	940	17	4	0	25
	Kalmunai	160	6183	127	498	84	5	4	14
	Trincomalee	186	3538	1113	276	279	95	32	354
North Central province	Anuradhapura	1442	9119	2545	2640	423	26	5	30
	Polonnaruwa	881	3795	515	4494	420	1055	0	230
North Western Province	Chilaw	1410	4764	888	518	829	12	4	167
	Kuliapitiya	260	0	151	23	259	0	0	23
	Kurunegala	2903	11717	358	0	1082	12	8	0
	Puttalam	207	45	34	471	186	23	3	24
Northern Province	Jaffna	362	5015	1809	3932	285	10	99	534
	Kilinochchi	117	1473	577	160	43	3	0	0
	Mannar	118	1525	792	170	2	1	0	2
	Mullaitivu	135	1103	216	22	75	144	8	0
	Vavuniya	188	2090	637	588	188	249	3	407
Sabaragamuwa Province	Embilipitiya	348	20	311	132	272	0	0	95
	Kegalle	684	4754	1494	1246	272	21	5	7
	Ratnapura	2094	15253	2591	3192	1250	6	3	2008
Southern Province	Balapitiya	313	3778	909	898	313	2	1	103
	Hambanthota	280	5751	2857	1336	642	17	3	516
	Mahamodara	553	4769	1599	271	871	261	32	270
	Matara	591	5756	2148	129	413	21	13	17
	Tangalle	231	76	82	34	229	76	79	34
UVA Province	Badulla	1258	5751	2081	1449	544	43	16	20
	Mahiyanganaya	588	1448	517	243	427	0	0	204
	Monaragala	370	4121	1442	301	173	13	1	4
Western Province	Avissawella	134	25	599	29	214	2	5	26
	Colombo	6206	32140	1624	15275	4363	444	38	4991
	Gampaha	765	7499	1523	109	410	44	11	34
	Homagama	236	3	404	12	243	0	1	1
	Kalubowila	1305	157	1402	385	688	75	1	284
	Kalutara	703	11642	17	0	680	15	0	0
	Negombo	534	4412	358	257	401	73	9	79
	Panadura	417	9	126	64	194	0	0	21
	Ragama	1510	4060	39	478	774	18	0	120
	Wathupitiwala	259	136	496	214	4	12	13	37
Total		31,462	187,587	36,429	43,479	19,076	3,018	411	11,159

ANNEX-2 Samples tested for cervical cytology (PAP smears) during 2022					
Province	Clinic	Number screened for cervical cytology	Total number of reports received	Number satisfactory for reporting	Number reported as CIN-I or above
Central Province	Dambulla	0	0	0	0
	Kandy	61	61	53	0
	Matale	44	37	34	0
	Nawalapitiya	4	4	4	0
	Nuwara Eliya	0	0	0	0
Eastern Province	Ampara	1	0	0	0
	Batticaloa	0	0	0	0
	Kalmunai	1	1	1	0
	Trincomalee	5	4	4	2
North Central province	Anuradhapura	8	5	2	0
	Polonnaruwa	10	5	5	0
North Western Province	Chilaw	10	2	2	0
	Kuliyapitiya	7	7	6	1
	Kurunegala	30	34	28	1
	Puttalam	0	0	0	0
Northern Province	Jaffna	0	0	0	0
	Kilinochchi	0	0	0	0
	Mannar	0	0	0	0
	Mullaitivu	0	0	0	0
	Vavuniya	12	12	11	1
Sabaragamuwa Province	Embilipitiya	0	0	0	0
	Kegalle	9	9	7	0
	Ratnapura	5	1	1	0
Southern Province	Balapitiya	0	0	0	0
	Hambanthota	7	3	3	0
	Mahamodara	61	42	41	1
	Matara	18	19	19	0
	Tangalle	0	0	0	0
UVA Province	Badulla	7	8	8	0
	Mahiyanganaya	0	0	0	0
	Monaragala	8	0	0	0
Western Province	Avissawella	29	15	17	0
	Colombo	338	270	259	11
	Gampaha	18	18	16	0
	Homagama	9	4	4	0
	Kalubowila	167	161	149	2
	Kalutara	29	15	18	2
	Negombo	41	61	61	0
	Panadura	9	9	9	0
	Ragama	31	4	4	0
	Wathupitiwala	15	30	27	0
Total		994	841	793	21

ANNEX-2

Condom and lubricants distribution 2019-2022

Province	Clinic	2020		2021		2022	
		Number of condoms	Number of lubricants	Number of condoms	Number of lubricants	Number of condoms	Number of lubricants
Central Province	Dambulla	490	-	4,864	-	5,180	170
	Kandy	3,952	11,900	4,015	360	3,468	10
	Matale	30,107	1,890	44,652	720	49,333	1,180
	Nawalapitiya	-	-	2,182	-	3,824	-
	Nuwara Eliya	25,932	200	4,256	-	16,700	-
Eastern Province	Ampara	15,290	900	38,130	3,775	32,072	3,375
	Batticaloa	3,266	1,100	4,600	700	4,060	1,700
	Kalmunai	9,791	80	8,994	670	11,919	230
	Trincomalee	4,480	1,113	4,550	842	1,595	300
North Central province	Anuradhapura	22,142	2,000	30,576	4,080	55,779	3,620
	Polonnaruwa	32,863	1,000	53,380	-	60,264	80
North Western Province	Chilaw	8,050	5,170	14,523	2,735	41,797	17,460
	Kuliyaipitiya	1,820	1,350	1,308	450	4,764	594
	Kurunegala	25,034	3,900	54,778	10,181	70,115	21,819
	Puttalam	150	10	5,304	90	27,280	5,940
Northern Province	Jaffna	18,233	6,700	9,889	1,295	44,689	10,249
	Kilinochchi	3,500	4,500	5,000	-	3,300	1,500
	Mannar	980	-	1,500	-	1,550	2,000
	Mullaitivu	1,728	200	2,172	200	1,200	700
	Vavuniya	7,200	1,600	4,320	800	7,344	400
Sabaragamuwa Province	Embilipitiya	4,744	20	1,336	110	420	370
	Kegalle	22,348	2,800	16,804	2,500	30,324	5,000
	Ratnapura	21,995	6,600	13,680	150	16,835	1,400
Southern Province	Balapitiya	4,320	200	10,800	800	30,816	1,800
	Hambanthota	25,889	9,790	48,069	11,100	59,333	19,075
	Mahamodara	14,611	3,960	80,490	11,180	120,921	15,190
	Matara	21,335	2,650	79,196	5,358	46,807	4,656
	Tangalle	40	8	205	73	189	24
UVA Province	Badulla	6,474	800	43,867	-	26,816	135
	Mahiyanganaya	4,449	986	11,751	800	18,603	800
	Monaragala	-	-	5,038	305	7,712	595
Western Province	Avissawella	4,469	52	2,078	173	1,990	195
	Colombo	29,260	5,956	36,196	7,400	54,449	19,817
	Gampaha	4,762	800	6,580	365	7,713	660
	Homagama	-	-	-	-	544	-
	Kalubowila	24,432	11,350	26,977	7,665	28,368	5,960
	Kalutara	11,060	-	12,368	4,850	41,304	5,970
	Negombo	6,419	1,500	11,607	200	21,096	3,632
	Panadura	150	98	614	2	1,783	625
	Ragama	4,805	310	4,058	260	6,427	690
	Wathupitiwala	2,003	234	779	486	7,803	425
Total		428,573	91,727	711,486	80,675	976,486	158,346

ANNEX-2

HIV Testing and Counselling for Key populations-2022 -I

Province	Clinics	Sex Workers		MSM		Transgender	
		Total tested for HIV by STD_out-reach <25 y	Total tested for HIV by STD_out-reach >=25 y	Total tested for HIV by STD_out-reach <25 y	Total tested for HIV by STD_out-reach >=25 y	Total tested for HIV by STD_out-reach <25 y	Total tested for HIV by STD_out-reach >=25 y
Central	Dambulla	0	0	0	1	0	0
	Kandy	23	78	37	60	0	0
	Matale	4	153	5	26	0	1
	Nawalapitiya	0	0	0	0	0	0
	Nuwara Eliya	0	0	1	0	0	0
Eastern	Ampara	3	8	7	10	0	0
	Batticaloa	0	0	0	2	0	0
	Kalmunai	0	0	3	2	0	0
	Trincomalee	0	0	1	1	0	0
North Central	Anuradhapura	14	55	5	26	0	0
	Polonnaruwa	5	42	11	14	0	0
North Western	Chilaw	1	41	3	2	0	0
	Kuliyapitiya	0	6	0	6	0	0
	Kurunegala	12	380	91	166	0	0
	Puttalam	1	11	0	3	0	0
Northern	Jaffna	0	0	156	104	39	30
	Kilinochchi	0	0	0	0	0	0
	Mannar	0	0	0	0	0	0
	Mullaitivu	0	0	0	0	0	0
	Vavuniya	0	0	0	0	0	0
Sabaragamuwa	Embilipitiya	2	3	6	5	0	0
	Kegalle	0	0	2	5	0	0
	Ratnapura	1	3	0	0	0	0
Southern	Balapitiya	0	27	0	115	0	0
	Hambanthota	1	6	17	8	0	4
	Mahamodara	1	90	21	58	0	0
	Matara	2	21	4	11	0	0
	Tangalle	0	0	1	1	0	0
UVA	Badulla	26	142	0	0	0	0
	Mahiyanganaya	0	44	1	2	0	0
	Monaragala	0	4	2	4	0	0
Western	Avissawella	6	26	15	11	0	0
	Colombo	37	542	309	1157	8	40
	Gampaha	3	28	39	78	2	6
	Homagama	0	1	0	5	0	0
	Kalubowila	31	211	70	250	4	6
	Kalutara	0	7	4	22	0	0
	Negombo	1	8	12	21	0	5
	Panadura	2	5	21	22	0	0
	Ragama	2	15	14	51	0	0
	Wathupitiwala	0	0	18	43	0	0
Total		178	1,957	876	2,292	53	92

ANNEX-2

Cont., HIV Testing and Counselling for Key populations-2022-2

Province	Clinics	Beach boys		PWID or Drug users		Prisoners		Others	
		Total tested for HIV by STD_ out-reach <25 y	Total tested for HIV by STD_ out-reach >=25 y	Total tested for HIV by STD_ out-reach <25 y	Total tested for HIV by STD_ out-reach >=25 y	Total tested for HIV by STD_ out-reach <25 y	Total tested for HIV by STD_ out-reach >=25 y	Total tested for HIV by STD_ out-reach <25 y	Total tested for HIV by STD_ out-reach >=25 y
Central	Dambulla	-	-	-	-	-	-	-	-
	Kandy	-	-	-	-	15	36	-	-
	Matale	-	-	-	1	-	-	23	404
	Nawalapitiya	-	-	-	-	-	-	-	-
	Nuwara Eliya	-	-	-	-	-	-	36	19
Eastern	Ampara	-	-	3	2	-	-	116	196
	Batticaloa	-	-	-	-	-	6	-	411
	Kalmunai	-	-	-	-	-	-	-	-
	Trincomalee	-	-	-	-	2	2	-	-
North Central	Anuradhapura	-	-	-	-	27	134	28	185
	Polonnaruwa	-	-	-	-	48	218	1,511	3,473
North Western	Chilaw	22	34	-	-	-	-	-	-
	Kuliyapitiya	-	-	-	-	-	-	51	74
	Kurunegala	-	-	-	-	59	323	-	-
	Puttalam	-	-	-	-	-	-	-	-
Northern	Jaffna	-	-	2	2	-	-	-	-
	Kilinochchi	-	-	-	-	-	-	-	-
	Mannar	-	-	-	-	-	-	-	72
	Mullaitivu	-	-	-	-	-	-	-	-
	Vavuniya	-	-	45	103	-	-	-	-
Sabaragamuwa	Embilipitiya	-	-	-	-	-	-	-	-
	Kegalle	-	-	-	-	-	-	-	-
	Ratnapura	-	-	-	-	-	-	-	-
Southern	Balapitiya	-	-	-	-	-	-	-	-
	Hambanthota	-	-	-	-	151	392	267	183
	Mahamodara	7	32	-	7	21	42	265	707
	Matara	-	-	-	-	12	160	-	429
	Tangalle	-	-	-	-	18	9	-	-
UVA	Badulla	-	-	-	-	175	695	60	344
	Mahiyanganaya	-	-	-	-	-	-	-	-
	Monaragala	-	-	-	-	58	325	7	70
Western	Avissawella	-	-	24	54	-	-	-	-
	Colombo	-	-	17	60	213	1,214	586	3,448
	Gampaha	-	-	-	-	-	-	-	-
	Homagama	-	-	-	-	-	-	-	-
	Kalubowila	-	-	1	2	4	30	301	1,986
	Kalutara	1	1	-	-	-	-	-	-
	Negombo	33	108	5	68	45	100	-	264
	Panadura	-	-	1	4	1	-	-	203
	Ragama	-	-	-	-	-	14	-	-
	Wathupitiwala	-	-	59	107	-	-	10	15
Total		63	175	157	410	849	3,700	3,261	12,483

ANNEX-2

Details of the awareness programmes conducted by STD clinics in 2022

Province	Clinic	Lectures		Exhibitions		Workshops	
		Number of Programmes	Number of participants	Number of Programmes	Number of participants	Number of Programmes	Number of participants
Central Province	Dambulla	9	350	-	-	-	-
	Kandy	18	873	1	850	1	144
	Matale	73	6,225	-	-	3	113
	Nawalapitiya	4	130	-	-	-	-
	Nuwara Eliya	12	455	-	-	-	-
Eastern Province	Ampara	82	8,421	1	570	-	-
	Batticaloa	19	885	-	-	-	-
	Kalmunai	19	1,260	-	-	-	-
	Trincomalee	28	1,198	-	-	5	361
North Central province	Anuradhapura	40	2,180	-	-	9	270
	Polonnaruwa	27	3,546	-	-	-	-
North Western Province	Chilaw	19	1,908	2	7,203	-	-
	Kuliyapitiya	9	920	-	-	1	40
	Kurunegala	96	9,434	-	-	-	-
	Puttalam	13	650	-	-	-	-
Northern Province	Jaffna	23	1,320	-	-	-	-
	Kilinochchi	13	830	-	-	-	-
	Mannar	27	1,343	-	-	6	233
	Mullaitivu	36	1,457	-	-	-	-
	Vavuniya	58	2,589	-	-	-	-
Sabaragamuwa Province	Embilipitiya	-	-	-	-	-	-
	Kegalle	60	4,433	1	400	8	274
	Ratnapura	16	1,630	-	-	4	200
Southern Province	Balapitiya	4	586	-	-	-	-
	Hambanthota	53	3,873	-	-	11	620
	Mahamodara	1	60	-	-	-	-
	Matara	11	2,098	9	16,260	1	30
	Tangalle	12	402	-	-	2	172
	Badulla	135	13,648	-	-	-	-
UVA Province	Mahiyangana	15	1,276	-	-	1	48
	Monaragala	104	6,080	-	-	-	-
	Avissawella	7	268	-	-	-	-
Western Province	Colombo	10	429	2	8,000	-	-
	Gampaha	2	110	-	-	-	-
	Homagama	-	-	-	-	-	-
	Kalubowila	63	1,276	-	-	3	85
	Kalutara	19	9,615	-	-	5	242
	Negombo	15	565	-	-	1	45
	Panadura	15	4,398	-	-	4	254
	Ragama	6	263	-	-	-	-
	Wathupitiwala	6	870	-	-	-	-
Total		1,179	97,854	16	33,283	65	3,131



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